A NON-CLINICAL PREVENTION COMPANION

EDITION 1.0 — 2026

The Student Well-being *Guide*

Evidence-informed recommendations, situations, and protective practices for university and college students navigating depression and anxiety risk.

FOR STUDENTS · BY THE XAI-RAG PROJECT TEAM

IMPORTANT — PLEASE READ

This guide is for prevention and psychoeducation only. It is not a substitute for professional care, diagnosis, or treatment. It does not provide therapy instructions, medication advice, or crisis intervention. If you are in immediate danger, contact your local emergency services or a crisis line (see Chapter 15).

HOW TO USE THIS GUIDE

A Companion, Not a Prescription

This guide was written for students. It collects what the research literature says about protecting your mental health during the university and college years — and translates it into language you can actually use.

SAFETY NOTE This guide is for prevention and psychoeducation only. It is not a clinical instrument, does not diagnose depression or anxiety, and does not replace a qualified counsellor, psychologist, or physician. If you are in crisis, contact your local emergency services or a crisis line listed in Chapter 15.

Student life is one of the most psychologically demanding transitions in modern life. You are simultaneously managing an academic load, an evolving social world, financial pressures, identity questions, and the long shadow of what comes after graduation. For many students, this is also the first time they are making daily decisions about sleep, food, movement, money, and relationships without the structure of a family household. It is a lot. It is normal to feel stretched thin. It is also normal to need a map.

This guide is that map. It is built on a simple, evidence-based premise: **most depression and anxiety in students is not a personal failure but the predictable result of identifiable risk factors meeting insufficient protective factors**. Some of those factors are inside your control. Some are not. Knowing which is which — and acting on the ones you can influence — is most of what prevention actually looks like.

What this guide does

- **Explains** what depression and anxiety are (and are not) in plain language, without diagnostic checklists.
- **Maps** the risk and protective factors that the research literature consistently links to student mental health.
- **Translates** those factors into concrete, daily practices — sleep, movement, social connection, study habits, financial wellness, digital boundaries, mindfulness, and help-seeking.
- **Normalises** the experience of struggling and the act of asking for help.

- **Signposts** when and how to seek professional support, including what to expect from a first counselling appointment.

What this guide does not do

- It does **not** diagnose depression, anxiety, or any other condition.
- It does **not** provide therapy protocols, treatment plans, or medication information.
- It does **not** perform crisis intervention. Chapter 15 lists emergency contacts but does not instruct you in how to intervene in someone else's crisis.
- It does **not** replace the judgement of a qualified clinician, counsellor, or physician who knows your specific situation.
- It does **not** claim that any single practice will "prevent" depression — mental health is shaped by biology, history, environment, and chance, not by lifestyle alone.

A note on language

Throughout this guide we use phrases like *elevated screening risk*, *model-based estimate*, *associated with*, and *may benefit from speaking with a qualified counsellor*. These are deliberate choices. They reflect the truth that mental health exists on a continuum, that correlation is not causation, and that no book — however well-researched — can tell you what you "have". When we say a habit is "associated with" lower distress, we mean that across many students, the habit and the lower distress tend to show up together. We are not saying the habit caused the lower distress, and we are not saying it will work the same way for you. You are a person, not an average.

How the chapters are organised

The guide moves from understanding to action. Chapters 1 to 3 lay the foundation: what depression and anxiety are, what raises risk, and what buffers it. Chapters 4 to 12 walk through the major domains of student life where prevention actually happens — sleep, movement, social connection, academics, money, screens, mindfulness, transitions, and identity. Chapters 13 to 15 cover the boundary between self-care and professional support, including how to help a friend and what to do in a crisis. Chapter 16 is a workbook for building your own plan. The vignettes and resource directory close the guide.

You do not need to read this guide front to back. If you are sleep-deprived and anxious before exams, go straight to Chapter 7. If you have just moved to a new city and feel profoundly alone, Chapter 6 is for you. If you are worried about a

friend, Chapter 14. Use the table of contents as a menu, not a syllabus.

A word on the evidence base

The recommendations in this guide draw on the peer-reviewed literature on student mental health, sleep, exercise, social connection, financial stress, mindfulness, and help-seeking. Where the evidence is strong, we say so. Where it is suggestive but not conclusive, we say that too. Where something is widely recommended but not actually well-studied in students, we say so honestly. We have avoided drawing on clinical treatment manuals, diagnostic guides, or medication literature — not because those sources are unimportant, but because they are tools for clinicians, not for students reading on their own.

> A SMALL INVITATION
>
> Read this guide the way you would read a good friend's advice: seriously, but not solemnly. Underline what resonates. Argue with what does not. Notice what makes you feel more able to cope, and what makes you feel worse. The goal is not to memorise a list of recommendations but to gradually build a personal sense of what protects *you*.

If at any point while reading you find yourself thinking "this sounds like me, and it is more than I can manage alone", please skip ahead to Chapter 13. Asking for help is not a failure of self-care. It is, very often, the most important form of self-care there is.

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CHAPTER 1

Understanding Student Mental Health

Before we talk about what to do, we need a shared picture of what depression and anxiety actually are — and what they are not — in the lives of students.

SAFETY NOTE This chapter describes depression and anxiety in non-clinical, psychoeducational language. It does not provide diagnostic criteria. If you recognise yourself in these descriptions and the experience is interfering with your daily life, please see Chapter 13.

Mental health, in the simplest sense, is the ongoing relationship between you and your experience. It is not a fixed trait you either have or lack. It shifts across days, weeks, semesters, and years in response to sleep, stress, relationships, season, hormones, history, finances, and sheer luck. The World Health Organization defines mental health as "a state of well-being in which an individual realizes his or her own abilities, can cope with the normal stresses of life, can work productively and fruitfully, and is able to make a contribution to his or her community." Notice what this definition does *not* say. It does not say "the absence of all distress." It does not say "always feeling good." It says that, on the whole, you can function, cope, and contribute. Distress is part of the deal. Sustained inability to function, cope, or connect is when distress becomes something more.

For students, this distinction matters because university life is, by design, a period of high normal stress. You are learning difficult things, navigating new social structures, often living away from family for the first time, and making consequential decisions about identity and vocation. A certain amount of anxiety before exams, sadness after a breakup, or loneliness during the first weeks of a new term is not a sign that something is wrong with you. It is a sign that you are a human being in a demanding environment. The question is not "do I ever feel bad?" but "is the way I feel bad starting to shrink my life?"

What we mean by "depression" in this guide

In everyday speech, "depressed" can mean anything from "a bit down today" to a clinical condition that severely impairs functioning. In this guide we use the word carefully. When we say "depression", we mean a pattern of low mood, loss of interest in things that usually matter to you, low energy, changes in sleep or appetite, difficulty concentrating, and negative thoughts about yourself, the world, or the future — a pattern that has lasted for at least two weeks and that is interfering with your ability to study, socialise, or care for yourself. We are not diagnosing you. We are describing a cluster of experiences that, when they show up together and persist, warrant attention and often professional support.

Depression in students often does not look the way people imagine. It is not always weeping in bed. Among high-achieving students in particular, depression frequently shows up as **anhedonia** — a flattening of pleasure in things you used to love — combined with **high-functioning avoidance**: you keep going to class, keep submitting work, keep smiling at friends, but inside everything feels grey

and effortful. You might describe it as "running on fumes" or "going through the motions." This presentation is easy to miss in yourself and easy to hide from others, which is why it often goes unaddressed until something breaks.

> **A USEFUL REFRAME**
>
> Depression is not a character flaw, a weakness of will, or a sign that you are not tough enough for university. It is a multifactorial condition in which biology, psychology, and environment all play a role. Blaming yourself for being depressed is like blaming yourself for having asthma. The useful question is never "why am I weak?" but "what is contributing to this, and what would help?"

What we mean by "anxiety" in this guide

Anxiety, in its adaptive form, is the body-mind's response to perceived threat. A spike of anxiety before an exam sharpens attention and improves performance — up to a point. Anxiety becomes a problem when it is too intense, too frequent, too easily triggered, or too slow to subside. The student who cannot sleep because their mind will not stop racing about a presentation three days away is experiencing anxiety that has crossed from helpful to harmful. So is the student who avoids checking their email because they are afraid of what a professor might have said. So is the student whose stomach turns every time they walk into a seminar room.

Like depression, anxiety in students often wears a socially acceptable mask. "I'm just a perfectionist." "I'm just a worrier." "I just care a lot about my grades." Some of this is genuine temperamental variation. But when the worry is constant, when it spreads from one domain (exams) to many (friendships, health, the future, the state of the world), when it produces physical symptoms like a racing heart, shallow breathing, digestive distress, or muscle tension, and when it starts driving avoidance — procrastination, skipping class, not opening messages — it has moved from personality into something that warrants attention.

~1 in 3

University students worldwide report symptoms consistent with clinically significant depression or anxiety at any given time. Prevalence rose sharply during and after the COVID-19 pandemic and has not returned to pre-pandemic levels.

The spectrum: from normal stress to distress to disorder

It helps to think of mental health as a spectrum rather than a binary. At one end is **flourishing**: you feel good, function well, handle setbacks, and find meaning in your activities. Next is **moderate mental health**: you have your ups and downs but generally cope. Then comes **languishing** — a sense of stagnation, emptiness, and "going through the motions" that is not yet depression but is a clear warning sign. Further along is **subthreshold distress**: more persistent symptoms that don't quite meet criteria for a diagnosis but are clearly affecting your life. At the far end is **clinical depression or anxiety**, where symptoms are persistent, intense, and disabling.

This spectrum view matters for two reasons. First, it normalises the middle of the spectrum — most students spend most of their time somewhere between flourishing and languishing, and that is not pathology. Second, it means that **moving one step toward flourishing is a meaningful outcome**, even if you never reach the far end of the spectrum. Prevention is not about eliminating all distress. It is about nudging yourself, and your environment, in the direction of flourishing.

Common myths about depression and anxiety in students

Myth 1: "If you just tried harder, you would feel better."

Effort matters, but depression and anxiety are not effort problems. Telling a depressed student to "just push through" is like telling someone with a sprained ankle to "just run faster." The useful intervention is not more willpower — it is removing the obstacles (sleep debt, isolation, perfectionism, unaddressed trauma) that are making the effort so much harder than it should be.

Myth 2: "Anxiety is always bad."

Low-to-moderate anxiety is a normal, often helpful, response to challenging situations. The goal is not to eliminate anxiety but to keep it in the range where it informs rather than paralyses you. Some of the most accomplished people in the world are also some of the most anxious; they have learned to work *with* their nervous system rather than against it.

Myth 3: "Talking about it makes it worse."

The evidence is the opposite. Across many studies, students who acknowledge distress and talk about it with a trusted person — friend, family member, counsellor, or peer supporter — tend to recover faster and more completely than those who suppress it. Naming an emotion reduces its intensity in the brain. Silence amplifies it.

Myth 4: "Young people are just more anxious these days."

Partially true, partially a misread. Young people *report* more anxiety and depression than in previous decades — but this is a combination of genuine increases (driven by sleep loss, social media, economic uncertainty, climate anxiety, and post-pandemic effects) and decreased stigma about reporting. The "kids these days are weak" framing is both unkind and inaccurate. The environment has changed; the humans have not.

Myth 5: "You would know if you were depressed."

Not always. Depression often creeps in gradually, and because it affects the very faculties you would use to notice it (motivation, perspective, self-reflection), it can be more visible to friends and family than to the person experiencing it. If multiple people who care about you have expressed concern recently, take that seriously — even if you do not feel "depressed" in the dramatic way you might imagine.

Warning signs that warrant attention

None of the following signs, in isolation, means you have a mental health condition. But if several of them show up together and have persisted for two weeks or more, they are worth taking seriously as a signal that something needs to change — whether that change is in your habits, your environment, or your access to support.

IN YOUR MOOD

- Persistent low mood, sadness, or emptiness that does not lift with rest or pleasant activities
- Loss of interest in or pleasure from things you usually enjoy
- Feeling numb, flat, or "disconnected" from your own life

- Irritability, short temper, or disproportionate frustration over small things
- Hopelessness about the future, or feeling that nothing you do matters

IN YOUR BODY

- Difficulty falling asleep, staying asleep, or waking too early — or sleeping far more than usual without feeling rested
- Appetite changes (eating much less or much more than usual)
- Persistent fatigue, even after adequate sleep
- Unexplained headaches, stomach-aches, muscle tension, or chest tightness
- Noticeable slowing of movement or speech (in depression) or restlessness and being unable to sit still (in anxiety)

IN YOUR THINKING

- Difficulty concentrating, remembering, or making simple decisions
- Recurrent negative thoughts about yourself ("I'm a failure", "I'm a burden", "Everyone is doing better than me")
- Recurrent worry that you cannot switch off, often about future events you cannot control
- Intrusive thoughts of death, dying, or not wanting to be here (see Chapter 15 immediately if these are present)
- Difficulty reading, watching films, or following conversations — a kind of cognitive fog

IN YOUR BEHAVIOUR

- Withdrawing from friends, family, or activities you used to enjoy
- Avoiding classes, deadlines, or messages (the "I'll deal with it tomorrow" pattern that snowballs)
- Increased use of alcohol, cannabis, or other substances to cope
- Reckless behaviour, impulsive decisions, or taking unusual risks
- Drop in academic performance that you cannot easily explain

> *"The question is not 'do I ever feel bad?' but 'is the way I feel bad starting to shrink my life?'"*

The biopsychosocial model: why no single cause tells the whole story

Modern understandings of depression and anxiety reject single-cause explanations. Instead, the dominant framework is the **biopsychosocial model**: mental health emerges from the interaction of biological, psychological, and social factors. None of these three is primary; they shape each other continuously.

Biological factors include genetics (you can inherit a temperament more reactive to stress, or a lower baseline for certain neurotransmitter activity), hormones (thyroid, menstrual cycle, contraception, puberty), physical health (chronic illness, pain, nutritional deficiencies), and the direct effects of substances and sleep on brain function. None of these is destiny; all of them set the range.

Psychological factors include your core beliefs about yourself and the world (often laid down in childhood), your coping styles (do you tend to face problems, avoid them, or ruminate on them?), your history of trauma or adversity, your perfectionism, your self-compassion, and your sense of agency or control over your life. These are malleable — which is the entire basis of effective psychotherapy — but they are malleable slowly, through repetition, reflection, and often skilled help.

Social factors include your relationships, your sense of belonging, your financial security, your housing, your exposure to discrimination or violence, your access to green space and safe streets, the demands of your academic programme, and broader cultural forces such as economic inequality, political instability, or the aftermath of a pandemic. Some of these are within your control. Many are not. The student who is working two jobs to afford tuition while caring for a sick parent is not experiencing the same "stress" as the student whose main worry is choosing a major. Pretending otherwise is both inaccurate and unkind.

The biopsychosocial model is also why this guide spends so much time on social and environmental factors (sleep, money, belonging, screens, identity) rather than only on individual habits. **Prevention happens at the intersection of all three layers.** A student who is biologically vulnerable, psychologically self-critical, and socially isolated is at far higher risk than a student with only one of those risk factors — and intervening on any one layer can shift the whole system.

What we mean by "elevated screening risk"

Because this guide accompanies a screening-communication project, you will see the phrase "elevated screening risk" in several places. It is deliberately clinical-sounding but importantly non-diagnostic. What it means is: *when a validated screening questionnaire (such as the PHQ-9 for depression or the GAD-7 for anxiety) is administered, your score falls in a range that suggests a higher probability of currently experiencing a clinically significant condition, and follow-up with a qualified professional is recommended.*

What it does *not* mean: that you definitely have a diagnosis, that you must start treatment, that you are dangerous to yourself or others, or that there is something fundamentally broken about you. It is information. It is the mental-health equivalent of a smoke alarm going off — worth investigating, not worth panicking about, and definitely not something to ignore. If a screening tool, a friend, a doctor, or your own reading of this guide suggests elevated screening risk, Chapter 13 walks you through what to do next.

The good news

This chapter has covered a lot of difficult material, so it is worth closing on a clearer note. Depression and anxiety, even at clinically significant levels, are among the most thoroughly studied and most treatable of all mental health conditions. The majority of students who experience them recover fully or substantially, especially when they receive appropriate support reasonably early. Many of the risk factors covered in the next chapter are modifiable. Many of the protective factors covered in Chapter 3 are accessible to almost any student, regardless of background or circumstances. The point of reading this guide is not to scare you but to put tools in your hands. The next chapter explains what those tools are up against.

CHAPTER 2

Risk Factors Common in Student Life

Risk factors are conditions or experiences that, across many students, are associated with a higher probability of depression or anxiety. Knowing them is not the same as predicting your future — but it tells you where to look.

SAFETY NOTE Risk factors are statistical associations, not predictions. Having several risk factors does not mean you will develop depression or anxiety. Having none does not guarantee you will not. Use this chapter to understand the landscape, not to label yourself.

A useful way to think about risk factors is to borrow an image from public health. Imagine depression and anxiety as a fire. A fire needs fuel, oxygen, and a spark. Risk factors are the fuel — they make a fire *more likely* and, when it starts, *more intense*. Protective factors, covered in Chapter 3, are the firebreaks — they make a fire less likely to start and easier to contain if it does. None of this is deterministic. People with many risk factors never develop a condition; people with few sometimes do. But on average, across populations, the pattern is clear: more risk factors, fewer protective factors, more depression and anxiety.

Student life amplifies several risk factors in particular ways. The transition from a structured family environment to a largely self-structured academic environment, often combined with a new city, new social network, new financial responsibilities, and the developmental task of forming an adult identity, creates a perfect storm of fuel. This chapter walks through the most consistent and well-studied risk factors in the student mental-health literature, what each one looks like in daily life, and a preview of "what helps" — the practices and conditions that, according to the research, can blunt the effect of each risk factor. The detailed practices themselves are covered in Chapters 4 to 12.

2.1 Loneliness and social isolation

Loneliness is the single most consistent psychosocial predictor of depression in the student literature. It is also one of the most under-acknowledged. There is a cultural narrative that university is automatically a social, vibrant time of life, and students who feel lonely often conclude that there is something uniquely wrong with them. The data say otherwise: in the Healthy Minds Study, the largest annual survey of student mental health in the United States, more than half of students report moderate to severe loneliness, and the figure has been rising for over a decade.

It is important to distinguish **being alone** from **feeling lonely**. Being alone is a state — and many students genuinely enjoy and need solitude. Feeling lonely is a subjective experience of *the gap between the social connection you want and the social connection you have*. You can feel lonely in a crowded lecture hall, at a party, or in a relationship. You can also be alone for a weekend and feel perfectly content. Loneliness is not about the number of people around you; it is about the perceived quality and depth of your connection to them.

Loneliness raises depression risk through several mechanisms. Acutely, it activates the same threat-detection circuits in the brain that respond to physical pain — humans are social primates, and isolation registers as danger. Chronically, it elevates cortisol (the body's main stress hormone), disrupts sleep, increases systemic inflammation, and reduces the practice of emotion regulation that comes from talking through problems with trusted others. Lonely students ruminate more, sleep worse, and recover more slowly from setbacks.

> **WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 6)**
>
> Loneliness is one of the most modifiable risk factors in this list. Belonging can be built. The evidence-based moves are: small repeated contact with the same people (clubs, study groups, part-time work), depth over breadth in friendships, explicit disclosure (vulnerability invites vulnerability), peer-support programmes, and reducing the comparison-and-withdrawal cycle driven by social media. See Chapter 6 for the full picture.

2.2 Academic stress and perfectionism

Academic stress is the most universal student risk factor — universal because it is built into the structure of higher education. Deadlines, exams, grades, comparisons to peers, and the perceived link between academic outcome and life trajectory are constant sources of activation for the stress-response system. Not all academic stress is harmful; the right amount, with the right recovery, can sharpen performance and build resilience. The problem is that modern student life rarely allows sufficient recovery. Back-to-back deadlines, the encroachment of academic work into evenings and weekends, and the "always available" expectation created by email and learning-management systems mean that the body's stress response is activated for weeks or months without the dips that allow it to reset.

Within academic stress, **perfectionism** deserves special attention. Perfectionism is not the same as having high standards. It is the belief that anything less than flawless is failure, combined with harsh self-criticism when perfection is (inevitably) not achieved. Perfectionism is strongly and specifically associated with anxiety, depression, eating disorders, and burnout in students. It is also socially rewarded — universities select for it, employers reward it, and families praise it — which makes it easy to mistake for a virtue rather than a vulnerability.

A closely related pattern is **maladaptive procrastination**, which is not laziness but an anxiety-driven avoidance response. The perfectionist student does not put off work because they do not care; they put it off because starting it confronts them with the possibility of not doing it perfectly. The avoidance provides short-term relief and long-term catastrophe — late, rushed, lower-quality work, combined with days or weeks of dread. Understanding this loop is the first step out of it.

> WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 7)
>
> Time management that respects energy, not just hours. Realistic standards ("good enough" is often the optimal standard). Breaking large tasks into the smallest possible first step. Self-compassion practices that interrupt the perfectionism–procrastination loop. Honest conversations with advisors about workload. Use of accommodations when needed. See Chapter 7.

2.3 Financial stress and food insecurity

Financial stress is one of the most under-discussed risk factors in student mental health, in part because money is a particularly shame-laden topic in many cultures. Students who are struggling financially often hide it from peers, which deepens isolation and prevents access to support. The Healthy Minds Study and parallel research in Europe, Asia, and Latin America consistently find that financial stress is one of the strongest predictors of depression, anxiety, and suicidal ideation in students — stronger, in many studies, than academic stress.

A particularly severe form is **food insecurity**, defined as limited or uncertain availability of nutritionally adequate and safe food. Surveys in the United States and elsewhere find that between 20 and 40 percent of students at four-year institutions experience some level of food insecurity in a given year. Food

insecurity is associated with depression, anxiety, lower GPA, and higher rates of dropout. It is not a "lifestyle" issue; it is a basic-needs failure with direct mental-health consequences.

Financial stress raises depression risk through several pathways. Directly, it chronically activates the stress response and limits access to the very things that would protect mental health (nutritious food, stable housing, time for sleep and exercise, social activities that cost money, mental-health care). Indirectly, it forces students into long work hours that compete with sleep, study, and social connection, and it produces a constant low-grade cognitive load — the mental bandwidth consumed by tracking expenses, deciding what to skip, and planning around shortfalls. This cognitive load is itself exhausting and is sometimes called "scarcity mindset": the brain, preoccupied with immediate deficits, loses capacity for longer-term planning and enjoyment.

> **WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 8)**
>
> Knowing what support exists and how to access it: campus food pantries, emergency aid funds, government benefits you may be eligible for, fee waivers, scholarships, and basic-needs navigation services. Reducing shame by talking to a trusted advisor. Budgeting practices that reduce cognitive load rather than increase it. See Chapter 8.

2.4 Sleep disruption

If you read only one section of this chapter, read this one. Sleep disruption is the single most biologically direct risk factor for both depression and anxiety in students. It is also, conveniently, one of the most modifiable. Sleep is not "recovery from life"; it is an active neurological process during which the brain consolidates memory, regulates emotion, clears metabolic waste, and recalibrates the stress-response system. When sleep is short, fragmented, or mistimed, every one of those processes degrades — and mood, attention, and resilience degrade with them.

Students are uniquely vulnerable to sleep disruption. The transition to university often means the loss of parental sleep structure; later class times; the social pull of late-night activities; the use of screens, often in bed, late into the night; the biologically driven shift in circadian rhythm that occurs in late adolescence and pushes bedtimes later; and the consumption of caffeine, alcohol, and other

substances that fragment sleep. The result, in many student populations, is an average sleep duration of six hours or less on weekdays — about 90 minutes short of the seven to nine hours most young adults need.

Chronically short sleep does not just make you tired. It produces measurable changes in the brain's amygdala (the structure that processes threat), making it roughly 60 percent more reactive to negative stimuli. It reduces prefrontal cortex activity, weakening the executive control that normally keeps the amygdala in check. It impairs the consolidation of positive emotional memories while sparing negative ones — meaning that, over time, sleep-deprived students remember the bad parts of their week and forget the good. It dysregulates appetite hormones, increasing cravings for high-sugar, high-fat foods. It raises inflammation and cortisol. And it directly worsens symptoms of both depression and anxiety.

> WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 4)
>
> A consistent wake time (more important than a consistent bedtime). Morning light exposure. Screens off or shifted to warm/dim modes in the hour before bed. Caffeine cutoff in early afternoon. Treating sleep as non-negotiable protected time, not as the flexible part of your schedule. See Chapter 4.

2.5 Identity-based stress and discrimination

For students who belong to one or more marginalised groups — by race, ethnicity, gender, sexuality, disability, religion, immigration status, or social class — the standard risk factors above are compounded by **identity-based stress**. This includes overt experiences of discrimination and harassment; subtler but persistent microaggressions; the cognitive load of code-switching (adjusting your speech, behaviour, or appearance to fit the dominant culture); the experience of being a "token" representative of your group in classrooms or social settings; and the constant low-level question of whether a given interaction or outcome was shaped by bias.

Identity-based stress is not just additive; it is multiplicative. A first-generation student of colour who is also LGBTQ+ and financially stressed is not experiencing four separate risk factors — they are experiencing a single, interlocking system of stressors that shape every aspect of university life. The mental-health impact is well-documented: students from marginalised groups show higher rates of depression and anxiety, lower rates of formal help-seeking,

and higher rates of negative experiences when they do seek help. This is not a deficiency in the students. It is a deficiency in the environments they are navigating.

It is worth naming one specific form of identity-based stress that is often invisible to those who do not experience it: **minority stress**, the chronic excess stress that results from being a member of a stigmatised group, independent of any single discriminatory event. Minority stress explains why, even in the absence of overt discrimination on a given day, students from marginalised groups can show measurable physiological activation (elevated cortisol, restless sleep) that their peers do not. It is the weather, not the storm.

> WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 12)
>
> Finding community with others who share your identity. Seeking out culturally responsive counsellors or peer supporters. Naming microaggressions when you have the energy to. Identifying allies who will do the naming for you when you do not. Recognising that the problem is the environment, not your reaction to it. See Chapter 12.

2.6 Transition stress

Every transition — even a wanted one — is stressful. The first year of university is one of the largest transitions in modern life: new city, new housing, new social network, new academic expectations, new financial responsibilities, new food, new sleep schedule, new identity. The brain handles novelty well in small doses and poorly in avalanches, and the first term of university is an avalanche. The same is true, to a lesser extent, of returning after a break or a leave of absence, transferring institutions, studying abroad, and graduating.

Transition stress is often misread by students as personal failure. "I should be enjoying this." "Everyone else is having the time of their life." The reality is that adjustment to a major transition takes six to twelve weeks for most students, and the dip in mood and increase in anxiety during the first weeks is normal, not pathological. The risk arises when the dip does not lift, when it deepens into withdrawal, or when it interacts with pre-existing vulnerabilities — a student with a history of depression is at higher risk of relapse during a transition, and a student with no history may experience their first depressive episode during one.

WHAT HELPS (PREVIEW — FULL CHAPTER IN CHAPTER 11)

Setting realistic expectations for the transition period. Keeping anchors from before the transition (a friend from home, a familiar routine, a hobby). Building structure into the first weeks. Avoiding the "everyone else is fine" comparison by talking honestly with peers. See Chapter 11.

2.7 Substance use

Substance use occupies an awkward place in student mental health. Alcohol, cannabis, nicotine, and (increasingly) prescription stimulants are woven into the social fabric of many universities, and the line between use and misuse is often blurred. The relationship between substance use and depression or anxiety runs in both directions: students who are depressed or anxious often self-medicate with substances, and substance use itself can cause or worsen depression and anxiety. Untangling the two is a job for a clinician; what this guide can do is describe the patterns and their risks.

Alcohol is the most common. It is a central nervous system depressant — meaning it literally depresses neural activity — and while it produces short-term relaxation and disinhibition, it disrupts sleep architecture, worsens mood over the following days, and interacts dangerously with many antidepressant medications. Binge drinking (typically defined as five or more drinks in a single occasion for men, four or more for women) is associated with markedly higher rates of depression, anxiety, academic problems, and sexual assault in student populations.

Cannabis occupies a more contested space. For some students, occasional use produces relaxation and social enjoyment without obvious harm. For others — particularly those with a personal or family history of psychosis, and particularly with high-THC products — cannabis use is associated with increased anxiety, panic attacks, motivational syndrome, and, in vulnerable individuals, the onset of psychotic symptoms. The relationship with depression is more ambiguous but appears to be dose-dependent: heavy daily use is consistently associated with worse outcomes; occasional use is harder to distinguish from no effect.

Prescription stimulants (used without a prescription, or above prescribed doses) are common in academic environments as "study aids". They produce short-term focus but disrupt sleep, raise anxiety, and produce rebound fatigue that worsens the next day's performance. Their use as cognitive enhancers is also associated with higher rates of anxiety disorders in the medium term.

WHAT HELPS

Tracking your own use honestly. Noticing whether substances are being used for fun (often fine) or to cope with distress (often a warning sign). Talking to a counsellor or doctor if you are concerned. Avoiding mixing substances. Following low-risk drinking guidelines if you choose to drink. Recognising that "everyone does it" is not the same as "this is working for me".

2.8 Childhood adversity and trauma history

Students who experienced significant adversity in childhood — abuse, neglect, household dysfunction, parental mental illness or substance use, loss of a parent, bullying, or exposure to violence — are at substantially elevated risk for depression, anxiety, and several other mental health conditions during the university years. The Adverse Childhood Experiences (ACE) literature has documented a strong dose-response relationship between the number of childhood adversities and adult risk of mental and physical health conditions. This does not mean adversity is destiny. It means it changes the baseline.

Trauma-informed care has become the standard framework in university counselling services, and most services now screen for trauma history and adjust their approach accordingly. If you have a trauma history, you may find that certain experiences in university — a romantic breakup, a difficult roommate situation, a medical procedure, a classroom discussion of sensitive material — produce reactions that feel out of proportion to the current event. This is not weakness. It is the nervous system recognising a pattern. Skilled support can help you distinguish past from present and build capacity to stay regulated in triggering situations.

WHAT HELPS

Working with a trauma-informed therapist (most university counselling services can refer you). Learning grounding techniques (see Chapter 10). Building predictability into your routine. Recognising that healing is non-linear and that setbacks are part of the process, not failures.

2.9 Family history and genetic vulnerability

Depression and anxiety run in families, partly through shared environment and partly through genetic influence. Having a first-degree relative (parent or sibling) with major depression roughly doubles your lifetime risk; for anxiety disorders the increase is similar. This is not a gene "for" depression; it is a set of many genetic variants, each with small effect, that collectively shape temperamental traits like neuroticism (sensitivity to negative emotion) and stress reactivity.

Knowing your family history is useful, not because it dooms you, but because it informs how seriously to take early warning signs. If you have a strong family history and you notice persistent low mood, it is reasonable to seek support sooner rather than later. The same applies to other conditions that travel with depression and anxiety: bipolar disorder, attention-deficit/hyperactivity disorder, eating disorders, substance use disorders, and psychotic disorders. Family history is one piece of information among many — but it is a piece worth knowing.

2.10 The interaction effect: why risk factors multiply rather than add

The most important thing to understand about risk factors is that they do not simply add up. They interact. A student who is sleep-deprived, lonely, financially stressed, and socially anxious is not four times as likely to develop depression as a student with none of those factors — they are many times more likely, because each factor makes the others worse. Sleep deprivation worsens social anxiety. Financial stress worsens sleep. Loneliness amplifies the cognitive impact of sleep deprivation. And so on.

This is the bad news. The good news, which we will return to repeatedly, is that the same interaction effect works in reverse. Improving sleep makes it easier to handle financial stress. Building social connection makes it easier to improve sleep. Reducing substance use makes it easier to build social connection. Prevention is rarely about a single intervention; it is about shifting the system in a healthier direction, which then tends to keep shifting.

> *"You do not have to fix everything at once. You have to fix something, and let the system rebalance around it."*

The next chapter turns to the other half of the picture: the protective factors that, across many studies, are associated with lower risk of depression and anxiety, and with faster recovery when they do occur.

CHAPTER 3

Protective Factors and Sources of Strength

If risk factors are the fuel, protective factors are the firebreaks. They do not guarantee you will never struggle — but they make struggling less likely, less severe, and shorter when it happens.

SAFETY NOTE Protective factors are not a "cure" for depression or anxiety. They support mental health in general. If you are currently experiencing significant symptoms, please see Chapter 13 — protective practices and professional support work together, not instead of each other.

Protective factors are the mirror image of risk factors. Where risk factors raise the probability of depression or anxiety across a population, protective factors lower it. They do so through three broad mechanisms. Some **buffer** — they reduce the impact of stress when it arrives, like shock absorbers on a car. Some **build** — they increase your baseline capacity, like strengthening a muscle so it is less likely to tear under load. Some **restore** — they help you return to baseline after a stressor, like sleep after a hard day. Most protective factors do all three.

An important framing: protective factors are not "things you must do, or else". The goal is not to optimise every factor on this list. The goal is to know which ones are strong in your life already (those are your existing sources of strength), which ones are weak or absent (those are your leverage points), and which ones you could realistically strengthen without making well-being itself another stressful obligation. A guide that turns self-care into a checklist of failures is doing the opposite of what it should.

3.1 Social support and belonging

Social support is the most consistently documented protective factor for mental health in the entire research literature — not just in students, but across the lifespan and across cultures. The Harvard Study of Adult Development, the longest-running study of human well-being ever conducted, concluded after more than eighty years that the quality of close relationships in midlife predicted late-life health and happiness more powerfully than any other factor, including income, education, and genetics. The same pattern holds for students: those with at least one or two genuinely close relationships show markedly lower rates of depression and anxiety, and recover faster when they do struggle.

Three features of social support matter more than the raw number of contacts. **Perceived support** — your felt sense that someone would be there for you in a crisis — is more predictive than actual support received. **Emotional support** (being heard and understood) is more protective than **instrumental support** (being helped with practical tasks), though both matter. And **belonging** — the felt sense of being part of a group that values you — is a distinct construct from either, and is particularly important in the student transition years when older belonging structures (family, school, hometown) are loosening and new ones (friend groups, clubs, departments, residence halls) are forming.

Belonging is not about popularity. A student with two close friends and one steady community (a club, a sports team, a part-time job, a religious group) often has a stronger sense of belonging than a student with a hundred acquaintances and no steady community. The research on loneliness interventions consistently finds that *depth* of connection matters more than *breadth*. Quality over quantity is not a cliché here; it is the empirical finding.

> **A USEFUL SELF-CHECK**
>
> Name one person you could call at 3 a.m. if you were in trouble. Name one group or community in which you feel known. Name one place on or near campus where you feel you belong. If any of these is hard to answer, Chapter 6 has concrete suggestions for how to build them — and how long it realistically takes.

3.2 Sleep quality

Sleep was covered as a risk factor in Chapter 2; here we reframe it as a protective factor, because the relationship runs in both directions. Just as poor sleep raises depression risk, good sleep is one of the strongest protective factors

against both depression and anxiety, and is one of the strongest predictors of recovery when symptoms are present. Many students who believe they have treatment-resistant depression or anxiety are, on closer inspection, chronically sleep-deprived; restoring sleep produces substantial improvement without any other intervention.

"Good sleep" in the protective sense is not just about total hours, though that matters. It is about **consistency** (similar bed and wake times across the week, including weekends), **timing** (sleep that aligns with your circadian rhythm, which for most young adults means sleeping roughly 11 p.m. to 7 a.m.), **continuity** (few awakenings, and quick return to sleep when they happen), and **sufficiency** (seven to nine hours for most young adults). The detailed practices for achieving these are in Chapter 4.

3.3 Regular physical movement

If sleep is the most biologically direct protective factor, physical movement is the second. Across hundreds of studies and dozens of meta-analyses, regular moderate physical activity is associated with lower rates of depression and anxiety, faster recovery from both, and lower risk of relapse. The effect sizes are not trivial: in some meta-analyses, regular exercise compares favourably with first-line treatments for mild to moderate depression. The mechanisms are multiple — improved sleep, reduced inflammation, increased brain-derived neurotrophic factor (BDNF, a protein that supports neural growth and resilience), social contact if the activity is done with others, time in nature if the activity is outdoors, and a sense of agency and accomplishment that counters the helplessness central to depression.

"Regular physical movement" deserves unpacking. The evidence base points to **moderate-intensity aerobic activity** (brisk walking, cycling, swimming, dancing) for roughly 150 minutes per week, plus some **strength training** (bodyweight exercises, weights, climbing, yoga) on two or more days per week, as a target associated with substantial mental-health benefits. But the relationship is dose-dependent from zero — meaning that *any* movement is better than none, and the largest jump in benefit comes from going from sedentary to even small amounts of regular activity. The student who is currently doing nothing does not need to train for a marathon. Walking twenty minutes a day, four days a week, is a meaningful intervention.

It is also worth naming what movement is *not* a substitute for. Exercise is not a treatment for severe depression, eating-disorder recovery, trauma work, or any condition in which professional support is warranted. The "exercise cures

everything" framing is well-meaning but, for students with significant symptoms, can become a guilt-inducing demand. The relationship between movement and mental health is real and substantial; it is also partial. Use it as one protective layer among several.

150 min/week

The moderate-intensity movement target most consistently associated with mental-health benefits in students. But any movement is better than none — the curve is steepest at the low end.

3.4 Flourishing, meaning, and purpose

Flourishing is a slightly awkward word for an important concept: the experience of functioning well, not just feeling good. The psychologist Corey Keyes, who developed the most widely used measure of flourishing, defined it as a combination of emotional well-being (positive affect, life satisfaction), psychological well-being (self-acceptance, personal growth, purpose, environmental mastery, autonomy, positive relations), and social well-being (social acceptance, social actualisation, social contribution, social coherence, social integration). A student can be flourishing even on days when they feel sad, and a student can have low flourishing even on days when they feel fine — because flourishing is about how your life is going over time, not how you feel in a given moment.

Flourishing is protective against depression and anxiety in two ways. First, it represents a higher baseline — the higher your baseline of well-being, the further you have to fall before reaching clinical concern. Second, the components of flourishing (especially purpose, growth, and positive relations) actively counter the psychological processes that drive depression: hopelessness, helplessness, self-criticism, and isolation. A student who feels that their life has direction and meaning is more resilient to academic setbacks, relationship breakups, and the inevitable disappointments of early adulthood.

Purpose does not require a grand mission. It can be as specific as "I want to become a teacher" or as broad as "I want to be a good friend and leave things better than I found them". What matters is that the purpose feels genuinely yours, that it connects to something larger than your immediate comfort, and that it is actionable in daily life. Many students experience a "purpose gap" in the university years — the old certainties about who they are and what they are for have loosened, and new ones have not yet formed. This is a developmental task, not a deficiency. It takes time, experimentation, and often a few false starts.

A SMALL REFLECTION

What is one thing in the last week that gave you a sense of meaning or being fully yourself? It does not need to be impressive. It might be a conversation, a moment of focus on a piece of work, a small kindness, a creative act, a walk in a place you love. Noticing these moments is itself a flourishing practice.

3.5 Self-compassion

Self-compassion, as defined by the researcher Kristin Neff, has three components: **self-kindness** (treating yourself with the same warmth you would offer a friend) instead of harsh self-judgment; **common humanity** (recognising that struggle is part of being human, not a personal failing) instead of isolation; and **mindful awareness** (noting your pain without suppressing it or over-identifying with it) instead of rumination. Self-compassion is not the same as self-esteem, which is contingent on evaluations of worth and tends to collapse precisely when it is most needed. Self-compassion is available regardless of how you are doing.

Across many studies, self-compassion is associated with lower rates of depression and anxiety, lower perfectionism, lower fear of failure, greater resilience after setbacks, and healthier relationships. It is also one of the few protective factors that can be cultivated relatively quickly — brief structured practices, often taking ten minutes a day for a few weeks, have been shown in randomised trials to produce measurable improvements in self-compassion and reductions in self-criticism. Chapter 10 includes several such practices.

The most common misconception about self-compassion is that it will lower your standards or make you lazy. The evidence is the opposite. Self-compassionate students tend to take more responsibility for failures (because the cost of admitting a mistake is lower when you are not also attacking yourself for it), persist longer at difficult tasks, and recover more quickly from setbacks. Self-criticism, by contrast, is associated with procrastination, avoidance, and worse performance over time — not because self-critical students do not care, but because the emotional cost of trying is so high.

3.6 Help-seeking attitudes

Willingness to seek help when you need it is, paradoxically, one of the strongest protective factors against the worsening of mental-health conditions. Students who hold positive attitudes toward help-seeking — who believe that asking for

support is reasonable, that counselling is effective, that mental-health struggles are not shameful — tend to access support earlier, when problems are more manageable, and recover faster. Students who hold negative attitudes tend to delay, sometimes for years, by which point problems have often compounded.

Help-seeking attitudes are shaped by many things: cultural background, family messages about vulnerability, prior experiences with helpers (positive or negative), gender norms (men, on average, hold more negative attitudes), and the perceived accessibility and cultural responsiveness of the support available. The attitudes are not fixed. Exposure to stories of peers who have sought help and benefited, conversations with trusted mentors, and even brief contact with a sympathetic counsellor can shift them substantially.

It is worth noting that "help-seeking" includes a wide range of behaviours, not only formal counselling. Talking to a friend, a family member, a religious leader, a peer supporter, an academic advisor, or a GP can all be forms of help-seeking. The protective factor is not "going to therapy" specifically; it is the willingness to acknowledge distress and let another person in. Chapter 13 walks through the range of options in detail.

3.7 Routine and structure

It is unglamorous but well-established: students with regular daily routines show lower rates of depression and anxiety than students with chaotic schedules, even when total sleep, study time, and social contact are held constant. Routine is protective for several reasons. It reduces the cognitive load of constant decision-making. It anchors sleep and meal timing, which in turn anchors circadian rhythm. It provides a sense of agency and predictability that buffers against the unpredictability of student life. And it makes healthy behaviours the default rather than something that requires willpower every time.

"Routine" does not mean rigidity. A good routine has anchors — fixed points around which the day is organised (a regular wake time, a regular meal time, a regular study block) — and flexibility in between. It is also personal: a routine that works for a morning person will not work for an evening person, and a routine that works during term will not work during exam week or holidays. The goal is not to find the perfect schedule but to have *some* schedule, and to revisit it when it stops serving you.

3.8 Connection to nature

Time in natural environments — parks, forests, coastlines, even well-treed urban streets — is associated with lower levels of depression, anxiety, and perceived stress, and with improved attention and mood. The mechanisms are likely multiple: exposure to natural light (which supports circadian rhythm and vitamin D synthesis), reduced exposure to the sensory overload of urban environments, the invitation to slower breathing and walking pace, and the cognitive restoration that comes from what environmental psychologists call "soft fascination" — the gentle, effortless attention that natural scenes invite.

For students, the practical implication is not that you need to retreat to a forest. It is that *some* regular contact with nature — a walk through a park on the way to class, studying outside on good-weather days, a weekend hike, even tending a small plant in your room — appears to confer measurable benefit. The effect has been documented with as little as 120 minutes per week in natural settings, and the benefit flattens out beyond that — meaning you do not need to live in a national park to get most of the upside.

3.9 Cognitive flexibility

Cognitive flexibility is the capacity to shift perspectives, consider alternative interpretations of events, and adjust your behaviour when circumstances change. It is the opposite of rumination (the repetitive, narrowly focused negative thinking central to depression) and of rigid all-or-nothing thinking (central to anxiety and perfectionism). It is also one of the few protective factors that can be deliberately strengthened through practice.

Cognitive flexibility is not the same as positive thinking, which can shade into denial. It is the ability to hold multiple honest interpretations of an event at once. A flexible response to a poor grade, for example, might be: "This grade is disappointing, and it does not define my ability or my future, and I can learn from what went wrong, and I am allowed to feel upset about it for a while." An inflexible response might be: "I am bad at this subject, I will never succeed, this proves I should drop out." Same event, very different consequences for mood and behaviour.

The practices in Chapter 10 — mindfulness, self-compassion, journaling, perspective-taking — are largely cognitive flexibility practices. They work not by changing what happens to you but by widening the space between event and interpretation, in which your own agency lives.

3.10 Financial stability and basic-needs security

Just as financial stress is a powerful risk factor, financial stability and basic-needs security are powerful protective factors. The student who knows their rent, food, and tuition are covered has substantially more cognitive and emotional bandwidth for everything else — academic work, social connection, sleep, exercise, identity exploration — than the student who is constantly calculating trade-offs. This is not a personal virtue; it is a structural condition. The protective factor here is not "being responsible with money" but *actually having enough*, which is shaped far more by family wealth, financial aid policy, national student support systems, and labour market conditions than by individual choices.

For students who do not have the protective factor of financial stability, the equivalent protective move is **knowing how to access the support that does exist**: emergency aid funds, food pantries, government benefits, fee waivers, scholarships, and basic-needs navigation services on campus. Chapter 8 walks through these in detail. Knowing the landscape does not erase the underlying stress, but it reduces the additional cognitive load of not knowing where to turn when a crisis hits.

3.11 Spiritual, religious, or philosophical practice

For students who hold a spiritual, religious, or philosophical practice, that practice can be a substantial protective factor. The mechanisms are several: a framework for meaning-making in the face of suffering, a community of shared practice, regular rituals that structure time and provide emotional regulation, practices of prayer or meditation that overlap with evidence-based contemplative techniques, and an ethical orientation that can counter some of the individualism and achievement-orientation of university life.

It is worth noting that the protective effect is not uniform across all forms of religious or spiritual involvement. Forms that emphasise a benevolent relationship with the divine and a supportive community are generally protective; forms that emphasise divine punishment, exclusivity, or conflict with one's identity (particularly around sexuality or gender) can be risk factors in their own right. The protective factor is not "religion" in the abstract but a specific kind of relationship with meaning, community, and the transcendent.

3.12 A protective mindset: how to use this list

Reading a list of twelve protective factors, it is easy to feel that you "should" be doing all of them, and to feel like a failure for not. That response, while understandable, is the opposite of what protective factors are for. A more useful exercise is to read the list as an inventory of your current strengths and your current leverage points.

YOUR PROTECTIVE FACTOR INVENTORY

For each of the twelve protective factors below, mark whether it is currently **strong**, **moderate**, **weak**, or **absent** in your life. Be honest, not aspirational. Then circle the two or three you would most like to strengthen, and note one small step you could take for each. This is not a contract; it is a snapshot.

1. Social support and belonging — _____
2. Sleep quality — _____
3. Regular physical movement — _____
4. Flourishing, meaning, and purpose — _____
5. Self-compassion — _____
6. Help-seeking attitudes — _____
7. Routine and structure — _____
8. Connection to nature — _____
9. Cognitive flexibility — _____
10. Financial stability / knowledge of support — _____
11. Spiritual, religious, or philosophical practice — _____
12. Other protective factor specific to your life: _____ — _____

Two or three I would most like to strengthen:

One small step for each:

The remaining chapters in this guide go into depth on the protective factors most amenable to change: sleep (Chapter 4), movement (Chapter 5), social connection (Chapter 6), academic habits (Chapter 7), financial wellness (Chapter 8), digital boundaries (Chapter 9), mindfulness and self-compassion (Chapter 10), and identity and belonging (Chapter 12). They are not a checklist. They are a menu. Pick what speaks to your current life, and trust that strengthening one protective factor tends, over time, to strengthen several.

CHAPTER 4

Sleep and Circadian Health

If there is a single habit most worth protecting in student life, it is sleep. Almost every other protective factor in this guide is harder to maintain when sleep is short, and easier when sleep is sufficient.

SAFETY NOTE This chapter covers sleep hygiene and circadian health for general well-being. It does not address diagnosed sleep disorders. If you suspect you have sleep apnoea, narcolepsy, restless legs syndrome, or another sleep disorder, please consult a physician — these conditions require clinical assessment, not lifestyle changes.

Sleep is the foundation on which almost every other mental-health-relevant process rests. During sleep, the brain consolidates the day's learning into long-term memory, processes emotional experiences, clears metabolic waste products (including amyloid-beta, the protein implicated in Alzheimer's disease), regulates the hormones that control appetite and stress, and recalibrates the neural circuits that govern mood and attention. None of this is optional. None of it can be done while awake. And almost all of it is degraded when sleep is short, fragmented, or mistimed.

The student-specific challenge is that university life is structurally hostile to good sleep. Late classes, social events that run into the early hours, shared living spaces with mismatched schedules, the ease of streaming another episode, the constant cognitive activation of unfinished academic work, the social pressure to be available late at night, and the widespread use of caffeine and alcohol all conspire to push bedtimes later and fragment the sleep that does occur. Add the biologically driven circadian shift of late adolescence — which pushes the natural bedtime of an eighteen-year-old roughly two hours later than that of a forty-year-old — and you have a perfect recipe for chronic sleep deprivation disguised as normal student life.

4.1 How much sleep do students actually need?

The National Sleep Foundation and most major health bodies recommend seven to nine hours of sleep per night for young adults (eighteen to twenty-five). Most students need somewhere in this range, with individual variation. A useful way to find your own number: on a week when you have no alarm, no deadlines, and no social pressure to be up at a particular time, how many hours do you sleep before waking naturally? That number, give or take thirty minutes, is your sleep need. For most students it falls between seven and a half and nine hours.

The gap between need and reality is striking. In the Healthy Minds Study and similar surveys, the median student reports sleeping six hours or less on weekdays — a deficit of ninety minutes to three hours per night compared to their need. Over a week, that compounds into ten to twenty hours of sleep debt, which the brain does not simply "make up" by sleeping in on Saturday. The cognitive and emotional effects of chronic partial sleep deprivation — even at modest deficits — are well documented: reduced attention, poorer memory consolidation, slower reaction time, impaired judgment, greater emotional reactivity, lower frustration tolerance, and, over months, measurably higher rates of depression and anxiety.

7–9 hrs

The recommended nightly sleep range for young adults. The median student reports six hours or less — a chronic deficit with measurable effects on mood, attention, and resilience.

4.2 The two things that matter most: when you wake, and when you see light

If you remember only two things from this chapter, remember these. **First, your wake time sets your biological clock more powerfully than your bedtime does.** The brain's master clock (the suprachiasmatic nucleus, in the hypothalamus) takes its primary cue from the onset of light in the morning. Waking at a consistent time, seven days a week, anchors your circadian rhythm and makes falling asleep at a reasonable hour the following night far easier. Sleeping in on weekends — the classic student pattern — produces what chronobiologists call "social jetlag": the body clock drifts later by two or three hours over the weekend, then is jolted back early on Monday morning, producing the cognitive and emotional equivalent of flying across several time zones every week.

Second, morning light exposure is the single most powerful external signal your brain uses to set its clock. Getting bright light into your eyes within thirty to sixty minutes of waking — ideally outside, even on an overcast day (outdoor light is far brighter than indoor light, even when it does not feel that way) — produces a cascade of effects: it suppresses lingering melatonin (the sleepiness hormone), raises cortisol to its appropriate morning peak, advances your circadian rhythm so that you become sleepy earlier in the evening, and improves mood directly through pathways that are not fully understood but are well documented. Fifteen to thirty minutes of outdoor light in the first hour of waking is one of the highest-leverage sleep interventions available, and it is free.

> THE TWO ANCHORS
>
> 1. Wake at the same time every day, including weekends, within a thirty-minute window.
> 2. Get bright outdoor light into your eyes within an hour of waking, for at least fifteen minutes.
>
> If you do nothing else from this chapter, do these two. They will, over a few weeks, move your bedtime earlier and improve your sleep quality without any direct effort on bedtime itself.

4.3 The evening wind-down

The two anchors above handle the morning. The evening requires a different set of practices, all aimed at one goal: allowing your nervous system to shift from the sympathetic activation of the day into the parasympathetic state required for sleep. You cannot *make* yourself sleep, any more than you can make yourself fall in love. You can only create the conditions in which sleep becomes possible, and then let it arrive.

LIGHT MANAGEMENT

Just as morning light advances your clock and signals daytime, evening light delays your clock and signals daytime-extended. The brain is particularly sensitive to blue-spectrum light (the kind emitted by screens, LED bulbs, and many overhead lights), which directly suppresses melatonin production. The ideal evening light environment is dim, warm-toned, and indirect — think lamp-light rather than overhead fluorescents, and warm amber rather than cool white. Practically: in the two hours before bed, lower the lights in your room, switch devices to night-shift or warm-colour modes, and consider using a small warm-toned reading lamp rather than a full-spectrum screen.

SCREEN USE IN BED

The single most common sleep-disruptive habit in students is the use of phones, tablets, or laptops in bed. The disruption is twofold: the light suppresses melatonin (as above), and the content — whether social media, news, video, or messaging — produces cognitive and emotional activation that is the opposite of the wind-down state. Even apparently "relaxing" content (a film, a feed of funny videos) keeps the brain in a state of vigilance that delays sleep onset and fragments the sleep that follows. The most protective practice is also the simplest: **keep screens out of the bed entirely**. Charge your phone across the room, not on the nightstand. Use a physical alarm clock if you need one. Treat the bed as a place for sleep (and intimacy), not for content consumption.

This is hard. The phone-in-bed pattern is reinforced by social norms, by the fear of missing messages, by the use of the phone as an alarm, and — for many students — by the use of the phone as the last social contact of the day. The shift is worth making anyway. Most students who manage it report, within two to three weeks, that they fall asleep faster, wake less in the night, and feel more rested in the morning. The first week is difficult; the habit stabilises quickly after that.

CAFFEINE

Caffeine is the most widely used psychoactive substance in the world, and the most common pharmacological disruptor of student sleep. Its half-life — the time it takes for the body to eliminate half of a given dose — is approximately five to six hours in healthy adults, but varies considerably (up to ten hours in some people). This means that a coffee consumed at 4 p.m. still has half its caffeine active in your system at 9 to 10 p.m., and a quarter of it active at 2 to 4 a.m. Caffeine blocks adenosine, the neurotransmitter that builds up during wakefulness and produces sleep pressure; with adenosine blocked, you do not feel sleepy even when your brain needs sleep.

The practical guidance, supported by sleep research, is to **stop consuming caffeine by early afternoon** — ideally 2 p.m., and no later than 4 p.m. for most people. This includes coffee, most teas, energy drinks, many soft drinks, and some pain medications. Decaf coffee is not caffeine-free; a typical cup contains 2 to 15 mg of caffeine (compared to 80 to 200 mg in regular coffee), which is usually fine in the evening but worth knowing. If you are caffeine-sensitive or a slow metaboliser, an earlier cutoff will help.

ALCOHOL

Alcohol is often misperceived as a sleep aid because it produces sedation. It is, in fact, a major disruptor of sleep quality. While it may shorten the time to fall asleep, it suppresses REM sleep in the first half of the night (the stage most involved in emotional processing and memory consolidation), causes fragmentation and awakening in the second half as it is metabolised, worsens snoring and sleep apnoea, and produces a net loss of restorative sleep even when total time in bed is normal. The cumulative effect of regular evening drinking on student mood and cognition is substantial and often underestimated.

The practical guidance: if you choose to drink, do so earlier in the evening rather than right before bed, keep quantities moderate, and recognise that even one or two drinks will measurably reduce the restorative quality of that night's sleep. The "I sleep fine after a drink" perception is usually a misread of sedation as sleep.

COGNITIVE OFFLOADING

Many students struggle to fall asleep because their minds begin processing the day's events and tomorrow's tasks the moment they lie down. This is not a character flaw; it is what the brain does when it has not had a dedicated time and space for that processing. A simple, evidence-based practice is the **"brain dump"**: ten or fifteen minutes before bed, write down everything on your mind — tasks for tomorrow, unfinished conversations, worries, ideas. The act of externalising them reduces the cognitive load of holding them in working memory and signals to the brain that they are safely stored elsewhere. Many students who try this report a noticeable reduction in sleep-onset latency within a week.

4.4 What to do if you cannot sleep

Even with perfect sleep hygiene, there will be nights when you cannot sleep — because of stress, because of a late coffee, because of a disrupted routine, or for no obvious reason at all. How you respond to a bad night matters more than the bad night itself. The most common mistake is to stay in bed, increasingly anxious about not sleeping, calculating the hours until the alarm, and inadvertently conditioning the brain to associate the bed with wakefulness and anxiety. This is how acute insomnia becomes chronic.

THE 20-MINUTE RULE

If you have been in bed for roughly twenty minutes and are not asleep — and especially if you are feeling frustrated — get up. Go to another room, keep the lights low, and do something calm and not screen-based: read a physical book, stretch, listen to a quiet podcast, do a brief breathing exercise. Return to bed only when you feel genuinely sleepy. Repeat as many times as needed. This protects the bed-sleep association and prevents the spiral of sleep anxiety.

The day after a bad night, two things help. **First, do not "catch up" by sleeping in or napping extensively.** A short nap (twenty to thirty minutes, before 3 p.m.) is fine; a three-hour afternoon nap or sleeping until noon will shift your body clock and perpetuate the problem. **Second, get bright light and some movement early in the day, and resist the temptation to over-caffeinate.** The goal is to build sleep pressure through the day so that the following night's sleep comes easily and resets the cycle.

4.5 Naps: use carefully

Naps can be a useful tool for managing sleep debt, but they are easy to misuse. The evidence base points to a few principles. **Short is better than long** — naps of ten to twenty minutes produce alertness and mood benefits without significant sleep inertia (the grogginess that comes from waking from deep sleep). Naps longer than thirty minutes enter deep sleep and produce more inertia than benefit, unless you complete a full ninety-minute cycle. **Early is better than late** — napping before 3 p.m. is unlikely to disrupt that night's sleep; napping after 4 p.m. often will. **Consistency helps** — if you nap regularly at the same time, your body learns to expect it and the nap becomes more restorative.

Naps are not a substitute for insufficient nighttime sleep. A student who is sleeping five hours a night and "catching up" with daily naps is still chronically sleep-deprived, even if the naps take the edge off. Use naps as a supplement to adequate nighttime sleep, not as a replacement for it.

4.6 Chronotype: working with, not against, your biology

Chronotype is the natural tendency of your body to sleep at a particular time — popularised as "morning larks" versus "night owls", though the reality is a continuum. Chronotype is partly genetic, partly age-dependent (late adolescence and early adulthood skew later; midlife and beyond skew earlier), and not a moral failing. The student who genuinely cannot fall asleep before 1

a.m. and genuinely cannot wake before 9 a.m. is not lazy or undisciplined; they are operating on a different biological schedule than the student who naturally wakes at 6 a.m.

The challenge is that much of university life — early lectures, exams scheduled for 9 a.m., institutional expectations of morning productivity — is built around morning-type schedules, which disadvantages evening-type students. Where you have flexibility, work with your chronotype: schedule demanding cognitive work for your personal peak hours (which, for evening types, may be late afternoon or evening), protect your morning sleep if your schedule allows, and advocate for yourself with advisors and disability services if early starts are producing genuine dysfunction.

That said, chronotype is not infinitely malleable but is also not fixed. The two anchors above — consistent wake time and morning light — can shift an evening chronotype earlier by an hour or two over several weeks, which can make a meaningful practical difference. The goal is not to force yourself into a schedule that fights your biology, but to find the latest schedule that still allows you to function in the institutional structures you need to navigate.

4.7 When sleep does not improve: red flags

If you have implemented the practices in this chapter consistently for four to six weeks and your sleep has not improved — or if you have any of the following — please consult a physician or your university's student health service:

- Loud snoring, gasping, or pauses in breathing observed by a roommate or partner (possible sleep apnoea)
- Persistent difficulty falling asleep (more than thirty minutes) or staying asleep, three or more nights per week, for a month or more (possible chronic insomnia, which responds well to specific clinical treatments)
- Overwhelming daytime sleepiness despite adequate time in bed (possible sleep apnoea, narcolepsy, or other sleep disorder)
- Restless legs, frequent limb movements, or uncomfortable sensations in the legs at night (possible restless legs syndrome)
- Sleep that is significantly disrupted by anxiety, intrusive thoughts, or trauma-related dreams (consider evaluation for an anxiety disorder or trauma-related condition)
- Sleep that has deteriorated sharply in association with low mood, loss of interest, or hopelessness (consider evaluation for depression)

These signs do not mean something is fundamentally wrong with you. They mean a clinician should help you figure out what is going on. Sleep disorders are common, frequently missed in student populations, and often highly treatable. The fastest route to better sleep, in those cases, is professional assessment, not more sleep-hygiene tips.

> *"You cannot always control when you sleep. You can almost always control when you wake, and when you see light. Start there."*

Chapter 5 turns to the body in motion — the second major biological protective factor, and the one most tightly linked to sleep in its effects on mood.

CHAPTER 5

Movement, Body, and Mood

The body and the mind are not separate systems that happen to share an address. They are one system, and what you do with your body shapes what you feel in your mind.

SAFETY NOTE This chapter is about general well-being through movement, nutrition, and substance awareness. It does not provide exercise prescriptions for medical conditions, eating-disorder recovery guidance, or substance-use treatment. If you have a history of disordered eating, please approach the nutrition section with care, and consider reading it with a trusted support person or clinician.

The relationship between body and mood is bidirectional, constant, and largely underestimated. When you are depressed, your body slows — movements become effortful, posture collapses, breathing shallows, facial expression flattens. When your body is held in a slow, collapsed, shallow state for long periods — sitting at a desk for hours, scrolling on a phone, lying in bed — your mood tends to follow. The brain is constantly reading the body's signals and inferring an emotional state from them. If the body says "I am exhausted and shut down", the brain is inclined to agree.

This is not a claim that you can posture or stretch your way out of depression. It is a claim that the body is one of the levers through which mood is regulated, and that small changes in how you hold and move your body can produce meaningful shifts in how you feel. The lever is not magic, but it is real, and it is one of the most accessible protective factors available to a student.

5.1 What kind of movement, and how much?

The research literature on exercise and mental health is large and occasionally contradictory, but a few consistent findings emerge. **Moderate-intensity aerobic activity** — brisk walking, jogging, cycling, swimming, dancing, rowing, anything that raises your heart rate to roughly 60 to 75 percent of maximum and sustains it — produces the largest and most reliable mood benefits. **Resistance training** (weightlifting, bodyweight exercises, resistance bands) shows comparable benefits for depression and may be particularly helpful for anxiety. **Mind-body practices** like yoga, tai chi, and qigong show robust benefits for anxiety in particular, likely through their combination of movement, breath regulation, and meditative attention. **High-intensity interval training** shows benefits but is harder to sustain and carries higher injury risk for sedentary beginners.

The most-studied dose is roughly **150 minutes per week of moderate-intensity activity**, plus two sessions of resistance training. This is the same target recommended for general cardiovascular health, and it is associated in meta-analyses with a roughly 25 to 30 percent reduction in depression risk. But — and this is important — the dose-response curve is steepest at the low end. Going from zero activity to even thirty minutes of walking per day produces a larger mood benefit than going from thirty minutes to two hours. The student who is currently sedentary does not need to find time for the gym five days a week. Walking for twenty minutes, four days a week, is a meaningful and sustainable intervention.

~30 min

Three to five times per week of moderate movement is enough to produce meaningful mental-health benefits for most students. More is fine, but the additional benefit per minute declines sharply past this threshold.

5.2 Practical movement for student lives

University life does not always make movement easy. Gym memberships may be expensive or intimidating; schedules may be irregular; weather may be uncooperative; shared living spaces may be small. The good news is that almost none of the mental-health benefit of movement requires a gym, special equipment, or large blocks of time. What follows is a menu of realistic options, not a prescription.

Walking

The single most accessible form of movement. Twenty to thirty minutes a day, ideally outside. Walking to campus instead of taking the bus, taking the stairs, walking between buildings for breaks, walking with a friend (which doubles as social connection).

Active transport

Cycling to campus, skateboarding, using a scooter. Combines transportation and exercise, which is one of the most reliable ways to maintain a movement habit without finding additional time.

Bodyweight exercise

Push-ups, squats, lunges, planks, yoga flows. Requires no equipment, can be done in a small room, and provides both strength and mobility benefits. Many free, high-quality routines are available online (Yoga with Adriene, the NHS fitness studio, etc.).

Group classes

Many universities offer free or low-cost group fitness classes. The scheduled structure and social element can help students who struggle to maintain a solo habit. Dance, martial arts, climbing, and team sports all count.

Running / jogging

Time-efficient and mood-elevating, especially outdoors. Couch-to-5k programmes are a well-designed gradual entry point for beginners. Run with a friend or club for accountability.

Swimming

Particularly good for students with joint issues, asthma, or high anxiety. The water provides sensory regulation that some students find uniquely calming. Often available cheaply through university pools.

5.3 The mood-while-moving effect

One of the most reliable findings in exercise psychology is that a single bout of moderate exercise produces an immediate, short-term improvement in mood — reduced anxiety, increased positive affect, improved cognitive function — that lasts for one to three hours afterward. This "acute" effect is distinct from the longer-term benefits of regular exercise and is one of the most practically useful tools in the student mental-health toolkit. A twenty-minute walk before a stressful seminar, a quick bodyweight routine before an exam, a swim after a difficult conversation — these are not workouts in the fitness sense; they are mood regulation in real time.

The implication is that movement can be deployed strategically, not just habitually. If you know a particular class or meeting reliably raises your anxiety, scheduling fifteen minutes of movement beforehand is one of the most effective interventions available. If you find yourself ruminating in the evening, a walk outside often breaks the loop more effectively than another hour of trying to think your way out of it. Movement is not just "good for you in general"; it is a tool you can use for specific situations, in real time.

5.4 Outdoor time, light, and nature

Movement done outdoors is, on average, more beneficial for mood than the same movement done indoors — likely because of the combined effects of natural light (which supports circadian regulation and vitamin D synthesis), the cognitive restoration of natural environments (the "soft fascination" mentioned in Chapter 3), and the simple fact that outdoor movement tends to be sustained longer and feels less effortful. The mental-health benefits of "green exercise" — activity in natural settings — are documented across many studies, with effects appearing even in urban parks and quite small green spaces.

The practical takeaway: when you have a choice, take your movement outside. Walk through a park rather than on a treadmill. Cycle along a river rather than on a stationary bike. Study outside on a blanket rather than at a desk by a window. The combined protective effects of movement, light, and nature are larger than any of the three alone.

5.5 Nutrition: basics, not diet culture

Nutrition occupies an awkward place in student mental-health guides. On one hand, what you eat genuinely affects how you feel — through blood sugar regulation, gut-brain signalling, nutrient status, and the simple fact that hungry or undernourished students struggle to regulate mood. On the other hand, the cultural space around food and body is saturated with misinformation, moralism, and disordered eating patterns, and a well-meaning chapter on nutrition can easily become a trigger for students with eating disorders or body-image struggles.

What follows is deliberately basic, deliberately non-prescriptive, and deliberately framed around adequacy rather than restriction. The goal is not to optimise your diet; it is to make sure that food is supporting your mood rather than undermining it.

EAT ENOUGH

The most common nutrition-related cause of low mood and anxiety in students is simply not eating enough — either because of food insecurity (see Chapter 8), because of skipped meals due to a chaotic schedule, or because of restrictive eating patterns. The brain runs on glucose; when blood sugar drops, mood drops, attention drops, irritability rises, and anxiety increases. Eating regular meals — three meals a day, plus snacks as needed — is more protective of mood than the specific composition of those meals. If you are skipping breakfast and lunch and eating one large meal in the evening, your mood and concentration are paying for it.

EAT REASONABLY

A "reasonable" diet, in the evidence-based sense, looks roughly like: plenty of vegetables and fruit (whatever is affordable and accessible); adequate protein (beans, lentils, eggs, dairy, fish, meat, tofu — whatever fits your preferences and budget); some whole grains; some healthy fats (olive oil, nuts, seeds, avocado, oily fish); and limited but not zero intake of highly processed foods, sugary drinks, and alcohol. The Mediterranean diet pattern has the strongest evidence

base for mental health, but the specific pattern matters less than the basics: enough food, regular meals, some plants, some protein, not too much sugar or alcohol.

CAFFEINE

Caffeine was covered in Chapter 4 in the context of sleep. From a mood perspective, low to moderate caffeine intake (up to roughly 400 mg per day — about four cups of coffee) is associated with improved alertness, concentration, and mood for most people. Higher intake, and intake later in the day, produces anxiety, jitteriness, and disrupted sleep — which then worsens mood the next day. If you notice your anxiety rising with your caffeine intake, the relationship is probably causal, not coincidental.

ALCOHOL

Alcohol is a central nervous system depressant. While it produces short-term relaxation and disinhibition, it worsens mood over the following one to three days, disrupts sleep (see Chapter 4), interacts dangerously with many medications, and is associated with substantially higher rates of depression and anxiety in students who drink heavily. Low-risk drinking guidelines (when not contraindicated by medication, pregnancy, or personal history) suggest no more than fourteen drinks per week for men and women, with no more than four drinks on any single occasion for women and five for men. Many students exceed these guidelines without recognising it, particularly in cultures where heavy weekend drinking is normative.

OTHER SUBSTANCES

Cannabis, nicotine, prescription stimulants used non-medically, and other recreational substances all have documented relationships with student mental health, covered briefly in Chapter 2. The general principle: any substance that produces short-term relief or enhancement at the cost of sleep, mood, or functioning in the following days is, on balance, working against you. The occasional use may be fine; the regular use-as-coping is a warning sign. If you are using substances to manage distress, please talk to a counsellor or doctor — not because you are in trouble, but because there are usually more effective and less costly ways to address the underlying distress.

5.6 Movement and eating-disorder recovery

For students in recovery from an eating disorder, or with a complicated relationship with food and exercise, much of the standard advice in this chapter needs modification. Movement that becomes compulsive, food rules that

become restrictive, and body-tracking that becomes obsessive can all masquerade as "healthy habits" while actively undermining recovery. If this resonates with you, please consider working with a clinician who specialises in eating disorders — the principles of recovery are specific, evidence-based, and very difficult to navigate alone. University counselling services and student health services can refer you to appropriate specialists.

5.7 A note on body image

Body image is a significant mental-health issue in its own right for many students, and one that intersects with movement, food, and mood in complex ways. The cultural pressure to achieve a particular body shape — intensified by social media, fitness marketing, and peer comparison — can turn the protective practices of movement and eating into sources of distress. A student who exercises primarily to alter their body, and who feels anxious or self-punishing when they cannot, is not getting the mental-health benefits of movement; they are getting a different problem dressed up as self-care.

The reframe worth considering: **movement is for what your body can do, not for how it looks**. Strength, endurance, flexibility, balance, the capacity to walk up a hill without breathlessness, the capacity to play with a child, the capacity to recover from an injury — these are the meaningful outcomes of movement, and they are achievable at almost any body shape and size. If your movement practice is making you more preoccupied with your body, it is worth asking whether the practice is serving you or serving a cultural script you did not choose.

> *"Move because it changes how you feel, not because it changes how you look. The first is reliably achievable. The second is a trap."*

5.8 Building a sustainable movement habit

Most students do not have a movement problem; they have a habit-formation problem. The following principles, drawn from the behaviour-change literature, make habit formation substantially easier.

- **Start absurdly small.** A daily five-minute walk is a better foundation than a three-times-weekly hour at the gym, because the daily practice is more likely to stick. You can scale up later; first establish the habit.

- **Attach to an existing anchor.** "I will walk for ten minutes after my morning class" is more likely to stick than "I will walk more". The anchor makes the cue automatic.
- **Reduce friction.** Lay out your workout clothes the night before. Keep walking shoes by the door. Choose a movement you can do without travelling far. Friction is the enemy of consistency.
- **Make it social.** Walking with a friend, joining a class, signing up for a club sport — social commitment converts movement from a willpower task to a relational one, which is far more durable.
- **Track without obsessing.** A simple checkmark on a calendar is enough. Detailed tracking apps can help but can also become a source of pressure and perfectionism.
- **Expect lapses and plan for them.** A missed day is not a failed habit; it is a normal part of the process. The students who maintain movement long-term are not the ones who never miss; they are the ones who return to it after missing.
- **Choose activities you actually like.** The "best" form of movement is the one you will do. If you hate running, do not run. Walk, dance, swim, climb, cycle, garden. Movement you enjoy is movement you will repeat.

Chapter 6 moves from the body to one of the most important protective factors of all: social connection and the experience of belonging.

CHAPTER 6

Social Connection and Belonging

We are social primates wired for connection. The single most powerful protective factor for student mental health is the quality of your close relationships — and the most powerful risk factor is their absence.

SAFETY NOTE This chapter addresses loneliness and social difficulty in general terms. It does not address abusive relationships, harassment, or assault. If you are in an unsafe relationship or have experienced assault, please contact your campus safety office, a trusted staff member, or a crisis line (see Chapter 15).

The previous chapter explained how movement protects the brain. This chapter explains how other people do. The evidence is overwhelming: across decades of research, across cultures, across the lifespan, the quality of close relationships is the single strongest predictor of human well-being. The Harvard Study of Adult Development, following hundreds of men (and eventually their partners and children) for more than eighty years, found that the quality of relationships in midlife predicted health and happiness in late life more powerfully than income, education, social class, IQ, or genetics. The finding has been replicated in many other cohorts. The shape of the relationship is consistent: closeness matters; isolation harms; the effect is large.

For students, this finding lands at a particularly delicate moment. The transition to university loosens the older belonging structures — family, school, hometown, childhood friendships — before the new ones have fully formed. The first weeks and months of university are often experienced as profoundly lonely, even by students who appear socially successful from the outside. And the modern student environment, with its heavy use of digital communication, its competitive academic culture, and its often fragmented social geography, can make the formation of deep connection harder than at any previous point in the student's life.

6.1 Alone versus lonely: a critical distinction

Solitude and loneliness are not the same thing, and confusing them causes a great deal of unnecessary suffering. **Solitude** is the physical state of being alone. It can be restorative, creative, deeply enjoyable — many people, including many students, need regular solitude to function well. **Loneliness** is the subjective distress that arises from the gap between the social connection you want and the social connection you have. You can feel lonely in a crowd, lonely in a relationship, lonely at a party. You can also be alone for an entire weekend and feel perfectly content.

The practical importance of this distinction is that **the antidote to loneliness is not always more people**. Sometimes it is — when you are genuinely isolated, more contact is the answer. But often the antidote is *deeper* connection with the people already in your life, or a shift in the quality of your relationship with yourself when you are alone. The student who has a hundred acquaintances and no one to call at 3 a.m. is lonely in a way that no quantity of additional acquaintances will fix. The student with two close friends and a rich inner life when alone is not lonely at all.

A USEFUL SELF-CHECK

In the last week, how many conversations did you have that felt genuinely connecting — where you felt seen and where you saw the other person? One or two such conversations per week is associated with substantially lower loneliness than daily surface-level interactions.

6.2 Why university is uniquely lonely

The loneliness of the early university years is not a personal failing. It is a structural feature of the transition. Several specific factors converge:

- **Loss of established community.** Family, school friends, teachers, coaches, neighbours — the web of relationships that provided automatic belonging for eighteen years is suddenly loosened. Even if those relationships continue, they are no longer daily and ambient.
- **New social environment without scripts.** University social life lacks the structured, repeated contact that school provided. There is no single classroom where everyone sees everyone else daily. Friendships have to be actively built, often from scratch, in environments designed for transience (large lectures, shared residences, short-term group projects).
- **Comparison culture.** Students see curated versions of each other's lives on social media and conclude that everyone else is having the time of their life. This makes loneliness feel like a personal failure rather than a near-universal experience of the transition.
- **Identity transition.** The self you were in your hometown may not fit naturally in your new environment. Finding people who see and value the new self you are becoming takes time.
- **Practical barriers.** Money, transport, scheduling, and the awkwardness of initiating social contact with strangers all make the mechanics of building connection harder than they look.

The result is that loneliness in the first term of university is closer to the norm than the exception. The Healthy Minds Study finds that more than half of students report moderate to severe loneliness, with peaks in the first year and during examination periods. The loneliness typically eases over the course of the first year as friendships solidify, but for some students it persists and becomes a major contributor to depression and anxiety.

6.3 How connection is actually built

The research on how adults form friendships is surprisingly consistent, and the principles apply directly to students. **Repeated, unplanned interaction** is the single most important factor. We form friendships with people we see regularly, in contexts where interaction is natural and low-stakes. The classic university examples are residence halls, part-time jobs, recurring student clubs, lab groups, sports teams, and seminar classes. The student who is lonely should look first at where they have repeated, unplanned interaction in their week — and if the answer is "nowhere", that is the leverage point.

Shared activity is the second factor. Friendships formed around a shared activity — a sport, a creative project, a cause, an academic interest — form faster and more robustly than friendships formed through conversation alone. The activity provides a script, a purpose, and a reason to meet again. The student who joins one club and attends consistently for a term will, almost without trying, end up with a social network in a way that the student who "tries to make friends" in lectures will not.

Vulnerability is the third factor, and the one most students find hardest. Surface-level conversation — classes, weather, sports results — can be sustained indefinitely without producing closeness. Closeness requires the gradual exchange of more personal material: worries, hopes, family stories, embarrassing moments, opinions that might not be shared. The exchange does not need to be dramatic; it needs to be slightly more honest than the social norm. A small disclosure from you invites a small disclosure from the other person, and over weeks and months the relationship deepens. Students who never disclose — out of shyness, fear of judgment, or perfectionism — often find their friendships stuck at the pleasant-but-shallow level indefinitely.

> THE THREE CONDITIONS FOR FRIENDSHIP
>
> 1. Repeated, unplanned interaction (a context where you see the same people regularly without having to engineer it)
> 2. Shared activity that gives the interaction a purpose
> 3. Gradual vulnerability that moves the conversation below the surface
>
> If your current social life is missing one of these, that is your leverage point. The most common missing factor in lonely students is the first — they have no context of repeated, unplanned interaction. Joining one recurring activity fixes this within weeks.

6.4 The first-year social myth

There is a widespread belief among students that "freshman year friendships are forever" and that if you have not found your people by the end of the first term, you have missed your chance. This is false, but it is corrosive. It creates a self-fulfilling prophecy in which students who have not yet found close friends conclude that something is wrong with them, withdraw, and thereby reduce the very social contact that would solve the problem.

The actual pattern is much more gradual. Most students make their first round of friends in the first few weeks — often based on proximity (roommates, hallmates, orientation groups) rather than fit. These friendships often thin out over the first year as students discover who they actually click with. Deeper, more durable friendships often form in the second and third years, around shared majors, clubs, off-campus housing, and post-partying life. The student who feels at the end of first year that they have not yet found "their people" is, in most cases, perfectly on schedule.

The useful reframe: **your social life at university is a multi-year project, not a one-week orientation event**. Keep showing up to contexts where you might meet your people. Keep being slightly more honest than is comfortable. Keep being patient with the gap between when you arrive and when you feel you belong.

6.5 Maintaining friendships over distance

For many students, the relationships that matter most are the ones they left behind — family, childhood friends, partners. Maintaining these relationships over distance takes deliberate effort, because the ambient contact that sustained them is gone. The following practices are not glamorous but they work:

- **Schedule recurring contact.** A weekly Sunday phone call with a parent, a fortnightly video call with a school friend, a daily good-morning text to a partner. Recurrence is more important than duration; ten minutes weekly beats two hours monthly.
- **Use voice and video, not just text.** Text communication loses tone, facial expression, and the rhythm of conversation. Voice notes are a useful middle ground — more personal than text, less synchronous than a call.
- **Share small daily details, not just major news.** Closeness is built from the texture of daily life, not only from the headlines. A photo of your lunch, a complaint about a professor, a small win — these are the material of intimacy.

- **Be honest about how you are doing.** The temptation to perform "everything is great" for people back home is strong, and it cuts them off from the parts of your life that most need their support. Honest updates — including the hard parts — sustain closeness across distance.
- **Visit when you can.** In-person contact, even occasional, recharges the relationship in ways that digital contact cannot. Prioritise visits where possible.
- **Let some friendships change shape.** Not every friendship from before university will survive the transition, and that is okay. Some friendships become occasional-catch-up relationships rather than daily-contact relationships, and that is a legitimate form of friendship, not a failure.

6.6 Roommate and housemate relationships

For residential students, the roommate or housemate relationship is one of the most consequential relationships of the university years — for better or worse. A good roommate can be a primary source of support, structure, and belonging. A difficult one can be a chronic source of stress that disrupts sleep, study, and mood. Most roommate relationships are somewhere in between, and almost all benefit from explicit communication about expectations.

The most common sources of roommate conflict are: cleanliness standards, sleep schedules, noise, guests (especially romantic partners), shared belongings, and food. The most effective prevention is an explicit conversation, early in the arrangement, about expectations in each of these areas. Many residence-life programmes provide conversation guides for this. The conversation may feel awkward at the time, but it is far less awkward than months of unspoken resentment.

When conflict does arise, the most effective response is direct, specific, and non-accusatory communication. "When you played music at 2 a.m. last night, I couldn't sleep, and I'd appreciate it if you used headphones after midnight" is more useful than "You're so inconsiderate". If the conflict cannot be resolved directly, involving a residence advisor or mediator early is better than letting it escalate.

WHEN A ROOMMATE SITUATION IS HARMING YOUR MENTAL HEALTH

If a roommate relationship is producing sustained distress — sleep disruption, anxiety about returning to your room, fear, or any form of harassment — please contact your residence-life office or student services. You do not have to "just deal with it". Room changes are usually possible, and most institutions have processes for handling serious conflicts. Your living environment is a mental-health variable, not a test of endurance.

6.7 Romantic relationships

Romantic relationships are a substantial source of both support and stress for students. A healthy relationship provides emotional regulation, practical support, a sense of being known, and a context for growth. An unhealthy one can produce sustained anxiety, depression, sleep disruption, academic decline, and, in worst cases, abuse. The fact that a relationship exists does not mean it is good for either party.

What follows is not relationship advice in the broad sense — that would require its own book — but a few mental-health-relevant principles:

- **A good relationship makes both people bigger, not smaller.** If your social world, ambitions, friendships, and sense of self are shrinking around a relationship, that is a warning sign regardless of how "in love" you feel.
- **Conflict is normal; contempt is not.** The researcher John Gottman identified contempt — eye-rolling, mockery, disgust directed at the partner — as the single strongest predictor of relationship failure. Healthy couples disagree; they do not demean.
- **Long-distance relationships can work, with structure.** Explicit expectations about communication frequency, visits, and the timeline for being in the same place again substantially reduce the anxiety of distance.
- **Breakups are a normal part of student life and a significant mental-health event.** The end of a relationship produces genuine grief, often disrupts sleep and appetite, and can trigger depressive episodes in vulnerable students. Treat a breakup as you would a major loss — with time, support, and reduced expectations of yourself for a few weeks.
- **Abuse is never your fault, and help is available.** If a partner is controlling, threatening, physically violent, sexually coercive, or emotionally abusive, please reach out to your campus safety office, a counsellor, or a domestic-

violence hotline (listed in Chapter 15). What you are experiencing is not a relationship problem; it is a safety problem.

6.8 Social support mapping

A useful exercise, adapted from social-work practice, is to map your social support network explicitly. The exercise takes fifteen minutes and often produces surprising insights about where your support is strong and where it is thin.

YOUR SOCIAL SUPPORT MAP

List the people in each of the following categories. Names only — no need to overthink. If a category is empty, leave it blank; the empty categories are the leverage points.

People I could call at 3 a.m. in a crisis:

People I would tell about a personal problem:

People I see or speak to weekly:

People I do regular activities with (study, sport, hobbies):

People who challenge me to grow:

People who feel safe and undemanding:

Mentors, advisors, or older people I trust:

Looking at your map: which categories are well-stocked, and which are thin? What is one small step you could take in the next two weeks to strengthen a thin category — joining a club, reaching out to an old friend, signing up for a mentoring programme, attending a religious or community gathering?

6.9 When social difficulty is more than shyness

For some students, social difficulty is not a matter of habit or opportunity but of underlying social anxiety that warrants professional support. **Social anxiety disorder**, characterised by persistent and disproportionate fear of social judgment, is one of the most common anxiety conditions in students and is highly treatable. The student who experiences physical symptoms (racing heart, blushing, shaking, dry mouth) in social situations, who avoids classes or events because of social fear, who ruminates for hours after a social interaction about what they said wrong, may be experiencing social anxiety rather than ordinary shyness. If this resonates, please see Chapter 13 — effective support is available, and it works.

Other students may be navigating social difficulty as a result of neurodivergence — autism spectrum, ADHD, or other conditions that affect social communication. For these students, the standard advice to "join a club and be more outgoing" may be unhelpful or even harmful. Finding neurodivergent-affirming social spaces, working with a therapist who understands neurodivergence, and adjusting expectations about what social connection looks like for you specifically can be far more productive than trying to fit a neurotypical mould. University disability services can help with assessment and accommodation.

> *"You do not need to be liked by everyone. You need to be known by a few. The first is exhausting. The second is sustaining."*

Chapter 7 turns to the academic side of student life — the structured demands that fill most of your waking hours, and the way you meet them.

CHAPTER 7

Academic Stress and Study-Life Balance

Academic work is the central project of student life, and the way you relate to it shapes your mental health as much as the work itself does.

SAFETY NOTE This chapter addresses study habits and academic stress in general. If academic stress is producing significant symptoms of depression or anxiety, or if you are considering leaving your programme for mental-health reasons, please speak with a counsellor and your academic advisor — decisions about your programme should be made with support, not in isolation.

Academic stress is the most universal student experience and one of the most powerful drivers of depression and anxiety. It is also one of the most paradoxical: the same academic work that gives student life meaning and direction can, when mishandled or when amplified by perfectionism and comparison, become the primary source of suffering. This chapter is about how to do academic work in a way that protects rather than undermines your mental health — not by working less, necessarily, but by working differently.

A useful starting reframe: **academic stress is not a sign that you are not tough enough**. It is a sign that you are attempting a demanding task under demanding conditions, with limited time, limited feedback, and high stakes. The students who appear to handle it effortlessly are, almost without exception, either managing it less well than they appear, drawing on more resources than are visible (family support, prior preparation, financial security), or running on coping strategies that will not last. The goal is not to be one of those students. The goal is to develop a sustainable relationship with academic work that you can carry into the longer career that follows.

7.1 Time management: hours versus energy

Most student time-management advice focuses on hours — how to find more of them, how to allocate them efficiently, how to schedule every block. This is helpful up to a point, but it misses a more important variable: **energy**. An hour of academic work done when you are rested, fed, and emotionally regulated produces two to three times the output of an hour done when you are exhausted, hungry, and anxious. The student who is "working" twelve hours a day but

producing four hours of quality output is not actually working twelve hours; they are working four and pretending for the other eight, while depleting the recovery they need to work well the next day.

The energy-aware approach to time management starts with identifying your personal peak cognitive windows — for most students, one or two periods of the day when attention, memory, and creative thinking are at their best. These windows are typically mid-morning and mid-to-late afternoon for morning types, and later in the day for evening types, but they vary considerably by individual. Once you know your windows, the principle is simple: **protect your peak windows for the most cognitively demanding work, and use off-peak windows for administrative tasks, email, reading, and recovery**. The single most common time-management mistake in students is spending peak cognitive windows on low-value tasks (email, social media, organising notes) and then trying to do demanding work in the evening when they are depleted.

> IDENTIFY YOUR PEAK WINDOW
>
> For one week, note the time of day when you find it easiest to focus on difficult material. Most students find one or two windows of two to three hours each. Protect those windows for deep work — phone off, notifications off, single task. Use the rest of the day for everything else.

7.2 Time-blocking and the Pomodoro technique

Two specific techniques have strong evidence and wide adoption among students who manage academic stress well. **Time-blocking** is the practice of assigning specific tasks to specific time blocks in advance, rather than working from a to-do list. The advantage is that it forces realistic estimation (you cannot schedule eight hours of work in a four-hour block) and reduces the cognitive load of constant task-switching. The disadvantage is that it can become rigid and produce stress when blocks are missed; the solution is to plan with significant buffer time and to treat the schedule as a hypothesis rather than a contract.

The **Pomodoro technique** is the practice of working in fixed short intervals — typically twenty-five minutes — separated by short breaks of five minutes, with a longer break every four intervals. The advantage is that it makes starting easier (twenty-five minutes is not intimidating), produces built-in recovery, and creates a sense of accomplishment that builds momentum. The disadvantage is that twenty-five minutes may be too short for deep creative or analytical work;

many students find that fifty-minute or ninety-minute blocks suit them better once they have built the habit of starting. Experiment and find what works for you.

7.3 Procrastination: not laziness, but anxiety

Procrastination is one of the most misunderstood student behaviours. The popular framing — that procrastinators are lazy, undisciplined, or unmotivated — is both inaccurate and unhelpful. The research evidence points to a different explanation: **procrastination is, in most cases, an emotion-regulation problem, not a time-management problem**. We procrastinate not because we do not care about the task, but because thinking about the task produces negative emotions — anxiety about failure, boredom, overwhelm, self-doubt — and avoiding the task provides short-term relief from those emotions. The cost (a worse outcome later) is temporally distant; the relief (not feeling bad right now) is immediate. The brain, particularly under stress, prefers immediate relief.

This reframe matters because the interventions that follow from it are different from those implied by the "laziness" framing. Telling a procrastinator to "just be more disciplined" is like telling an anxious person to "just calm down" — it identifies the right outcome but offers no useful mechanism. The interventions that actually work target the emotional resistance, not the discipline.

SPECIFIC ANTI-PROCRASTINATION PRACTICES

- **Make the first step absurdly small.** "Write the essay" is paralysing. "Open the document and write one sentence" is achievable. Once you have started, momentum usually carries you further. The hurdle is almost always the start, not the continuation.
- **Lower your standards for the first draft.** Perfectionism drives procrastination; the harder you try to make the first draft, the harder it is to start. Give yourself explicit permission to write badly at first. You can edit bad writing; you cannot edit no writing.
- **Name the emotion.** When you notice yourself avoiding a task, pause and ask: what am I feeling? Often the answer is fear — of failure, of judgment, of not being good enough. Naming the fear reduces its power, and opens the possibility of addressing it directly rather than running from it.
- **Use implementation intentions.** "I will study this afternoon" is vague and easily deferred. "I will sit down at my desk at 2 p.m. and read the first ten pages of chapter four" is specific and harder to escape. The formula is: *When [situation], I will [specific action]*.

- **Reduce friction.** If you procrastinate on going to the library, pack your bag the night before and put it by the door. If you procrastinate on opening your laptop, close all distracting tabs the night before. If you procrastinate on writing, leave the document open with a half-finished sentence from yesterday.
- **Forgive yourself for past procrastination.** Students who beat themselves up for procrastinating procrastinate more, not less — the self-criticism adds to the negative emotion that drives the avoidance in the first place. Acknowledge the pattern, gently redirect, and move on.

7.4 Perfectionism: when high standards become a vulnerability

Perfectionism was introduced in Chapter 2 as a specific risk factor for depression and anxiety. Here we look at how it operates in academic life and what to do about it. Perfectionism is not the same as having high standards or caring about quality. It is a specific combination of **unrealistically high standards**, **conditional self-worth** ("I am only valuable if I achieve at this level"), **harsh self-criticism** when standards are not met, and **fear of failure** that drives both overwork and avoidance.

The academic environment reinforces perfectionism at every turn. Grades are visible and rank-ordered. Competitive programmes select for it. Employers and graduate schools reward it. Parents and peers praise it. The result is that many high-achieving students arrive at university with perfectionism so deeply embedded that it feels like a personality trait rather than a coping strategy — and one that appears to be working, because it produces high grades. The cost — anxiety, procrastination, brittle self-esteem, exhaustion, eating disorders, and, eventually, burnout — is less visible but accumulates over years.

The evidence-based interventions for perfectionism target each of its components. **Standards** can be revised by setting explicit "good enough" criteria for each task: what would a competent, non-perfectionist version of this work look like? Aim for that, not for the impossible ideal. **Conditional self-worth** can be loosened by deliberately cultivating sources of value outside achievement — relationships, hobbies, character, contribution. **Self-criticism** can be replaced with self-compassion (see Chapter 10). **Fear of failure** can be reduced by exposure: deliberately attempting things you might fail at, in low-stakes contexts, to recalibrate the brain's threat response to failure.

THE 80/20 REFRAME

In most academic work, 80 percent of the value is produced by 20 percent of the effort — and the remaining 20 percent of the value requires 80 percent of the effort. The perfectionist student spends the last 80 percent polishing work that is already good enough, leaving no time or energy for the next task. The sustainable student identifies the 80-percent-quality point, stops there, and moves on. Your grades may differ slightly. Your mental health, your sustainability, and your capacity to take on more interesting work will differ enormously.

7.5 Exam anxiety

Exam anxiety is a specific form of performance anxiety that affects a large proportion of students and that, paradoxically, tends to lower the performance it threatens. The student who has studied thoroughly but "freezes" in the exam is not underprepared; they are overactivated. Their sympathetic nervous system has shifted into a fight-or-flight state that impairs the very cognitive functions — working memory, flexible retrieval, sustained attention — that the exam requires.

The interventions for exam anxiety fall into three categories. **Preparation** matters, but only up to a point — beyond a certain level of preparation, additional study does not reduce anxiety and may increase it. **Cognitive techniques** include reframing the physiological arousal (a racing heart is energy, not catastrophe), challenging catastrophic predictions ("I will fail and my life is over"), and using self-compassion to interrupt the self-critical inner voice. **Somatic techniques** include slow diaphragmatic breathing (which directly activates the parasympathetic nervous system), grounding exercises (the 5-4-3-2-1 sensory exercise, described in Chapter 10), and progressive muscle relaxation. **Behavioural techniques** include deliberate exposure to exam-like conditions (taking timed practice exams under realistic conditions) and developing a pre-exam routine that signals safety to the nervous system.

For students whose exam anxiety is severe enough to impair performance despite preparation, formal accommodations (extra time, a quiet room, breaks) may be available through disability services. Exam anxiety that rises to the level of a panic disorder is a treatable condition; please see Chapter 13 if this resonates.

7.6 Asking for extensions and accommodations

Many students endure significant academic stress because they are reluctant to ask for extensions or accommodations, even when they are entitled to them and even when asking would substantially improve their outcome. The reluctance is understandable — it feels like an admission of failure, a burden on the professor, a mark of being "less than" — but it is, in most cases, unnecessary. Most professors and institutions would far rather know early that you are struggling and adjust accordingly than discover at the end of the term that you have fallen behind irretrievably.

The mechanics of asking are simpler than students often imagine. A brief, factual email — "I have been dealing with a health issue this week and would like to request a 48-hour extension on the assignment due Friday. I have attached a doctor's note / I can provide documentation if needed. Thank you for your consideration." — is usually sufficient. Most institutions also have formal processes for medical or compassionate extensions, and most have disability accommodation processes for students with documented conditions (including mental-health conditions). Your university's student services or disability office can walk you through what is available.

> **A REFRAME WORTH INTERNALISING**
>
> Asking for an extension is not a sign of weakness. It is a sign that you understand your own capacity and are managing your work accordingly — which is a skill employers and graduate programmes actively value. The student who silently misses a deadline because they could not bring themselves to ask for help is not more impressive than the student who asks early and delivers good work slightly later.

7.7 When academic stress becomes a mental-health issue

Academic stress is normal; academic stress that produces sustained depression or anxiety symptoms is not. The line is crossed when the stress starts affecting sleep, appetite, mood, concentration, social functioning, or sense of self over weeks rather than days. If you notice the following patterns, please take them seriously:

- Inability to stop thinking about academic work, even during rest, social activities, or attempts to relax
- Sleep consistently disrupted by academic worry — either unable to fall asleep, or waking in the early hours with racing thoughts

- Persistent physical symptoms — headaches, stomach-aches, chest tightness, muscle tension — that track with academic demands
- Avoidance of academic work that has moved from occasional procrastination to a paralysing pattern
- Sense of dread about going to class, opening your laptop, or checking email
- Loss of interest in subjects or activities you previously enjoyed
- Thoughts of giving up, dropping out, or escaping that feel more urgent than considered
- Increased use of alcohol or other substances to manage academic stress

None of these mean you are weak or unfit for your programme. They mean the system is overloaded and needs recalibration — which may involve reducing load, seeking accommodation, getting professional support, or, in some cases, taking a leave of absence. These are legitimate options, not failures. Many students who take a reduced load or a leave return to complete their programmes with better outcomes than if they had pushed through.

7.8 Comparison with peers

Academic environments are comparison machines. You see other students' grades, other students' contributions in class, other students' opportunities, other students' social lives (curated, on social media). The natural conclusion — "everyone is doing better than me" — is almost always false, but it feels true because you are comparing your inside (your doubts, your struggles, your unfinished work) to other people's outside (their visible accomplishments, their composed presentations, their finished products).

The antidote to comparison is not to stop comparing — the brain does it automatically — but to compare *differently*. Compare to your past self, not to other people: am I making progress on the things that matter to me? Compare full contexts, not curated snapshots: the student who appears to be thriving may be struggling in ways you cannot see. Compare on dimensions you actually value, not on dimensions the environment happens to make visible. And, where possible, reduce the comparison input — unfollow social media accounts that produce comparison, limit time in environments that activate comparison, and spend more time with people who see you as a whole person rather than as a competitor.

> *"Compare yourself to who you were yesterday, not to who someone else is today. The first is motivating. The second is corrosive."*

7.9 Sustainable study habits: a summary

Bringing the pieces together, the student who manages academic stress sustainably tends to do the following:

- Protects their peak cognitive windows for deep work, and uses off-peak windows for everything else
- Uses some form of time-blocking or Pomodoro to structure work sessions
- Treats starting as the main problem, and makes first steps absurdly small
- Writes bad first drafts and edits them, rather than trying to write perfect first drafts
- Sets explicit "good enough" criteria and stops there
- Asks for extensions or accommodations early rather than late
- Builds in recovery — sleep, movement, social time — as a non-negotiable part of the schedule, not as what gets cut when work piles up
- Reduces comparison input where possible
- Treats academic stress that produces sustained symptoms as a mental-health issue, not a study-skills issue

Chapter 8 turns to a stressor that is often invisible in student mental-health discussions but that drives a large share of the actual distress: money, and the basic needs it secures.

CHAPTER 8

Financial Wellness and Basic Needs

Money is one of the most powerful and least discussed determinants of student mental health. Naming it clearly is the first step toward managing it.

SAFETY NOTE This chapter addresses financial stress in general terms and signposts sources of support. It does not provide financial advice, debt counselling, or benefit eligibility assessment. For specific guidance, please consult your institution's financial aid office or a qualified advisor.

Financial stress is one of the strongest predictors of depression, anxiety, and suicidal ideation in student populations — in many studies, stronger than academic stress or social isolation. It is also one of the most under-discussed. Money carries a particular shame in many cultures, and students who are struggling financially often hide it from peers, mentors, and even family. This hiding deepens isolation, prevents access to support, and turns a difficult situation into a private catastrophe. This chapter aims to put financial stress on the table as a normal, common, and addressable student experience — not by minimising it, but by clarifying what it is, why it matters, and what helps.

8.1 Why financial stress hits students hard

Student financial stress is structurally different from adult financial stress in several ways. First, students typically have **low or zero income** relative to their living costs, and what income they have is often irregular (part-time work, gig work, family contributions that vary month to month). Second, students face **large fixed costs** — tuition, rent, textbooks, transport — that cannot be reduced without major life changes. Third, students often have **limited financial literacy** and limited experience managing budgets independently, which makes the cognitive load of financial management higher than it will be later in life. Fourth, students are often **financially dependent on family** in ways that complicate autonomy — decisions about majors, careers, and even social life may be shaped by family financial expectations. Fifth, the **stakes feel existential**: dropping out, taking on debt, or failing to convert the degree into a job feel like life-defining failures rather than reversible setbacks.

The mental-health impact runs through several pathways. The **direct pathway** is the chronic activation of the stress response produced by financial precarity — the constant low-grade alertness of "will I have enough this month", which elevates cortisol, disrupts sleep, and produces anxiety. The **indirect pathway** is the way financial stress forces compromises on every other protective factor: students who work long hours to afford rent have less time for sleep, movement, social connection, and recovery; students who cannot afford nutritious food eat poorly and feel worse; students who cannot afford social activities withdraw and become lonely; students who cannot afford counselling do not get counselling. The **cognitive pathway** is "scarcity mindset" — the well-documented finding that financial stress consumes cognitive bandwidth, reducing capacity for planning, decision-making, and self-regulation. The student who seems to be making "bad decisions" about money, academics, or self-care may simply be operating with a depleted cognitive budget.

8.2 Food insecurity: a hidden crisis

Among the most under-recognised student stressors is **food insecurity** — limited or uncertain availability of nutritionally adequate and safe food. Surveys in the United States, Canada, the United Kingdom, Australia, and elsewhere consistently find that between 20 and 40 percent of students at four-year institutions experience some level of food insecurity in a given year. The figure is higher at community colleges and among first-generation students, students of colour, students with dependents, and LGBTQ+ students.

Food insecurity is associated with depression, anxiety, lower GPA, higher rates of dropout, and poorer sleep. It is not a "lifestyle" issue or a matter of being thrifty; it is a basic-needs failure with direct mental-health consequences. A student who is skipping meals to afford rent, who is eating the cheapest possible food regardless of nutritional value, or who is uncertain where the next meal will come from is operating under a level of physiological stress that makes every other task harder.

1 in 3

Students at four-year institutions experience some level of food insecurity in a given year. The figure is higher at community colleges and among first-generation students, students of colour, and student-parents.

8.3 What support exists (and how to find it)

Many students who are struggling financially are unaware of the support that exists on their campus and in their community. The following are commonly available, though specifics vary by institution and country:

- **Campus food pantries and basic-needs centres.** Many universities now operate food pantries, free meal programmes, or basic-needs centres that provide food, toiletries, and other essentials at no cost and with minimal paperwork. These are typically run by student affairs or student services.
- **Emergency aid funds.** Most institutions have emergency funds for students facing sudden financial crises (loss of housing, medical bills, family emergency). These are usually one-time grants, not loans, and do not require repayment. The application is typically short and decisions are fast.
- **Government benefits.** Depending on your country and circumstances, you may be eligible for food assistance (SNAP in the US, equivalent programmes elsewhere), housing support, healthcare subsidies, or income support. Eligibility rules for students are often complex; a benefits counsellor or basic-needs navigator can help.
- **Scholarships and bursaries.** Beyond the major scholarships awarded at admission, many institutions have smaller bursaries and emergency scholarships that are under-claimed. Your financial aid office can tell you what is available.
- **Fee waivers and payment plans.** If you cannot pay tuition or fees on time, contact the financial aid office before the deadline. Many institutions can defer payment, set up instalment plans, or waive late fees. Silence is the worst option; communication usually produces options.
- **Work-study and on-campus employment.** On-campus jobs often pay better than off-campus equivalents, have more flexible scheduling around classes, and may be reserved for students with financial need.
- **Childcare support.** For student-parents, many institutions have childcare subsidies, family housing, or partnerships with local childcare providers.
- **Basic-needs navigators.** Some institutions now have dedicated staff whose role is to help students identify and access the full range of basic-needs support. Even if your institution does not have a formal navigator, student services can usually direct you.

A REFRAME ON ASKING

The shame associated with accessing food pantries, emergency aid, or benefits is real, and it is the main reason students go without support they are entitled to. The reframe worth internalising: these programmes exist *because* student financial stress is common and structural, not because individual students have failed. Using them is not taking from others; it is using resources that were created for exactly your situation. If you are eligible, please use them. If you are not sure whether you are eligible, please ask.

8.4 Budgeting that reduces cognitive load

The goal of student budgeting is not to optimise every dollar; it is to reduce the cognitive load of constant financial decision-making. A budget that you actually use is far better than an ideal budget that you abandon after two weeks. The following principles, drawn from behavioural economics, make budgeting more sustainable:

- **Automate what you can.** Automated payments for rent, utilities, and savings eliminate decision-making and reduce the risk of late fees.
- **Use categories, not line items.** Three categories — fixed costs (rent, tuition, transport), food, and everything else — are usually enough. Detailed line-item budgets are abandoned quickly.
- **Track after the fact, not before.** Spending five minutes a week reviewing what you actually spent is more sustainable than planning every purchase in advance. Many banking apps now do this automatically.
- **Build a small buffer.** Even a tiny emergency fund — a month's rent, say — substantially reduces the cognitive load of financial precarity. Build it slowly; do not let the goal of an "ideal" emergency fund prevent you from starting.
- **Plan for irregular costs.** Textbooks at the start of term, travel home for holidays, deposits on new housing — these are predictable but irregular. Setting aside a small amount monthly for them prevents the shock when they arrive.
- **Be honest about what you can and cannot afford.** The most expensive financial decision many students make is trying to maintain a social life that exceeds their budget. Honest conversations with friends about what you can afford — and choosing friends who make those conversations easy — are a form of financial self-protection.

8.5 Work and study: balancing income and academic load

Many students work during term. For some, work is a financial necessity; for others, a way to gain experience or reduce debt. The relationship between work and academic outcomes is not linear — students who work a moderate amount (10 to 15 hours per week) often do as well academically as students who do not work at all, and may develop time-management skills that non-working students do not. Students who work more than 20 hours per week, however, tend to show lower grades, more stress, and higher rates of mental-health problems. The trade-off is not always avoidable — financial necessity may require more than 20 hours — but it is worth knowing where the threshold lies.

If you must work significant hours, the following can help protect your academic and mental health: prioritise on-campus or work-study jobs that accommodate your class schedule; choose jobs with predictable hours rather than gig work that can be called up at any time; build your class schedule around your work commitments rather than the reverse where possible; talk to your academic advisor about reducing your course load if necessary; and treat your sleep as non-negotiable, even if it means earning slightly less.

8.6 Debt and the future-self problem

For many students, taking on debt is unavoidable and is, in itself, a rational investment in future earnings and life options. The mental-health impact of student debt is not primarily about the absolute amount; it is about the *relationship* the student has with the debt — whether it feels like an investment in a manageable future or like an unpayable burden that will define the next twenty years.

Useful reframes: **debt is a tool, not a moral failing**. Student loans are typically structured to be repaid gradually after graduation, often with income-based repayment options that cap payments at a percentage of income. The presence of debt does not mean you have failed; it means you have accessed education through a mechanism that the system is designed around. **Focus on what you can control** — completing your programme, building employable skills, and making informed choices about further borrowing — rather than on the total debt figure, which can feel paralysing. **Talk to a financial aid counsellor** if you are struggling; they can often help with repayment planning, consolidation, or relief options you may not know about.

8.7 The shame around money

Money shame is one of the most powerful and least visible forces in student mental health. Students who grew up with less than their peers may feel embarrassed about what they cannot afford, where they live, what they eat, what they wear, and what they cannot participate in. Students who grew up with more may feel guilty about what they have, or pressure to maintain a lifestyle they cannot independently afford. Students who are financially dependent on families with expectations may feel that their choices are not their own. International students may be navigating exchange rates, family expectations, and visa restrictions that compound the stress.

The antidote to money shame is not disclosure for its own sake — you do not owe anyone the details of your finances. It is finding, in your life, one or two people with whom you can be honest about money. A trusted friend, a partner, a sibling, a mentor, a counsellor — someone who knows what your actual situation is and with whom you do not have to perform. The performance of financial normalcy is itself exhausting, and dropping it with even one person can substantially reduce the cognitive load.

> *"Money is not a personal failing or a personal virtue. It is a structural condition. Treating it as the first rather than the second is most of what financial wellness actually means."*

8.8 When financial stress is producing symptoms

If financial stress is producing sustained depression or anxiety symptoms — sleep disruption, persistent worry, panic attacks, hopelessness, intrusive thoughts — please treat it as the mental-health issue it is, not as a financial issue you should solve alone. University counselling services are familiar with financial stress and can help, both directly (with the emotional impact) and indirectly (by connecting you to financial support you may not know about). Please also see Chapter 13 for the broader picture of when and how to seek help.

Chapter 9 turns to a stressor that is genuinely new to your generation, and that interacts with almost every other factor in this guide: the digital environment.

CHAPTER 9

Digital Well-being

Your phone is not the enemy of your mental health. But your relationship with it almost certainly matters more than you think.

SAFETY NOTE This chapter addresses general digital well-being. If you are experiencing online harassment, stalking, or image-based abuse, please contact your campus safety office or a specialised helpline (see Chapter 15) — these are safety issues, not lifestyle issues.

No factor has reshaped student mental health in the last fifteen years as much as the arrival of the always-on smartphone and the social-media ecosystem it enables. This chapter takes a deliberately measured stance: the technology is not going away, and most of the moral panic about it is overblown, but the way it is designed and the way most students use it produces measurable effects on attention, sleep, mood, social comparison, and identity formation. The goal is not to abandon the tools — they are genuinely useful — but to use them on your terms rather than on the terms of the attention-extraction industry that built them.

9.1 What the evidence actually says

The research on screens and adolescent/young-adult mental health is large, contested, and often oversimplified in popular coverage. The honest summary is roughly as follows. **Total screen time** is a poor predictor of mental-health outcomes; the relationship is weak and disappears entirely when content and context are accounted for. **Active use** (messaging friends, creating content, participating in communities) is generally associated with neutral or positive mental-health outcomes. **Passive use** (endless scrolling of feeds, consuming content without interaction) is associated with negative outcomes. **Social comparison use** (looking at peers' curated lives and comparing to your own) is the most consistently negative pattern, particularly for young women. **Bedtime use** is consistently negative, through both light exposure (see Chapter 4) and emotional activation. **Use during academic work** consistently impairs performance, even when the user believes they are multitasking effectively.

The effect sizes are not catastrophic for most students, but they are real, and they compound. A student whose sleep is shortened by an hour of late-night scrolling, whose attention is fragmented by constant notifications, whose self-image is shaped by exposure to idealised peers, and whose social confidence is undermined by parasitic online relationships is accumulating a meaningful mental-health cost. The cost is rarely large enough to be the "cause" of depression or anxiety on its own, but it is large enough to shift the baseline.

9.2 The four big problems

1. ATTENTION FRAGMENTATION

The brain cannot truly multitask; it switches, and switching has a cost. Every notification, every check of the phone, every context shift between work and message and feed produces a small attentional residue that reduces the quality of the next task. The student who checks their phone every five minutes while studying is not studying — they are performing a low-quality version of studying interleaved with high-quality distraction. Research suggests that even the presence of a phone in view — not being used, just visible — reduces cognitive capacity, because the brain is devoting attention to suppressing the urge to check it.

2. SLEEP DISRUPTION

Covered in detail in Chapter 4. The two mechanisms — light exposure and emotional activation — combine to make bedtime phone use one of the most reliably disruptive habits in student life. The student who cannot sleep without their phone is, in many cases, not sleeping because of their phone.

3. SOCIAL COMPARISON

Social media platforms are comparison engines. They are designed to surface the most engaging content, which is typically the most idealised version of other people's lives — their best photos, their biggest achievements, their most curated moments. The brain has no built-in correction for this; it treats the curated feed as representative of the person's actual life, and concludes that everyone else is doing better than you. This is the "compare your inside to other people's outside" problem, amplified to industrial scale.

The comparison effect is strongest for the content most relevant to your life stage: peers' academic achievements, social lives, romantic relationships, body images, travel, and career moves. The students most vulnerable to comparison-driven distress are those whose self-esteem is already contingent on external

validation, those in competitive academic environments, and those in the early stages of major life transitions (first year, final year, post-graduation) when comparison feels most consequential.

4. PARASOCIAL SUBSTITUTION

A "parasocial relationship" is a one-sided relationship with a media figure — a YouTuber, a streamer, an influencer, a celebrity. These relationships can be genuinely meaningful and are not inherently problematic. But when they substitute for, rather than supplement, real-world relationships, they leave the user feeling both stimulated and lonely — the worst combination. The student who spends three hours a day with a favourite streamer and zero hours with real friends is filling time without filling the belonging need, which produces a particular kind of emptiness that is hard to name but easy to feel.

9.3 What helps: practical interventions

The evidence on digital-well-being interventions is mixed — many interventions produce small, short-lived effects because the underlying design of the platforms is unchanged. But the following practices have shown reasonably consistent benefits and are worth trying:

Move your phone charger out of the bedroom

The single most effective digital well-being intervention. Removes bedtime use, removes the morning scroll, removes the sleep-onset disruption. Use a separate alarm clock. Most students who try this for two weeks report meaningful sleep improvements.

Turn off non-essential notifications

Most apps default to notifying you about everything, because engagement is their business model. Turn off notifications for everything except messages from actual humans, calendar reminders, and alarms. You will check apps when you choose to, not when they summon you.

Use grey scale

Setting your phone screen to grey scale (in accessibility settings) removes the rewarding colour stimulation that makes apps addictive. Many users find that scrolling loses its pull within days. Try it for a week.

Curate your feeds aggressively

Unfollow accounts that produce comparison, anxiety, or self-loathing — even accounts you "like". Follow accounts that produce genuine interest, learning, or connection. The feed is shaped by your behaviour; you can teach it to serve you better.

Schedule offline blocks

Particular times of day — first hour after waking, the hour before bed, during meals with others, during deep work — are high-leverage times to be offline. Treat them as protected.

Use one screen at a time

Watching a film while scrolling your phone produces a worse experience of both. Single-tasking your leisure — giving the film, the book, the conversation your full attention — produces more rest and more pleasure than the divided version.

9.4 Social media specifically

Social media deserves specific attention because it is the category most consistently associated with student mental-health problems. The platforms differ — Instagram and TikTok are more comparison-driven; Twitter/X is more outrage-driven; messaging apps are more genuinely social; Reddit and Discord communities can be either, depending on use — but a few general principles apply across platforms.

Notice your mood before and after use. If a particular platform consistently leaves you feeling worse — more anxious, more envious, more hopeless, more irritated — that is information. The platform is not neutral for you, regardless of how neutral it is for others. You do not need a reason to reduce or leave a platform that is making you worse.

Distinguish active from passive use. Messaging a friend, commenting on a post you care about, sharing something you made — these are active uses and are generally fine. Scrolling a feed for forty minutes without interacting is

passive use and is generally the most damaging pattern. If you cannot avoid the passive use, consider removing the app from your phone and accessing it only on a desktop, where the friction naturally limits the use.

Be honest about why you are using it. Often, the reach for the phone is not about information or connection; it is about escaping an uncomfortable feeling — boredom, anxiety, loneliness, the difficulty of starting a task. The phone is a highly effective feeling-escape device, which is precisely why it is so hard to put down. Building other, less costly ways to manage those feelings — a short walk, a few breaths, a glass of water, a text to a friend — gives you options beyond the phone.

9.5 News, doomscrolling, and information overload

A particular digital pattern that has intensified in recent years is "doomscrolling" — the compulsive consumption of negative news, often late at night, often past the point of any informational value. The pattern is driven by the brain's threat-detection system: in uncertain times, monitoring for threats feels adaptive, even when the monitoring cannot meaningfully reduce the threat. The result is a state of sustained low-grade activation that disrupts sleep, elevates anxiety, and produces a sense of helplessness about the state of the world.

The antidote is not ignorance — staying informed matters — but **bounded information consumption**. Specific times of day, specific sources, specific duration. Avoiding news in the hour before bed. Avoiding breaking-news alerts that pull attention at all hours. Recognising that for almost any event of genuine consequence, you will hear about it through other channels even if you are not actively monitoring. The student who checks the news twice a day, for fifteen minutes each time, is as well-informed as the student who monitors it continuously, and substantially less anxious.

9.6 Online communities: genuine belonging or substitute?

Online communities — Discord servers, Reddit forums, Facebook groups, gaming clans — can be genuine sources of belonging, particularly for students whose offline options are limited (international students, students in isolated locations, students with niche interests, LGBTQ+ students in unsupportive environments, neurodivergent students). The relationships formed in these communities can be real, deep, and enduring. They are not inherently inferior to offline relationships.

That said, online communities have specific risks. They can become echo chambers that reinforce unhelpful beliefs (including, in some communities, beliefs that worsen depression or anxiety). They can expose users to harmful content, including pro-eating-disorder, pro-self-harm, or extremist material. They can substitute for offline relationships in ways that leave the user more isolated over time, not less. And they can be contexts for harassment, exploitation, and grooming, particularly of vulnerable users.

The principles for healthy online community use: be selective about which communities you join; pay attention to how they make you feel; maintain at least some offline relationships alongside online ones; be cautious about disclosing personal information; and if a community is producing harm rather than support, leave it. The internet is large; better communities exist.

> *"Your phone is a tool. Use it when it serves you. Put it down when it does not. The hard part is not the putting down — it is noticing when it has stopped serving you."*

Chapter 10 turns to a set of practices that have travelled from clinical and contemplative traditions into mainstream student life: mindfulness, self-compassion, and other coping skills that you can use in real time.

CHAPTER 10

Mindfulness, Self-Compassion, and Coping Skills

A toolkit of small, evidence-based practices you can use in real time — not as therapy, but as the everyday mental equivalent of stretching, drinking water, and taking deep breaths.

SAFETY NOTE The practices in this chapter are framed as self-care, not as therapy. They are not a substitute for professional support, and they are not appropriate as the only intervention for significant depression or anxiety. If you have a trauma history, please approach meditation practices with care — some can be activating rather than calming, and a trauma-informed therapist can help you find what works for you.

This chapter gathers a set of practices that the research literature consistently finds helpful for student mental health, and that have the advantage of being free, portable, and usable in real time. None of them is a cure. None of them replaces sleep, social connection, or professional support when needed. What they offer is a set of small tools for managing the everyday texture of stress — the moment of panic before a presentation, the rumination spiral at 2 a.m., the self-critical loop after a difficult conversation, the overwhelm of a stack of deadlines. Used regularly, they accumulate into a greater capacity to stay present with difficult experience rather than being swept away by it.

10.1 What mindfulness is (and is not)

Mindfulness, as defined by the researcher Jon Kabat-Zinn who did most to bring it into mainstream medicine, is "the awareness that arises from paying attention, on purpose, in the present moment, and non-judgmentally". It is not a religion, despite its Buddhist origins. It is not relaxation, though it can be relaxing. It is not emptying the mind of thoughts, which is impossible. It is not positive thinking, which is a different (and often less helpful) practice. It is the simple — though not easy — act of noticing what is happening right now, with acceptance rather than resistance.

The relevance to student mental health is direct. Much of the suffering in anxiety and depression is not in the present moment itself, which is often bearable, but in the brain's departure from it — into future worries ("what if I fail the exam?"), past ruminations ("why did I say that?"), or self-referential narratives ("I am a failure"). These departures are not just thoughts; they are full physiological events that activate the stress response, raise cortisol, and produce real changes in mood and body. The capacity to notice the departure and return to the present interrupts the spiral before it amplifies. This is a skill, not a trait, and it can be developed through practice.

10.2 Brief breathing practices

The breath is the most accessible lever for regulating the nervous system in real time. Unlike thoughts, which can be difficult to direct, the breath can be consciously controlled, and changes in breathing pattern produce rapid changes in heart rate, blood pressure, and autonomic nervous system activity. The following practices are simple, evidence-based, and effective for most students.

BOX BREATHING (4-4-4-4)

Used by military personnel and emergency responders to regulate arousal under pressure. Inhale for four counts, hold for four counts, exhale for four counts, hold for four counts. Repeat for two to five minutes. Useful before stressful events (exams, presentations, difficult conversations) and during acute anxiety.

PHYSIOLOGICAL SIGH (DOUBLE INHALE, LONG EXHALE)

The fastest evidence-based way to reduce acute anxiety. Two short inhales through the nose (the second one stacking on top of the first), followed by a long slow exhale through the mouth. Repeat three to five times. Works by opening collapsed alveoli in the lungs and slowing the heart rate via the vagus nerve. Useful in the moment of acute stress — before a presentation, during a panic attack, after receiving bad news.

4-7-8 BREATHING

Inhale through the nose for four counts, hold for seven counts, exhale through the mouth for eight counts. The long exhale activates the parasympathetic nervous system. Useful for falling asleep and for general anxiety reduction. Four cycles is usually enough; do not overdo it, as it can produce light-headedness.

THREE MINDFUL BREATHS

The shortest practice. Pause, take one full breath with attention on the sensation of the air entering and leaving the body, then two more. Takes less than thirty seconds. Useful as a transition between activities — between classes, between work and rest, between waking and reaching for the phone.

10.3 Grounding: the 5-4-3-2-1 exercise

When the mind is racing or dissociating, grounding exercises bring attention back to the body and the present moment. The 5-4-3-2-1 exercise is the most widely taught:

5-4-3-2-1 GROUNDING

Look around and name (silently or aloud):
5 things you can see — describe each briefly
4 things you can physically feel — the chair, your feet on the floor, the texture of your clothes
3 things you can hear — distant and nearby sounds
2 things you can smell — or two of your favourite smells
1 thing you can taste — or one thing you like about yourself

The exercise takes two to three minutes and reliably reduces acute anxiety by anchoring attention in sensory present-moment experience.

10.4 Body scan and progressive muscle relaxation

The **body scan** is a mindfulness practice in which you move attention slowly through the body, from feet to head or head to feet, noticing sensations in each region without trying to change them. It is particularly useful for students who live mostly in their heads and have lost contact with bodily sensation. The practice takes ten to twenty minutes and can be done lying down before sleep (in which case it often becomes a sleep aid) or sitting during a break. Many free guided versions are available online.

Progressive muscle relaxation (PMR) is a related practice in which you systematically tense and then release each muscle group in the body. It is particularly useful for students who carry physical tension — in the jaw, shoulders, stomach, or hands — that they may not even be aware of until they release it. PMR takes fifteen to twenty minutes and is often done in the evening. It is one of the most evidence-based relaxation practices for anxiety and insomnia.

10.5 Self-compassion practices

Self-compassion, introduced in Chapter 3 as a protective factor, can be deliberately cultivated through brief structured practices. The researcher Kristin Neff describes three core practices:

THE SELF-COMPASSION BREAK

A short practice for difficult moments. When you notice you are struggling — you have made a mistake, received a setback, or are feeling inadequate — pause and say to yourself (silently or aloud):

THE THREE PHRASES

1. **Mindfulness:** "This is a moment of suffering" (or "This is hard right now" — naming the difficulty without dramatising or minimising it).
2. **Common humanity:** "Suffering is part of life" (or "Other people feel this way too" — recognising that struggle is human, not a personal failure).
3. **Self-kindness:** "May I be kind to myself in this moment" (or "May I give myself the compassion I need" — offering yourself the warmth you would offer a friend).

The whole practice takes less than a minute. Used consistently, it interrupts the self-critical spiral and builds the capacity to respond to difficulty with care rather than attack.

THE FRIEND REFRAME

When you notice you are being harsh with yourself about a mistake or shortcoming, pause and ask: *what would I say to a good friend who came to me with this exact situation?* Then say that to yourself. Most students find that the words they would offer a friend are dramatically warmer and more reasonable than the words they are giving themselves. The point is not to lie to yourself or lower your standards; it is to apply the same compassion to yourself that you would extend to someone you care about.

SELF-COMPASSION JOURNALING

A brief evening practice: write down one thing that was difficult during the day, one way you struggled with it, and one kind thing you could say to yourself about it. Takes five minutes. Research shows that doing this daily for two weeks produces measurable increases in self-compassion and reductions in self-criticism.

10.6 Journaling

Journaling is one of the most evidence-based writing practices for mental health. The forms that work best for student distress are slightly different from the "dear diary" model and worth distinguishing:

EXPRESSIVE WRITING

Developed by the psychologist James Pennebaker. The practice: write continuously for fifteen to twenty minutes, for three or four consecutive days, about a difficult emotional experience — something you have not fully processed. The instruction is to write about your deepest thoughts and feelings,

without worrying about grammar, spelling, or what anyone else would think. The mechanism appears to be the construction of a coherent narrative around an experience that was previously held as fragmented emotional memory, which reduces its intrusive quality. The evidence base is substantial, with benefits shown for mood, immune function, and even academic performance.

GRATITUDE JOURNALING

Each evening, write down three things you are grateful for from the day. The items can be small (a good cup of coffee, a moment of sunlight, a kind word from a stranger). The mechanism is the redirection of attention toward what is going well, which counters the negativity bias that depression and anxiety produce. Benefits are shown with as little as two weeks of practice, though the effect is stronger when the practice is sustained. The gratitude should be genuine, not performed; if you cannot find three things, find one.

WORRY JOURNALING

A dedicated time and place for worries. Each evening (or whenever worries are interfering with sleep or focus), write down everything you are worried about. The act of externalising the worries reduces their hold on working memory. For each worry, note: is this something I can act on? If yes, what is one small step? If no, can I let it go for tonight? The practice converts amorphous anxiety into specific concerns, many of which turn out to be more manageable than they felt.

STREAM-OF-CONSCIOUSNESS JOURNALING

Write continuously for a set period (five to fifteen minutes) about whatever comes to mind, without editing or directing. Useful for clearing the mind, processing the day, and noticing patterns over time. Less structured than expressive writing, but more sustainable as a daily practice.

10.7 Cognitive reframing (lightly)

The cognitive-behavioural tradition offers a range of techniques for working with unhelpful thought patterns. This guide is not the place for a full CBT manual, but one technique — **cognitive reframing** — is portable enough to be worth introducing. The practice: when you notice a distressing thought, pause and ask four questions:

THE FOUR QUESTIONS

1. **Is this thought true?** (What is the evidence for and against it?)
2. **Is this thought helpful?** (Even if it were true, is dwelling on it helping me?)
3. **What would I say to a friend who had this thought?**
4. **Is there another way to see this?** (What alternative interpretation fits the same facts?)

The point is not to convince yourself that everything is fine. It is to loosen the grip of a single interpretation, which creates space for a more workable one. This is a self-care practice, not a substitute for the structured cognitive work that a therapist would guide.

10.8 Values clarification

Depression and anxiety often produce a narrowing of attention onto immediate threats and discomforts, at the cost of contact with what actually matters to you. Values clarification is a brief practice for reconnecting with the larger direction of your life. The exercise: take ten minutes and write down your answers to the following questions.

VALUES CLARIFICATION

1. What do you want your life to be about? (Not what you want to *do* — what you want to *stand for*.)

2. If you were at your eighty-year-old self's birthday party and people were speaking about you, what would you want them to say about how you lived?

3. What are three qualities you most admire in others?

4. When do you feel most like yourself? What are you doing, and who are you with?

5. What is one small way you could bring more of what matters to you into the next week?

The point of the exercise is not to produce a life plan. It is to remind yourself, in the middle of stress, that the stress is in service of something — and to check whether the way you are currently living is moving toward or away from that something. The answer is often surprising, and the surprise is the value of the exercise.

10.9 A note on meditation apps and programmes

Many students access mindfulness through apps — Headspace, Calm, Insight Timer, Smiling Mind, and others. These can be genuinely useful, particularly for beginners who want guided instruction. A few principles: **start short** (five to ten minutes is enough); **be consistent** (daily for ten minutes is far more useful than weekly for an hour); **do not expect dramatic results** (the benefits accumulate over weeks and months, not sessions); and **do not force it** if a particular app or style does not work for you (there are many approaches; find one that fits). Free alternatives, including the Smiling Mind app and the Palouse Mindfulness free MBSR course, are widely available and as evidence-based as the paid options.

10.10 When self-care practices are not enough

It is worth being honest about the limits of these practices. Mindfulness, self-compassion, journaling, and breathing are helpful for managing the everyday texture of student stress. They are not sufficient as the only intervention for significant depression or anxiety, for trauma-related conditions, for eating disorders, or for any condition in which professional support is warranted. The student who has been doing all the "right" self-care practices for months and is still struggling is not failing at self-care; they are experiencing something that self-care alone cannot address. Please see Chapter 13 for the broader picture.

It is also worth naming a specific risk: **self-care can become another thing to fail at**. The student who is already overwhelmed and who adds "thirty minutes of meditation, twenty minutes of journaling, and a gratitude practice" to their to-do list has not reduced their load; they have increased it. The practices in this chapter are tools, not obligations. Pick one or two that resonate, use them when they help, and drop the rest. The goal is a lighter life, not a more demanding one.

> *"Self-care is not a checklist. It is a relationship with yourself — one that, like any relationship, takes attention and kindness, not perfection."*

Chapter 11 turns to a predictable but underappreciated source of student distress: the transitions that punctuate university life, from first year to graduation and beyond.

CHAPTER 11

Transitions: Starting, Returning, and Finishing

Every transition — even a wanted one — is stressful. Knowing that the stress is normal, and that it has a typical shape, makes it easier to bear.

SAFETY NOTE This chapter addresses the ordinary stress of academic transitions. If a transition is producing sustained symptoms of depression or anxiety, or if you are returning from a medical or mental-health leave and need support with reintegration, please work with your counselling service and academic advisor.

Transitions are the punctuation of student life. They include the obvious ones — starting university, returning after a break, transferring institutions, graduating — and the less obvious ones: starting a new term, moving off campus, beginning a thesis, starting a placement or internship, studying abroad, returning from study abroad, finishing a long-term relationship, the end of a sport or club that has structured your time. Each of these transitions involves loss (of the previous structure, identity, or community) and gain (of the new one), and the brain handles the simultaneous loss and gain clumsily, often producing a dip in mood and a spike in anxiety even when the transition is entirely wanted.

This chapter walks through the major transition points of student life and the protective moves that, according to the research and clinical literature, make them easier.

11.1 The first-year transition

The transition into university is one of the largest transitions in modern life. It typically involves a new city, new housing, new social network, new academic expectations, new financial responsibilities, new food, new sleep schedule, and a new identity — all happening simultaneously, often within a single week. The brain, which handles novelty well in small doses and poorly in avalanches, is confronted with an avalanche. The first six to twelve weeks are, for most students, a period of measurable stress, elevated cortisol, disrupted sleep, and mood volatility — even when the student is enjoying the experience and would not want to be anywhere else.

The single most useful reframe for first-year students is that **the dip is normal**. The student who arrives excited and then, three weeks in, finds themselves overwhelmed, lonely, and questioning whether they made the right choice is not failing the transition; they are experiencing it. The dip typically bottoms out around weeks four to eight and gradually lifts over the rest of the first term as friendships form, routines establish, and the environment becomes familiar. Students who know this in advance weather the dip better than students who conclude, at week four, that they are uniquely unsuited to university.

PROTECTIVE MOVES FOR FIRST YEAR

- **Build structure early.** A regular wake time, a regular class attendance pattern, a regular meal time, a regular study block. Structure reduces cognitive load and supports sleep.
- **Join one recurring activity in the first two weeks.** Club, sport, society, volunteer group, part-time job. The specific activity matters less than the recurring contact with the same people.
- **Be honest with people back home** about how it is going. The temptation to perform "everything is great" is strong and isolating.
- **Limit major decisions in the first term.** Switching programmes, dropping out, ending a long-distance relationship, getting a tattoo — these are decisions better made after the transition dip has lifted, not during it.
- **Use orientation and first-year support programmes.** Most universities offer mentoring, first-year seminars, peer support, and academic skills programmes specifically designed to ease the transition. Use them.
- **Know where to seek help** before you need it. Find the counselling service, the academic advising office, the financial aid office, and the disability office in the first few weeks, when you do not need them. Knowing where they are makes it easier to go when you do.

11.2 Returning after a break or leave

Returning to university after a break — whether a summer, a year abroad, a medical or mental-health leave, or a withdrawal and re-entry — is its own transition, and often a harder one than the initial arrival. The student returning from a leave in particular is often navigating two transitions at once: the return to academic demands, and the re-engagement with whatever circumstances led to

the leave. The dip in mood and increase in anxiety that can accompany the return are not signs that the leave failed; they are signs that the return is, itself, a major transition.

For students returning from a mental-health leave specifically, the following can help: **have a written return plan** developed with the counselling service and academic advisor, including reduced load if appropriate, regular check-ins, and a clear set of warning signs that would prompt reaching out for support; **resume protective practices before the return** — sleep, movement, social contact, therapy appointments — so that they are already established when academic demands resume; **communicate with trusted people on campus** (an advisor, a friend, a residence advisor) about what you need and what they should look out for; **build in more recovery time than you think you need** in the first weeks; and **be patient** — the rhythm of academic life takes a few weeks to feel natural again, and that is normal.

11.3 Transferring institutions

Transferring to a new institution combines elements of the first-year transition (new environment, new social network) with elements of returning after a break (already-established academic identity and habits). Transfer students often report feeling "between" cohorts — too experienced for the first-year orientation, too new for the second-year cohort that has already formed social bonds. The protective moves are similar to those for first year: build structure, join recurring activities, be honest with people back home, and know where to seek help. In addition, transfer-specific support groups or mentoring programmes, where they exist, can substantially reduce the sense of being between cohorts.

11.4 Study abroad

Studying abroad is, for most students, a positive and developmental experience — but it is also a major transition with predictable stress points. The **arrival phase** involves culture shock, language adjustment, homesickness, and the logistics of a new city; the **middle phase** typically involves a peak of enjoyment and engagement as the environment becomes familiar; the **departure phase** can produce grief at leaving and anxiety about re-entry; and the **return home** often produces "reverse culture shock" — the disorienting experience of finding home less familiar than expected, and of finding that the people at home have not had the parallel experience you have had.

Protective moves: research and prepare for culture shock before you go; build a local social network early (clubs, classes, host family) rather than relying on calls home; limit calls home in the early weeks to force local engagement; document your experience (photos, journal) to support later integration; and prepare for re-entry before you return — anticipate that home will feel different, that you will have changed, and that the people at home may not fully understand. Many universities offer re-entry workshops for returning study-abroad students; if yours does, attend it.

11.5 International student transitions

International students navigate the standard student transitions plus additional layers: visa and immigration stress, language demands, cultural adjustment, financial complexity (often including exchange rates and family expectations across borders), homesickness that is harder to address with a quick visit home, and the experience of being visibly or invisibly "other" in a host culture. The mental-health impact is substantial — international students show higher rates of depression and anxiety than domestic students in most studies, and lower rates of formal help-seeking.

Protective moves specific to international students: connect with an international student office or association early; find compatriots (other students from your country) for shared cultural reference and language; build relationships with both compatriots and host-country students, rather than choosing one or the other; understand your visa conditions and where to seek advice if they are threatened; be aware that homesickness is normal and typically eases over the first year; and recognise that the experience of being "other" is a real stressor, not a personal failing to be powered through. Many universities have counselling services with specific expertise in international student issues; please use them.

11.6 The post-graduation transition

The transition out of university is, for many students, harder than the transition in — and far less prepared for. The structures that have organised life for three or four years (terms, courses, exams, social networks built around campus) disappear overnight. The next step — job, graduate programme, gap year, return home — is often uncertain. The social network disperses. The identity of "student", which has been central, is suddenly gone. The result, for many recent graduates, is a period of disorientation, anxiety, low mood, and identity confusion that can last for months.

The post-graduation transition is sometimes called "the post-graduation void" or "quarter-life crisis", and it is far more common than the cultural narrative acknowledges. The student who graduated with a good degree, has a job offer or a plan, and yet feels persistently lost or low in the months after graduation is not failing adult life; they are navigating a transition that the culture under-prepares them for. Protective moves: maintain contact with at least some of the university social network; build structure into the post-graduation weeks (a routine, a job search schedule, a fitness routine, a class); be patient with the absence of clear direction; avoid comparing your insides to other graduates' curated outsides; and recognise that the first year after graduation is a transition, not a verdict on the rest of your life.

> *"Transitions are not interruptions of your life. They are your life, in a different shape. Treat them with the respect you would give any major event."*

11.7 A general model for navigating transitions

Regardless of the specific transition, a few principles apply across the board:

- **Expect a dip.** Mood and energy typically dip for four to twelve weeks during a major transition. Knowing this is normal reduces the secondary anxiety of "why am I not fine?"
- **Keep anchors from before.** A friendship, a routine, a hobby, a piece of music, a familiar food. Anchors provide continuity while the new environment becomes familiar.
- **Build structure into the new.** Regular wake time, regular meals, regular activity. The structure does the work of stabilisation while the brain is processing novelty.
- **Resist the comparison narrative.** "Everyone else is fine" is almost always false. The students who appear to be handling the transition effortlessly are, in most cases, managing it less well than they appear.
- **Avoid major decisions during the dip.** Big calls — dropping out, ending relationships, changing programmes — are better made after the transition has settled, not during it.
- **Seek support earlier rather than later.** If the dip is deeper than expected, is not lifting after a few weeks, or is producing significant symptoms, please reach out. Transitions can activate underlying vulnerabilities, and early support is more effective than delayed support.

Chapter 12 turns to a factor that shapes every transition and every other chapter in this guide: the intersection of identity, culture, and mental health.

CHAPTER 12

Identity, Culture, and Mental Health

Your identity — race, gender, sexuality, disability, class, immigration status, religion — is not a side variable in your mental health. It shapes every other factor in this guide.

SAFETY NOTE This chapter addresses identity-based stress and minority stress in general terms. If you have experienced discrimination, harassment, or hate crimes on campus, please contact your campus equity office, student services, or campus safety. These are not just "stress" — they are events that institutions have a responsibility to address.

Mental health is not identity-neutral. The risk factors and protective factors covered in earlier chapters do not operate the same way for all students; they are shaped, often profoundly, by identity. A first-generation student's experience of academic stress is different from a legacy student's. A Black student's experience of campus social life is different from a white student's. A transgender student's experience of campus housing is different from a cisgender student's. A student with a visible disability navigates the campus differently from a student without one. These differences are not soft variables; they are concrete, daily, and they produce measurable effects on mental health.

This chapter is necessarily incomplete — a single chapter cannot do justice to the intersection of identity, culture, and mental health. What it can do is name the major patterns the research literature documents, validate the experiences of students whose identities are often unaddressed in mainstream mental-health discussions, and signpost where to find more specific support.

12.1 Minority stress: the concept

The concept of **minority stress**, developed by the researcher Ilan Meyer in the context of LGBTQ+ mental health, refers to the chronic excess stress experienced by members of stigmatised groups, independent of any single

discriminatory event. Minority stress has three components: **external stressors** (actual experiences of discrimination, violence, rejection); **expectation of stress** (the vigilance that comes from anticipating further discrimination); and **internalised stigma** (the absorption of negative cultural messages about one's group). The model has since been applied to racial and ethnic minorities, religious minorities, people with disabilities, and other marginalised groups, and is one of the most useful frameworks for understanding why students from marginalised groups show higher rates of depression and anxiety even in the absence of overt discrimination on a given day.

Minority stress is the weather, not the storm. It explains why students from marginalised groups can experience physiological activation — elevated cortisol, disrupted sleep, anxiety — that their peers do not, even on days when nothing obviously bad has happened. The constant low-level vigilance of "will I be misgendered", "will I be tokenised", "will I be followed in the shop", "will my accent be mocked" is itself a stressor, and it accumulates.

12.2 Race, ethnicity, and mental health

Students of colour, in predominantly white institutions and in many multicultural institutions, navigate a range of identity-based stressors that white students do not. These include overt racial discrimination (which remains common on university campuses, despite institutional commitments to the contrary); subtler microaggressions (being asked "where are you really from", being told you are "articulate", being confused with the only other student of your race in the class); the cognitive load of code-switching (adjusting speech, behaviour, hair, clothing, and even name pronunciation to fit dominant norms); the experience of tokenisation (being asked to speak for your race in seminars or on committees); stereotype threat (the well-documented performance decrement that occurs when students fear confirming negative stereotypes about their group); and the isolation of being one of very few students of colour in a programme or residence.

The mental-health impact is well-documented: students of colour show higher rates of depression and anxiety, lower rates of formal help-seeking, and higher rates of negative experiences when they do seek help (including misdiagnosis, cultural incompetence, and microaggressions from providers). The protective factors include: connection with same-race peers and mentors; culturally responsive counselling (many universities now have counsellors of colour or partnerships with community organisations); ethnic student associations and

cultural centres; involvement in faith communities where relevant; and the development of a strong positive racial identity, which buffers against the impact of external racism.

12.3 Gender and mental health

Gender shapes mental health in multiple ways. **Women** show higher rates of depression and anxiety than men across most studies, partly biological (hormonal factors, including menstrual cycle, contraception, pregnancy, and perimenopause), partly social (higher rates of sexual violence, harassment, unpaid care work, and economic precarity; higher exposure to appearance-based comparison; socialisation toward rumination rather than externalising). **Men** show lower rates of diagnosed depression and anxiety but higher rates of suicide, substance use, and externalising behaviours — a pattern that reflects, in part, socialisation away from emotional expression and help-seeking, and toward "toughing it out". The result is that men's mental-health problems are often underdiagnosed and undertreated, with serious consequences.

Transgender, non-binary, and gender-diverse students show substantially elevated rates of depression, anxiety, self-harm, and suicidal ideation — a disparity that is largely attributable to minority stress (discrimination, misgendering, lack of access to gender-affirming care, family rejection, hostile policy environments) rather than to being trans itself. The protective factors are well-documented: use of chosen name and pronouns, access to gender-affirming healthcare, support from family and peers, presence of trans-inclusive policies and facilities on campus, and connection with trans community. Many universities now have dedicated LGBTQ+ centres or services; please use them.

12.4 Sexuality and mental health

LGBTQ+ students, as a group, show higher rates of depression, anxiety, substance use, and suicidal ideation than heterosexual and cisgender peers — a disparity that, as with trans students, is largely attributable to minority stress rather than to sexual orientation itself. The protective factors are similar: family acceptance, peer support, presence of affirming communities and faith traditions, access to LGBTQ+competent healthcare, and institutional policies and services that are visibly inclusive. The single most important protective factor for many LGBTQ+ students is **having at least one trusted adult to whom they are out** — a parent, teacher, mentor, or counsellor. If you are not yet out to anyone, please consider reaching out to an LGBTQ+ helpline or campus service as a first step; you do not have to do it alone.

12.5 Disability and mental health

Students with disabilities — physical, sensory, cognitive, developmental, or psychiatric — navigate a campus environment that is often not designed for them, despite legal mandates for accessibility. The mental-health impact is substantial: chronic illness and disability are associated with higher rates of depression and anxiety (partly through the biological effects of some conditions, partly through the social and structural barriers students encounter); students with psychiatric disabilities often face compounded stigma; and students with invisible disabilities (including ADHD, autism, learning disabilities, and chronic illness) often face scepticism about the legitimacy of their needs.

Protective factors: **register with the disability office early**, even if you are not sure you need accommodations — having the relationship established makes it easier to access support when you do; **know your rights** under relevant disability law (the ADA in the US, the Equality Act in the UK, equivalent laws elsewhere); **find disability community** on campus or online, which can be a powerful counter to isolation; **work with clinicians who understand your specific condition**; and **do not internalise the message that needing accommodation is "less than"** — accommodation is a right, not a favour.

12.6 First-generation students

First-generation students — those whose parents did not attend university — face a specific set of stressors that are often invisible to continuing-generation peers and to the institution itself. These include **structural unfamiliarity** (not knowing the unwritten rules of academic life — how to talk to professors, what office hours are for, how to choose a major, what "networking" means, how to apply for internships); **financial precarity** (first-generation students are more likely to be working long hours, to be food insecure, and to lack family financial backup); **family dynamics** (families may be proud but unfamiliar with the demands of university life, may have expectations about contribution to the household, may not understand why their student is "still studying" rather than earning); **cultural mismatch** between home and university environments; and **imposter feelings** — the persistent sense of not belonging, of having got in by mistake, of being found out.

Protective factors: first-generation student programmes (many universities now have these); mentoring by first-generation faculty or upper-year students; explicit education about the unwritten rules; financial support targeted at first-generation students; and reframing the imposter feelings as a normal response to being in a new environment, not a verdict on your capacity. First-generation

students who graduate do so at rates comparable to continuing-generation students when they have access to appropriate support; the gap is in support, not in capacity.

12.7 Class and socioeconomic background

Class operates as a background variable that shapes almost every other factor in this guide. Students from low-income backgrounds face financial stress, food insecurity, work obligations, housing precarity, and the cognitive load of managing trade-offs that wealthier peers do not face. They also face subtle cultural mismatches: the assumption of having travelled, of having certain forms of cultural capital, of having family networks that include professionals, of having "backup options" if a programme does not work out. These mismatches produce a particular kind of exhaustion — the constant low-grade work of translating between the academic environment and the home environment — that is itself a stressor.

The protective factors for class-based stress overlap heavily with those for first-generation students, and include financial support, mentoring, community with other low-income students, and the reframing of class background as a source of resilience and perspective rather than a deficit. Many universities have specific programmes for low-income students; if yours does, please use it.

12.8 International students and acculturative stress

International students were addressed in Chapter 11 in the context of transitions. From an identity perspective, they also navigate **acculturative stress** — the stress of adapting to a new cultural environment while maintaining connection to the culture of origin. The patterns of adaptation vary: some students assimilate quickly into the host culture, sometimes at the cost of disconnection from their culture of origin; some remain primarily within their compatriot community, sometimes at the cost of slower language acquisition and host-culture integration; some integrate both, maintaining strong ties to both cultures; and some experience marginalisation in both. The integration pattern is generally associated with the best mental-health outcomes, but the right pattern for any individual student depends on their specific circumstances and preferences.

12.9 Religious and spiritual identity

For students whose religious or spiritual identity is central, that identity can be a substantial source of meaning, community, and resilience — and, in some contexts, a source of stress. The stress arises from several sources: **religious**

discrimination (which affects Muslim, Jewish, Hindu, Sikh, and other religious-minority students in different ways in different contexts); **conflicts between religious teachings and other aspects of identity** (particularly around sexuality and gender, where religious and LGBTQ+ identities can be experienced as in tension, sometimes producing profound internal conflict); **cultural mismatch between religious norms and campus norms** (around alcohol, dating, dress, food); and **spiritual crisis** — periods of doubt, questioning, or loss of faith that can be disorienting, particularly for students whose faith has been central to their identity and community.

Protective factors include: connection with a religious or spiritual community on or near campus; chaplaincy or multi-faith services, which most universities offer and which can provide support that is not exclusively religious; access to religiously literate mental-health providers (many counselling services can refer to faith-sensitive therapists); and, for students experiencing conflict between religious and other identities, support from communities that have navigated similar tensions (such as LGBTQ+ affirming faith communities, which exist within most major religious traditions).

12.10 Intersectionality: why single-axis framings fail

The categories above are not independent. A student who is Black, queer, first-generation, and low-income is not experiencing four separate stressors; they are experiencing a single, interlocking system of stressors that shape every aspect of their university life. The concept of **intersectionality**, developed by the legal scholar Kimberlé Crenshaw, names this reality: identity categories intersect in ways that produce experiences not captured by any single category. A Black woman's experience of campus is not the sum of "Black student" and "woman student"; it is its own specific experience, with its own specific stressors and protective factors. The same is true for every intersectional position.

The practical implication for this guide is that the recommendations here are starting points, not complete pictures. Each student's specific situation is shaped by their specific set of identities and contexts, and the most useful support often comes from communities and providers who share or are trained in those specific intersections. Please seek out such support where it exists, and please advocate for its existence where it does not.

12.11 Finding culturally responsive support

Not all counselling is equally helpful for all students. A counsellor who does not understand the role of family in your culture, the specific history of your community, the meaning of your religious practices, or the lived reality of your disability may, even with the best intentions, provide support that misses the mark. The following strategies can help you find support that fits:

- **Ask explicitly** when contacting a counselling service whether they have counsellors from your background or with specific experience working with your community. Most services will be transparent about this.
- **Use community-specific resources** where they exist: Black, Asian, Latine, LGBTQ+, Muslim, Jewish, and many other communities have mental-health organisations that provide or refer to culturally responsive care.
- **Consider online counselling** if local options are not a fit — services like Therapy for Black Girls, the Trevor Project, Identity LGBTQ+ support, and many culturally specific platforms now exist.
- **Use peer support** from students who share your identity, through student associations, mentoring programmes, or peer-support groups. Peer support is not a substitute for clinical care when needed, but it is a powerful protective factor in its own right.
- **Advocate for yourself** if a provider is not a fit. A first appointment that does not feel right is information, not a verdict. You are allowed to switch providers, ask for a referral, or seek care elsewhere.

> *"Your identity is not a complication to be worked around. It is the lens through which you experience everything else. Find support that sees the whole lens."*

Chapter 13 turns to one of the most important and most delayed decisions in student mental health: when and how to seek professional support.

CHAPTER 13

Recognising When Self-Care Is Not Enough

Self-care is necessary but not sufficient. Knowing when to add professional support is itself a protective skill — and one most students delay for far too long.

SAFETY NOTE This chapter helps you recognise when to seek professional support. It is not a diagnostic tool. If you are in immediate danger to yourself or someone else, please skip to Chapter 15 and contact emergency services or a crisis line now.

Most student mental-health struggles are manageable with the protective practices covered in earlier chapters — sleep, social connection, movement, study habits, and the coping skills of Chapter 10. But not all of them. Some struggles cross a line that self-care alone cannot address, and recognising that line is one of the most important skills in this guide. Students who recognise it early and seek appropriate support tend to recover faster, more completely, and with less disruption to their academic and social lives than students who delay. Students who delay often do so for understandable reasons — stigma, cost, uncertainty about what to expect, hope that things will improve on their own — but the delay usually makes the eventual recovery harder, not easier.

This chapter aims to demystify the process. It walks through how to tell when ordinary distress has crossed into something that warrants professional support, what "elevated screening risk" actually means, what to expect from a first counselling appointment, how to overcome the common barriers to seeking help, and how to integrate professional support with the self-care practices covered elsewhere in this guide.

13.1 The line between ordinary distress and something more

Distress is normal. Depression and anxiety, in the clinical sense, are not just "more distress" — they are patterns of distress that have become persistent, pervasive, and disabling. The following questions can help you tell the difference. None of them, alone, is diagnostic; but if several resonate, it is worth taking seriously.

A SELF-REFLECTION (NOT A DIAGNOSTIC TOOL)

Consider the last two weeks. Have you experienced the following?

1. Low mood, sadness, or emptiness on most days, for most of the day, for two weeks or more? ☐ Yes ☐ No
2. Loss of interest or pleasure in activities you usually enjoy, on most days? ☐ Yes ☐ No
3. Significant changes in sleep (much more or much less than usual) lasting two weeks or more? ☐ Yes ☐ No
4. Significant changes in appetite or weight? ☐ Yes ☐ No
5. Persistent fatigue or loss of energy, even after rest? ☐ Yes ☐ No
6. Difficulty concentrating, remembering, or making decisions that is affecting your daily life? ☐ Yes ☐ No
7. Persistent feelings of worthlessness, guilt, or self-blame? ☐ Yes ☐ No
8. Recurrent thoughts of death, dying, or not wanting to be here? ☐ Yes ☐ No *(If yes, please see Chapter 15 immediately.)*
9. Excessive worry or anxiety that is difficult to control, on most days, for six months or more? ☐ Yes ☐ No
10. Restlessness, feeling on edge, or difficulty relaxing? ☐ Yes ☐ No
11. Panic attacks — sudden episodes of intense fear with physical symptoms like racing heart, shortness of breath, dizziness? ☐ Yes ☐ No
12. Avoidance of situations because of anxiety (classes, social events, public spaces)? ☐ Yes ☐ No
13. Distress that is interfering with your ability to study, work, socialise, or care for yourself? ☐ Yes ☐ No
14. Increased use of alcohol, cannabis, or other substances to cope? ☐ Yes ☐ No
15. Withdrawal from friends, family, or activities that previously mattered to you? ☐ Yes ☐ No

If you answered "yes" to several of these — particularly if the experiences have lasted two weeks or more and are interfering with your daily life — please consider reaching out to a professional. This is not a diagnosis. It is a signal that talking to someone trained to help would be a good next step.

13.2 What "elevated screening risk" actually means

You may have encountered the phrase "elevated screening risk" through a screening questionnaire at your university health service, through an online screening tool, or in materials associated with this guide. The phrase sounds clinical, but it is important to understand what it does and does not mean.

A screening questionnaire — typically the PHQ-9 for depression or the GAD-7 for anxiety — asks about the frequency of symptoms over the past two weeks. The score provides a rough indication of the likelihood that you are currently experiencing a clinically significant condition. A score in the "elevated" range does **not** mean you have a diagnosis. It means that, statistically, a substantial proportion of people with that score do meet criteria for a diagnosis, and that follow-up with a qualified professional is recommended. It is the mental-health equivalent of a smoke alarm going off: worth investigating, not worth panicking about, and definitely not worth ignoring.

What elevated screening risk does *not* mean: that you are broken, that you must start treatment, that you are dangerous, that you will be labelled, that the information will go on your permanent record (it will not — health information is confidential), or that you have failed at self-care. It is information, and the appropriate response to the information is to talk to someone qualified to interpret it and to help you figure out what, if anything, to do next.

> A USEFUL REFRAME
>
> A screening score is not a verdict. It is a piece of data, like a temperature reading. A high temperature does not tell you what illness you have or what to do about it; it tells you that something is worth paying attention to. A high screening score does the same. The clinician you speak with will help you make sense of it in the context of your specific life.

13.3 What to expect from a first counselling appointment

One of the biggest barriers to seeking help is not knowing what will happen when you do. The reality, at most university counselling services, is more mundane and less intimidating than the fear. A first appointment — sometimes called an intake, an assessment, or an initial consultation — typically lasts thirty to sixty minutes and follows a roughly predictable structure.

The counsellor will ask about what brought you in. This is your chance to describe, in your own words, what has been going on. You do not need to have prepared a perfect narrative; you can simply describe your experiences, in

whatever order makes sense to you. The counsellor may ask clarifying questions — about when the difficulties started, what makes them better or worse, what you have tried so far, what your life looks like more broadly (sleep, social, academic, family, substance use). They may ask about your history — previous mental-health experiences, family history, significant life events. They may ask some standardised questions, often using the same screening tools you may have taken online. They may ask about safety — whether you have had thoughts of suicide or self-harm, and if so, whether you have any plans or intent. These questions are routine, not alarmist; please answer honestly.

The counsellor will then typically discuss with you what they are hearing, what options are available, and what they would recommend. Options may include: brief individual counselling at the service (often six to twelve sessions, free at most universities); referral to a longer-term or specialised service outside the university; group therapy or psychoeducational groups; psychiatric consultation (for medication evaluation, if relevant); referral to other campus services (academic advising, disability services, financial aid); or, if what you are experiencing is within the normal range of student distress, validation and discussion of self-care strategies.

You are not obligated to accept any particular recommendation. The appointment is a conversation, not a contract. You can ask questions, express preferences, request a different type of support, or take time to think about it. The counsellor's job is to help you figure out what would be most helpful for you, not to impose a plan.

13.4 How to make the first appointment

The mechanics of making an appointment vary by institution, but the general process is similar. Most university counselling services accept self-referral — you do not need a doctor's referral or a diagnosis to access them. You typically contact the service by phone, email, or online form, briefly describe that you would like to talk to someone, and they will offer you an initial appointment. Wait times vary; in periods of high demand, you may wait a week or two for an initial appointment, though most services have provisions for urgent same-day or next-day appointments if you are in crisis.

Many students find the act of making the appointment harder than the appointment itself. The following can help: **write down what you want to say** when you call or email, even if it is just "I have been struggling with my mental health and would like to make an appointment to talk to someone"; **ask a friend to sit with you or make the call for you** if you cannot face it alone; **use email or**

an online form if phone calls are too anxiety-provoking; and **do not minimise** what you are experiencing — saying "I am fine, I just want to talk" may result in a lower-priority appointment than saying "I have been feeling depressed and it is affecting my classes".

> IF THE FIRST APPOINTMENT IS NOT A FIT
>
> Not every counsellor is a fit for every student. The first appointment may feel unhelpful for many reasons — a mismatch of style, a lack of cultural understanding, a sense of not being heard. This is normal and not a verdict on you or on counselling in general. You are allowed to request a different counsellor, to ask for a different approach, or to seek a different service. Most counselling services expect this and will support you in finding the right fit. Please do not conclude from one unhelpful appointment that no support will help you.

13.5 Common barriers (and how to address them)

"I AM NOT BAD ENOUGH TO NEED HELP."

You do not need to be at a crisis point to benefit from support. Most counselling services expect to see students with a wide range of concerns, including ordinary stress, relationship difficulties, identity questions, and adjustment problems. If what you are experiencing is bothering you and you would like to talk to someone, that is sufficient reason to make an appointment. Waiting until you are "bad enough" usually means waiting until the problem is harder to address.

"I SHOULD BE ABLE TO HANDLE THIS ON MY OWN."

There is a cultural narrative, particularly strong in some communities and around masculinity, that seeking help is a sign of weakness. The evidence is the opposite. Students who seek appropriate support are using available resources to address a problem — which is the same skill involved in using a textbook to learn a subject, or seeing a doctor for a physical health concern. The narrative that mental health should be handled alone is itself a risk factor for worse outcomes.

"WHAT WILL GO ON MY RECORD?"

Health information, including mental-health information, is confidential. Counselling records are not part of your academic transcript. Employers and graduate programmes do not have access to them. The exceptions are narrow

— typically involving imminent risk to yourself or others, child or elder abuse, or court orders — and your counsellor would discuss any necessary disclosure with you. Confidentiality is a legal and ethical requirement, not a courtesy.

"I CANNOT AFFORD IT."

Most university counselling services are free to enrolled students. If you need longer-term or specialised care that the university cannot provide, the counselling service can usually refer you to low-cost community options, sliding-scale clinics, or services covered by your health insurance. If cost is a barrier, please say so; the people whose job it is to help you have encountered this before and have options.

"I AM FROM A CULTURE WHERE THIS IS NOT DONE."

Many cultures have strong norms against discussing personal problems with strangers, particularly mental-health problems. These norms are real and worth respecting. They are also, in many cases, contributing to higher rates of unaddressed mental-health problems in those communities. If formal counselling is not acceptable to you, please consider alternative forms of support: a religious leader, an elder, a family friend, a community organisation, a peer-support group, or a culturally specific mental-health service. The form of support matters less than the act of letting someone in.

"I TRIED IT BEFORE AND IT DID NOT HELP."

This is a real and discouraging experience, and it is more common than the counselling field often acknowledges. There are several reasons counselling may not have helped in the past: it was the wrong type of therapy for your concern; the therapist was not a fit; the timing was wrong; the number of sessions was too few; or your circumstances at the time made it difficult to engage. None of these mean that no support will help. Please consider trying again, ideally with a different type of therapy or a different provider. Many students who did not benefit from counselling the first time benefit substantially the second.

"I DO NOT HAVE TIME."

Time is a real constraint, and adding a weekly appointment to an already overloaded schedule is genuinely difficult. The counter-consideration is that untreated mental-health problems typically consume more time — through procrastination, lost productivity, missed classes, and the cognitive load of struggling — than the appointment would. A fifty-minute weekly appointment that allows you to function better in the rest of your week is a net time gain, not a loss.

13.6 Integrating professional support with self-care

Professional support and self-care are not alternatives; they are complements. The student who is in counselling and also sleeping adequately, moving regularly, maintaining social connection, and using the coping skills of Chapter 10 will typically progress faster than the student who is in counselling but neglecting the rest. Conversely, the student who is using self-care well but is still struggling will typically benefit from adding professional support rather than from "trying harder" at self-care.

The two also inform each other. The insights you gain in counselling — about your patterns, your history, your triggers — can sharpen the way you use self-care practices. The observations you make in self-care — what makes you better or worse, what you have tried, what has worked — can give your counsellor valuable material to work with. Please do not treat them as separate domains; treat them as two halves of a single project.

13.7 A note on medication

This guide does not provide medication information, and medication decisions are made with a clinician, not a book. What this guide can say is that psychiatric medication, where it is appropriate, is one tool among several, that it is often combined with therapy rather than replacing it, that decisions about starting, changing, or stopping medication should be made with a prescriber rather than independently, and that stigma about medication is widespread but largely unjustified. If a clinician recommends medication as part of your care, please consider the recommendation on its merits, ask questions, and make an informed decision — rather than dismissing it out of hand based on cultural or personal narratives about medication.

13.8 A final note: relapse is not failure

Mental-health recovery is rarely linear. Most students who recover from depression or anxiety experience periods of improvement followed by periods of setback, sometimes over months or years. A setback is not a failure of treatment or of self-care; it is part of the natural course of these conditions. The useful response to a setback is to recognise it early, reach out to your existing support, and adjust — not to conclude that "nothing works" and disengage. The students who recover most fully are typically not those who never relapse; they are those who learn to recognise early warning signs, respond to them, and return to the practices and supports that helped before.

> *"Asking for help is not the alternative to being strong. It is one of the ways strong people stay strong."*

Chapter 14 turns to the parallel question: what to do when it is not you who is struggling, but a friend.

CHAPTER 14

Supporting a Friend

When someone you care about is struggling, you want to help. The good news is that you can — often more than you think. The boundary to remember is that you are a friend, not a therapist.

SAFETY NOTE This chapter offers guidance on supporting a friend through ordinary distress. If a friend is in immediate danger to themselves or others, do not try to handle it alone — contact emergency services, campus safety, or a crisis line (see Chapter 15). A friend in crisis needs more than a friend can provide.

Friends are often the first to notice when a student is struggling — before the student themselves, before family, and long before any professional. The role of a friend in mental health is genuinely powerful: you are often the person who validates the experience, who reduces the isolation, who suggests seeking help, who stays connected when the student withdraws. Research on help-seeking consistently finds that the single most influential factor in a student's decision to seek professional support is the encouragement of a trusted friend. You matter more than you think.

That said, the role has limits. You are not trained to assess risk, to provide therapy, or to manage a crisis. Trying to play those roles — however lovingly — can harm both your friend and yourself. This chapter is about how to be a good friend in the spaces where friendship helps, and how to recognise and respect the boundaries where professional support is needed.

14.1 Noticing the signs

The signs that a friend is struggling are often subtle and accumulate gradually. They include:

- **Withdrawal** — pulling back from social activities, taking longer to reply to messages, skipping classes or commitments they previously attended.
- **Mood changes** — seeming flatter, more irritable, more anxious, or more hopeless than usual. A friend who used to laugh easily and now rarely does.
- **Behavioural changes** — changes in sleep (looking exhausted, sleeping at odd hours), appetite (weight loss or gain), substance use (drinking more, alone, or at unusual times), self-care (skipping showers, wearing the same clothes repeatedly).
- **Academic changes** — missing deadlines, skipping classes, dropping activities, talking about giving up or dropping out.
- **Conversation changes** — talking more about hopelessness, being a burden, feeling trapped; or, conversely, going quiet about how they are doing.
- **Social media changes** — posting noticeably more or less, posting darker content, posting about feeling hopeless or wanting to disappear.
- **Direct statements** — talking about death, dying, suicide, or "not being here". These should always be taken seriously (see below).

None of these signs, in isolation, means a friend is in crisis. But if several show up together and persist for more than a couple of weeks, it is worth reaching out. The act of reaching out — of showing that someone has noticed — is itself often a relief to the person struggling.

14.2 Starting the conversation

Many people avoid starting the conversation because they do not know what to say. The good news is that the specific words matter less than the act of showing up. The following structure, adapted from mental-health first aid training, can help:

A SIMPLE SCRIPT

1. Name what you have noticed, specifically and without judgment.
"Hey, I have noticed you have been quieter than usual the last couple of weeks, and you missed a few classes. I just wanted to check in."

2. Express care, not alarm.
"I am not trying to make a big deal of it. I just wanted you to know I have noticed, and I care about how you are doing."

3. Ask openly, and give them room to answer or not.
"How are you doing, actually? And it is okay if you do not want to talk about it right now."

4. Listen more than you talk. Do not rush to fix, advise, or minimise. Let them tell you what is going on, in their own words.

5. Ask what would be helpful. "Is there anything I can do that would help right now? Even just listening, or helping you find someone to talk to?"

6. Follow up. One conversation rarely resolves anything. The consistency of checking in — a text a few days later, an invitation to coffee next week — matters more than the eloquence of any single exchange.

14.3 What to say (and not say)

Certain responses, while well-intentioned, tend to shut down rather than open up conversation. The following contrast may be useful.

OFTEN UNHELPFUL	OFTEN MORE HELPFUL
"You have so much to live for."	"I am glad you are here. I care about you."
"Other people have it much worse."	"That sounds really hard. I am sorry you are going through it."
"You should just exercise / sleep more / think positive."	"Have you been able to do anything that helps, even a little?"
"I know exactly how you feel."	"I do not know exactly what this is like for you, but I want to try to understand."
"It will get better."	"I am here, and I will keep being here."
"You need to see a therapist."	"Have you thought about talking to someone at the counselling service? I could go with you if that would help."
"Promise me you will not do anything stupid."	"I am worried about you. Can we talk about what is going on, and whether you are safe right now?"

The general principle: **listening, validating, and staying present is almost always more helpful than advising, minimising, or fixing**. Your friend does not need you to have answers. They need to know they are not alone.

14.4 When to involve someone else

There are situations where being a good friend means involving someone with more training or authority than you have. These include:

- **Direct statements about suicide or self-harm.** If a friend talks about wanting to die, about a plan to harm themselves, or about feeling like a burden whose absence would be a relief, take it seriously. Ask directly: "Are you thinking about suicide?" (Research shows this does not increase risk; it reduces it, by opening the conversation.) If they say yes, or if you are seriously concerned, do not leave them alone. Contact a crisis line, campus safety, or emergency services. You can offer to be with them while they call, or to call on their behalf.
- **Severe withdrawal or self-neglect.** If a friend has stopped attending classes, stopped eating, stopped leaving their room, or is showing signs of severe self-neglect, the situation has moved beyond what friendship alone can address. Contact a residence advisor, the dean of students, or the counselling service.
- **Acute intoxication or substance overdose.** If a friend is in immediate medical danger from alcohol or other substances, call emergency services.

Do not wait to see if they "sleep it off".

- **Evidence of abuse or assault.** If a friend discloses abuse (current or past), assault, or harassment, believe them, support them in seeking appropriate help (campus safety,Title IX office, local police, sexual-assault service), and respect their decisions about what to do. Do not pressure them to report if they do not want to.
- **When you are out of your depth.** If you are unsure whether a situation is serious, it is okay to consult a residence advisor, counsellor, or helpline yourself, without naming the friend, to get advice on what to do.

> THE PROMISE OF CONFIDENTIALITY
>
> Friendship involves trust, and a friend who is struggling may share things they do not want shared. Take that trust seriously — but understand its limits. Do not promise blanket confidentiality in advance. If a friend asks you to promise not to tell anyone, a reasonable response is: "I will respect your privacy as much as I can, but if I am seriously worried about your safety, I might need to tell someone who can help. I would rather be honest with you about that now than break a promise later." A friend in genuine danger needs more than your silence; they need the help you cannot provide.

14.5 Your own well-being as a supporter

Supporting a friend who is struggling takes a real toll. Students who take on significant caring roles for friends in distress often show measurable increases in their own stress, sleep disruption, and academic difficulty. The role is worthwhile; it is also costly, and the cost needs to be acknowledged and managed.

Protective practices for supporters include: **maintaining your own life** — your studies, your sleep, your social connections, your recovery — not as selfishness but as the precondition for being able to help; **being clear with your friend about what you can and cannot do** — "I am always here to listen, but I cannot be your therapist, and there are people trained for that"; **seeking your own support** — talking to a counsellor about the impact of supporting a friend is a legitimate and common use of counselling services; **taking breaks** — you are allowed to step back, particularly if the situation is escalating or if your own well-being is suffering; and **recognising that you cannot save anyone** — you can support, accompany, encourage, and connect, but you cannot do the work of recovery for another person. Their recovery is theirs, not yours.

> *"You cannot be the only support a struggling friend has. The most loving thing you can do is help them build a wider net."*

Chapter 15 addresses the situation every other chapter has been carefully avoiding: what to do when there is a crisis.

CHAPTER 15

Crisis Awareness and Emergency Resources

This chapter is here for the moments when everything else in this guide is not enough. It does not teach you to be a crisis worker. It signposts where crisis workers can be found.

IF YOU ARE IN IMMEDIATE DANGER TO YOURSELF OR SOMEONE ELSE, PLEASE STOP READING THIS GUIDE AND CONTACT EMERGENCY SERVICES NOW. In most of Europe and many other regions, the emergency number is 112. In the US and Canada, it is 911. In the UK, 999. In Australia, 000. If you cannot find your local emergency number, contact your campus security office or any available helpline listed below.

A crisis, in the mental-health sense, is a situation in which a person's safety is at acute risk — usually through their own actions, sometimes through the actions of others. This guide is not a crisis-intervention manual. Crisis intervention is a specialised skill that requires training, and attempting it without training can harm both the person in crisis and the person trying to help. What this guide can do is help you recognise when a situation has crossed into crisis, signpost the resources that exist to respond, and offer some basic guidance on what to do — and what not to do — while waiting for help to arrive.

If you are reading this chapter because you are worried about yourself, please know that the experience you are having — however overwhelming, however hopeless it feels — is one that other people have had and survived, and that the right help can make a real difference. The fact that you are still reading is itself a sign that some part of you is reaching for help. Please honour that part by contacting one of the resources below, now, before you do anything else.

15.1 What counts as a crisis?

A mental-health crisis can take many forms. The following are situations in which immediate professional help is warranted:

- **Thoughts of suicide with a plan or intent.** Thinking about how you would do it, having the means to do it, intending to do it soon.
- **Self-harm that has caused or may cause serious injury.**
- **A suicide attempt.** If you have taken an overdose, injured yourself, or attempted suicide, please seek medical help immediately, even if you are having second thoughts. Some overdoses cause delayed damage that is not immediately apparent.
- **Acute psychosis** — hallucinations, delusions, or loss of contact with reality, particularly if the experience is frightening or involves commands to harm yourself or others.
- **Severe self-neglect** — not eating or drinking for days, not sleeping for days, inability to care for basic needs.
- **Acute intoxication with suicidal or violent ideation.**
- **Being a danger to others.**
- **Being the victim of abuse, assault, or exploitation** and needing immediate safety.
- **Being unable to keep yourself safe** for any other reason.

If any of these apply to you, please contact one of the resources below now. If any of these apply to a friend, please stay with them (in person or on the phone) and contact help on their behalf if they cannot or will not.

15.2 International crisis lines

Crisis lines are staffed by trained volunteers or professionals who can listen, support, assess risk, and connect you to local emergency services if needed. Most are free, confidential, and available 24 hours a day. The following list is not exhaustive but covers many countries. If your country is not listed, please search online for "crisis line [your country]" or contact your local emergency services.

COUNTRY / REGION	SERVICE	NUMBER / CONTACT
USA	988 Suicide & Crisis Lifeline	988 (call or text) · 988lifeline.org
USA	Crisis Text Line	Text HOME to 741741

Canada	9-8-8 Suicide Crisis Helpline	988 (call or text)
UK & Ireland	Samaritans	116 123 (24/7)
UK	Shout (text)	Text SHOUT to 85258
Australia	Lifeline Australia	13 11 14
Australia	Beyond Blue	1300 22 4636
European Union	European emergency number	112
France	Suicide Écoute	01 45 39 40 00
France	3114 (national suicide line)	3114
Germany	Telefonseelsorge	0800 111 0 111 or 0800 111 0 222
Spain	Teléfono de la Esperanza	717 003 717
Italy	Telefono Azzurro	19696
Netherlands	113 Zelfmoordpreventie	113 or 0800 0113
Belgium	Centre de Prévention du Suicide	0800 32 123
Switzerland	Die Dargebotene Hand	143
Sweden	Mind Självmordslinjen	90101
Japan	Tell Lifeline	03 5774 5591
India	iCall (Tata Institute)	9152987821
South Africa	SADAG Suicide Crisis Line	0800 567 567
Brazil	Centro de Valorização da Vida	188
Mexico	SAPTEL	55 5259-8121
International	Find A Helpline (directory)	findahelpline.com
LGBTQ+ youth (US)	The Trevor Project	1 866 488 7386 · thetrevorproject.org
LGBTQ+ youth (international)	Trevor Project global directory	thetrevorproject.org/get-help
Trans Lifeline (US)	Trans Lifeline	1 877 565 8860
Trans Lifeline (Canada)	Trans Lifeline Canada	1 877 330 6366

Sexual assault (US)	RAINN National Sexual Assault Hotline	1 800 656 4673
Domestic violence (US)	National Domestic Violence Hotline	1 800 799 7233
Eating disorders (US)	NEDA Helpline	1 800 931 2237

Please verify the current number for your region before relying on this list — services occasionally change. If you cannot reach a service, try your local emergency number or your university's campus security.

15.3 What to do while waiting for help

If you or someone else is in crisis and you have called for help, the following practices can help keep the situation as safe as possible while you wait.

IF YOU ARE IN CRISIS YOURSELF

- **Put distance between yourself and any means of harm.** If you are thinking about overdose, move medications out of reach. If you are thinking about a weapon, move away from it. The few seconds of friction often allow the crisis to pass.
- **Do not be alone.** If possible, go to a public place, call a friend, or stay on the phone with a crisis line until help arrives.
- **Avoid alcohol and other substances.** They lower inhibitions and increase impulsivity, which is the opposite of what you need right now.
- **Do something physical and grounding.** A cold shower, a walk outside, holding an ice cube, the breathing practices from Chapter 10. Anything that brings you back into your body and the present moment.
- **Remember that this state will pass.** Acute suicidal crises are typically time-limited — the intense urge often lifts within hours, even when the underlying distress persists. Your job right now is to stay safe through the peak of the wave.

IF YOU ARE WITH A FRIEND IN CRISIS

- **Stay with them.** Do not leave them alone. If you cannot be physically present, stay on the phone.
- **Remove means of harm** if it is safe to do so — medications, weapons, ligature points.
- **Listen without judging, arguing, or minimising.** Do not debate whether their life is worth living. Do not tell them to think of their family. Do say: "I am here. I am not leaving. Help is coming."

- **Ask directly about suicide.** "Are you thinking about killing yourself?" Asking does not increase risk; it often reduces it by opening the conversation.
- **Call for help.** Crisis line, emergency services, campus security. You do not need their permission if they are at imminent risk.
- **Do not promise confidentiality.** A life is more important than a promise.
- **Take care of yourself afterwards.** Being with someone in crisis is traumatic. Please seek your own support — a friend, a counsellor, a crisis line for supporters.

15.4 What happens when you call emergency services

Many students hesitate to call emergency services because they do not know what will happen. The reality varies by country and circumstance, but the general pattern is: a trained dispatcher will ask you questions to assess the situation; they may send an ambulance, police, or a crisis-response team; the responding professionals will assess the person in crisis and decide on the appropriate next step, which may include transport to an emergency department, a crisis assessment, or referral to community services. The goal is safety and stabilisation, not punishment or permanent institutionalisation.

In most jurisdictions, you cannot be forced into long-term treatment against your will unless you are assessed as lacking capacity to make decisions about your own safety at that moment. Short-term holds (typically 24 to 72 hours) for assessment are possible in some jurisdictions if there is imminent risk. The threshold for these is high and the process is reviewed. The cultural image of "being locked up for being suicidal" is largely outdated and exaggerated; modern crisis care emphasises the least restrictive intervention that keeps you safe.

15.5 After a crisis

A mental-health crisis is not the end of recovery; it is often the beginning of a more serious engagement with it. The period immediately after a crisis is a critical time for connecting with appropriate support, addressing the underlying factors, and building a plan to reduce the likelihood of future crises. The following are typically helpful:

- **Follow up with professional support quickly.** Within days, not weeks. Many crisis services offer follow-up appointments or connections to ongoing care.
- **Inform trusted people.** A friend, family member, partner, academic advisor, or residence advisor who can check in on you in the following weeks.

- **Reduce immediate stressors where possible.** Extension on assignments, time off work, reduced course load, time away from a difficult living situation.
- **Build a written safety plan** with a clinician (see Chapter 16 for a template). A safety plan is a personal document that lists your warning signs, your coping strategies, your support people, and your crisis contacts. Having it written in advance makes it easier to use when you need it.
- **Remove or secure means of harm.** If you have medications, weapons, or other means in your environment, ask a trusted person to hold them temporarily or to help you secure them.
- **Be patient with yourself.** Recovery from a crisis is not linear. The first weeks may include relief, shame, gratitude, fear, and exhaustion, often in rapid succession. None of these are wrong; they are all part of the process.

> **IF SOMEONE YOU KNOW HAS DIED BY SUICIDE**
>
> The death of a peer by suicide is a devastating event that can produce complex grief, trauma responses, and elevated suicide risk in those affected. If you have lost someone close to you, please seek support — from a counsellor, a grief support group, or a crisis line. Universities typically have postvention services for affected students after a student death; please use them. The grief you are experiencing is real and deserves support, not silence.

> *"A crisis is not the end of your story. It is the moment when the support around you becomes most important. Please let it in."*

Chapter 16 closes the main content of the guide with a workbook for building your own well-being plan — a personal, practical document that brings together everything you have read.

CHAPTER 16

Building Your Personal Well-being Plan

A guide is only useful if it changes what you do. This final chapter is a workbook for translating what you have read into a personal, practical plan.

SAFETY NOTE This workbook is a tool for general well-being planning. It is not a treatment plan. If you are currently experiencing significant depression or anxiety, or if you have a history of crisis, please complete this workbook with a counsellor or trusted support person rather than alone.

The previous chapters have laid out a substantial body of information — risk factors, protective factors, practices, and resources. Information, by itself, does not change lives. What changes lives is the translation of information into specific, personal, repeated actions. This chapter is designed to support that translation. It is a workbook, not a quiz. There are no right answers. The goal is to produce a document that is genuinely yours — one that you could come back to in six months and that would still make sense as a description of what protects you and what you are working toward.

Please consider completing this workbook with a friend, partner, mentor, or counsellor rather than entirely alone. The act of talking through your answers with another person often produces clearer and more honest responses than solitary reflection, and it builds the accountability that makes it more likely you will return to the plan.

16.1 Your current snapshot

Before planning forward, take stock of where you are now. Answer the following as honestly as you can, with reference to the last two weeks.

CURRENT SNAPSHOT

Sleep (average hours per night, and quality):

Movement (frequency and type in the last week):

Social connection (genuinely connecting conversations in the last week):

Eating (regularity and quality):

Screen use (estimated hours per day, including phone, laptop, TV):

Substance use (alcohol, cannabis, other — frequency and quantity):

Mood (general pattern over the last two weeks):

Anxiety (general pattern over the last two weeks):

Academic engagement (attending classes, keeping up with work, engagement level):

One thing that is going well:

One thing that is bothering you:

16.2 Your protective factor inventory (revisited)

In Chapter 3, you completed a protective factor inventory. If you did not, please return to that exercise now. If you did, please review your answers and update them based on what you have read since.

The goal of revisiting the inventory is to identify the two or three protective factors you would most like to strengthen over the next month. Not the entire list — two or three. Trying to change everything at once is a recipe for failure; changing two or three things, deliberately and consistently, is a recipe for actual change.

MY FOCUS FOR THE NEXT MONTH

Two or three protective factors I want to strengthen:

1. _____
2. _____
3. _____

For each, one specific small action I will take in the next week:

1. _____
2. _____
3. _____

How I will know if it is working (what will I notice?):

Who will I tell about this plan (for accountability)?:

When will I review how it is going?:

16.3 The 7-day well-being reset

If your current snapshot suggests that several protective factors have slipped at once — you are sleeping poorly, not moving, isolated, eating badly, screen-bound — a structured short reset can help. The reset is not a "lifestyle overhaul"; it is a deliberate return to baseline for one week, after which you can decide

what to keep. The reset is most useful when you have been slipping for a few weeks rather than months; if your pattern has been entrenched for longer, please consider working with a counsellor rather than attempting a self-reset.

7-DAY WELL-BEING RESET

Day 1 — Sleep: Set a consistent wake time for the next 7 days. Get outdoor light within 30 minutes of waking. No screens in bed tonight.

Day 2 — Movement: Add a 20-minute walk (ideally outside). Continue sleep practices from Day 1.

Day 3 — Connection: Send a message to one person you have been meaning to reach out to. Have one genuine conversation (in person, phone, or video) today.

Day 4 — Nutrition: Eat three actual meals today. Drink water. Limit caffeine after 2 p.m.

Day 5 — Screens: Move your phone charger out of the bedroom. Set a screen cutoff one hour before bed.

6 — Reflection: Take 15 minutes to journal about how the week has felt. What has helped? What has been hard?

Day 7 — Planning: Decide which practices to keep, which to adjust, and which to drop. Write down your plan for the next week.

Throughout: Be kind to yourself. The goal is not perfection. The goal is to remind your body and mind what baseline feels like, so that you can choose what to maintain.

16.4 Your early warning signs

Most students, looking back on a period of depression or anxiety, can identify early warning signs that preceded the full episode — patterns that, in retrospect, were clear signals that things were slipping. The signs are individual; what counts as a warning sign for one student may be normal for another. The exercise below is to identify your own early warning signs, so that you can recognise them earlier next time and respond.

MY EARLY WARNING SIGNS

Looking back at past periods when your mental health has slipped, what were the first signs? Consider each category:

Sleep:

Mood:

Thinking:

Behaviour:

Social:

Physical:

Substance use:

When I notice these signs, my plan is to:

1. _____
2. _____
3. _____

Who I will tell:

16.5 Your staying-well plan

A staying-well plan (sometimes called a wellness recovery action plan, or WRAP) is a personal document that brings together your protective practices, your early warning signs, and your response plan. It is a document you write when you are well, to use when you are not. The structure below is adapted from the WRAP model.

MY STAYING-WELL PLAN

1. My daily maintenance plan — what I do on a normal day to stay well:

2. My weekly maintenance plan — what I do each week to stay well:

3. My triggers — situations, events, or periods that I know are hard for me:

4. My early warning signs — patterns that tell me I am starting to slip (from 16.4):

5. When things are breaking down — signs that I am in serious trouble:

6. My crisis plan — who to contact, what has helped in past crises:

7. My post-crisis plan — what I need in the weeks after a crisis to recover:

16.6 Your support map

This is a more detailed version of the social support map from Chapter 6, expanded to include professional and institutional support as well as personal relationships.

MY SUPPORT MAP

Personal — friends, family, partner:

Community — clubs, religious group, peer support, mentors:

Professional — counsellor, therapist, GP, psychiatrist (with contact info):

Institutional — academic advisor, disability office, financial aid, residence advisor:

Crisis — local emergency number, crisis line, campus security:

16.7 Small commitments

The literature on behaviour change is consistent: small, specific, repeatable commitments are far more effective than large, vague aspirations. The following prompts invite you to make a few small commitments to yourself. They are not contracts; they are experiments. Try them for two weeks, see what happens, and adjust.

MY SMALL COMMITMENTS (FOR THE NEXT TWO WEEKS)

One thing I will do for my sleep:

One thing I will do for my body:

One thing I will do for my social connection:

One thing I will do for my academic work:

One thing I will do for my digital well-being:

One thing I will do for my inner life (mindfulness, journaling, reflection):

One thing I will stop doing:

One thing I will ask for help with:

16.8 A note on imperfection

This workbook, like the rest of the guide, can become another source of "I am not doing enough" if you let it. Please do not let it. The plans you have written here are not a standard to measure yourself against; they are a sketch of what matters to you, made at a particular moment, to be revised as you learn more. Some of what you have written will work; some will not. Some you will keep; some you will drop. The value is in the act of paying attention — in treating your own well-being as something worth thinking about deliberately — not in producing a perfect plan.

If you have read this far, you have already done something substantial. You have taken the time to inform yourself about your own mental health, to reflect on what protects you and what puts you at risk, and to consider how to translate that reflection into action. That is the work. Everything else — the specific practices,

the timing, the inevitable lapses and revisions — is detail. Please be patient with yourself as you work through it. Please be honest with the people who can help you. Please ask for help when you need it, and accept it when it is offered. The point of all of this is not to be a perfect student of well-being; it is to live a life that is, on the whole, more sustainable, more connected, and more yours.

> *"The goal is not to optimise your mental health. The goal is to live, as well as you can, the life that is actually yours."*

The appendices that follow — the situations and vignettes, and the resource directory — are reference material to return to as needed. The guide ends there. The work continues.

APPENDIX A

Situations and Case Vignettes

Twelve anonymised, composite student situations. Each is fictional in detail but typical in pattern. Use them to recognise your own or a friend's experience, and to see how the practices in this guide might apply.

SAFETY NOTE These vignettes are composites, not real individuals. Any resemblance to specific persons is coincidental. They are illustrative, not diagnostic. If you recognise yourself in a vignette and the experience is causing distress, please see Chapter 13 or Chapter 15.

VIGNETTE 1 · FIRST-YEAR LONELINESS

"I came here from across the country, and I have not made a single real friend in six weeks."

Maria, 18, moved 3,000 miles from home for her first year. She is doing well academically — classes are interesting, grades are fine — but she eats most meals alone, has not been to any social events since orientation week, and spends her evenings on video calls with her high-school friends, who are increasingly busy with their own new lives. By week six, she is crying most nights and starting to wonder if she made a mistake coming here.

What is happening: Maria is in the transition dip described in Chapter 11. She has not yet found a context of repeated, unplanned interaction (Chapter 6), and her heavy reliance on calls home is substituting for, rather than supplementing, local connection.

What helped: Maria joined a weekly film society and a recurring volunteer shift at the campus food pantry — two contexts where she would see the same people weekly without having to engineer social interaction. She set a limit on calls home to twice a week to force local engagement. She spoke to a peer mentor through the first-year support programme. By week ten, she had two acquaintances she regularly sat with at meals; by week sixteen, she would describe one of them as a close friend.

Non-clinical takeaway: The loneliness of the first weeks is normal, not a personal failing. The fix is rarely "try harder to make friends"; it is putting yourself in recurring contexts where friendship can form naturally over weeks.

VIGNETTE 2 · EXAM PANIC

"I knew the material. I walked into the exam and my mind went completely blank."

Daniel, 21, is a third-year engineering student. He has always been a high achiever and has never had a problem with exams before. In his mid-term thermodynamics exam, he sat down, opened the paper, and his mind went blank. His heart was racing, his hands were shaking, and he could not read the first question. He sat through the full two hours unable to write anything coherent. He is now terrified of the final.

What is happening: Daniel is experiencing acute exam anxiety, which has crossed from adaptive arousal into panic-level activation that impairs cognitive function. The fear of recurrence is now itself a stressor, raising the likelihood of recurrence.

What helped: Daniel learned the breathing techniques from Chapter 10 (the physiological sigh and 4-7-8 breathing) and practised them daily for two weeks. He worked with a study skills advisor to take several timed practice exams under realistic conditions, gradually exposing himself to the exam-like state while remaining regulated. He reframed the arousal ("this is energy, not catastrophe") and developed a pre-exam routine (a particular meal, a particular walk, a particular breathing exercise) that signalled safety to his nervous system. In the final, he still felt nervous but was able to complete the exam and achieved his usual standard.

Non-clinical takeaway: Exam panic is a nervous-system response, not a knowledge problem. The interventions target the nervous system, not the studying.

VIGNETTE 3 · FINANCIAL STRAIN AND SHAME

"I cannot afford to go out with my friends, so I keep making excuses, and now they have stopped asking."

Amara, 20, is a second-year student from a low-income family. She is working 22 hours a week at a coffee shop to cover rent and is struggling to keep up with reading. Her friendship group goes out to restaurants and bars regularly, and the cost — even splitting — is more than she can afford. She has been declining invitations with vague excuses for two months. Her friends have stopped inviting her. She is now socially isolated and the isolation is compounding the financial stress.

What is happening: Amara is caught in a cycle where financial stress forces social withdrawal, and the withdrawal worsens the mental-health impact of the financial stress. Her friends, not understanding the real reason, have interpreted her withdrawal as disinterest.

What helped: Amara talked to one trusted friend in the group and explained the situation honestly. The friend was supportive and proposed lower-cost alternatives — picnics, free campus events, study sessions — that the group has since adopted. Amara also contacted the financial aid office and learned she was eligible for an emergency bursary that reduced her work hours to 14 per week. She joined the campus food pantry, which freed up additional money.

Non-clinical takeaway: Money shame thrives in silence. Honest disclosure to even one trusted person often produces both emotional relief and practical options that would not have surfaced otherwise.

VIGNETTE 4 · ROOMMATE CONFLICT

"My roommate plays music until 2 a.m. and I cannot sleep, but I cannot confront them."

Liam, 19, shares a residence-hall room with a roommate whose sleep schedule is roughly 2 a.m. to 10 a.m., while Liam's is 11 p.m. to 7 a.m. The roommate plays music and games late into the night; Liam lies awake, increasingly angry and exhausted, but avoids raising it because he "doesn't want to make things awkward". After eight weeks, Liam is sleep-deprived, his grades are slipping, and he is starting to dread going back to his room.

What is happening: A solvable practical problem has escalated into a mental-health issue because of avoidance. The avoidance is itself driven by anxiety about conflict.

What helped: Liam spoke to his residence advisor, who offered to facilitate a conversation. In the conversation, Liam used direct, specific, non-accusatory language: "When you play music after midnight, I cannot sleep, and it is affecting my classes. Could we agree on a headphone rule after 11 p.m.?" The roommate, surprised, agreed readily — they had not realised the music was audible through Liam's earplugs. The two agreed on a written room-share agreement covering sleep times, guests, and shared items.

Non-clinical takeaway: Most roommate conflicts are solvable with direct, specific communication early. Avoidance converts solvable problems into chronic stressors. Residence advisors are trained to facilitate these conversations and are an under-used resource.

VIGNETTE 5 · INTERNATIONAL STUDENT ISOLATION

"I have been here four months and I still feel like I am performing a version of myself that is not really me."

Wei, 22, is an international graduate student from China. His English is fluent but he is self-conscious about his accent and occasional word-finding. He has made no close friends among domestic students; his social life is entirely within the Chinese student community, which he values but which does not feel like full engagement with the host culture. He is increasingly homesick, his sleep is poor, and he is questioning whether to continue his programme.

What is happening: Wei is experiencing acculturative stress and minority stress (Chapter 12), compounded by the international student transition (Chapter 11). His reliance on compatriot community is protective but is also limiting his engagement.

What helped: Wei joined a research group whose weekly meetings gave him structured, low-stakes interaction with domestic students around a shared task. He attended an international student support group run by the counselling service, where he discovered his experiences were typical. He reduced calls home from daily to twice weekly, which initially increased his anxiety but eventually forced more local engagement. He also accepted that he did not need to "perform" a different version of himself; the version that felt natural to him, accent and all, was sufficient.

Non-clinical takeaway: International student adjustment is a multi-year process. Integration of compatriot community and host-culture engagement is generally healthier than choosing one over the other.

VIGNETTE 6 · POST-BREAKUP DISTRESS

"It has been six weeks and I still cannot stop thinking about them. I should be over it by now."

Sofia, 20, ended a two-year relationship two months ago. The decision was mutual and the breakup was amicable, but she is struggling more than she expected. Sleep is disrupted (she wakes at 4 a.m. most days), appetite is reduced, concentration is poor, and she has withdrawn from most social activities. She is also self-critical about the struggle — "I should be over it by now" — which is itself a source of distress.

What is happening: Sofia is experiencing grief, which is the normal response to a significant loss. The timeline is within the normal range; the cultural expectation of being "over it" in a few weeks is unrealistic and unhelpful. The self-criticism about the grief is compounding it.

What helped: Sofia read about grief timelines and recognised that her experience was normal. She started a brief journaling practice (Chapter 10) to process the relationship and the breakup. She resumed two social activities she had dropped. She spoke to a counsellor for four sessions, which she used to talk through the relationship and what she wanted to learn from it. Her sleep improved gradually over the following month; her mood lifted over the following two.

Non-clinical takeaway: Grief has its own timeline. Self-criticism about grief extends it; self-compassion shortens it.

VIGNETTE 7 · PERFECTIONISM-DRIVEN BURNOUT

"I cannot remember the last time I enjoyed any of this. I am just running on fumes."

Aiden, 23, is a final-year medical student. He has been a top performer throughout his programme, but in the last six months he has noticed that he no longer enjoys any of his work, takes no pleasure in his achievements, and is functioning on willpower and caffeine. He is sleeping five hours a night, has stopped exercising, and has not seen friends outside the programme in months. He is considering dropping out but is terrified of the consequences.

What is happening: Aiden is experiencing burnout — a state of emotional exhaustion, depersonalisation, and reduced sense of accomplishment that results from chronic, unrelieved stress, particularly in demanding professional training. The perfectionism that produced his academic success is now driving the patterns that are undermining him.

What helped: Aiden took a two-week medical leave, which he initially resisted but which his GP recommended. He used the time to sleep, walk outside, and reconnect with two close friends. On return, he worked with a counsellor on the perfectionism patterns (Chapter 7), set explicit "good enough" criteria for his work, reduced caffeine, and committed to one social activity and two exercise sessions per week. He did not drop out. His performance remained strong; his experience of the work changed substantially.

Non-clinical takeaway: Burnout in high-achieving students is often mistaken for "not being tough enough". The fix is rarely more effort; it is usually a change in the patterns that produced the burnout.

VIGNETTE 8 · LGBTQ+ IDENTITY AND FAMILY CONFLICT

"My family found out I am seeing someone of the same gender, and they have stopped speaking to me."

Jamal, 19, came out to his family during winter break. His parents, who are religious, responded with silence that has now lasted three weeks. They have stopped answering his calls. He is back at university, financially dependent on them for the rest of his degree, and is experiencing acute anxiety, sleep disruption, and intrusive thoughts about his future. He is questioning whether he should "take it back".

What is happening: Jamal is experiencing minority stress (Chapter 12) at acute intensity, with the additional financial precarity of being dependent on the rejecting family. The situation is genuinely serious and warrants professional support.

What helped: Jamal contacted the campus LGBTQ+ centre, which connected him with a counsellor experienced in family-rejection cases. He spoke with the financial aid office about emergency options if family support were withdrawn. He joined a peer-support group for LGBTQ+ students with family difficulties. He was honest with two close friends about what had happened, which reduced the isolation. The family eventually resumed limited contact; the situation remained difficult but no longer felt unmanageable.

Non-clinical takeaway: Family rejection is a serious adverse event with real mental-health consequences. The most powerful protective factors are community with others who share the experience, professional support from providers who understand the specific dynamics, and practical planning for the financial and logistical implications.

VIGNETTE 9 · DOOMSCROLLING AND SLEEP

"I cannot fall asleep without scrolling, and I cannot get through the day without three coffees."

Yuki, 20, has a familiar pattern: she gets into bed around midnight, opens her phone "for a few minutes", and surfaces two hours later having scrolled through news, social media, and video. She falls asleep around 2 a.m., wakes at 8 a.m. exhausted, drinks three coffees before noon to function, naps in the afternoon, and repeats. Her mood has been low and her anxiety high for several months.

What is happening: A self-reinforcing loop of bedtime phone use, sleep disruption, daytime caffeine dependence, afternoon napping, and worsened nighttime sleep — with mood and anxiety degraded by all of the above.

What helped: Yuki moved her phone charger to the kitchen and bought a £10 alarm clock. The first week was difficult — she lay awake for an hour the first three nights — but by week two she was falling asleep by 12:30 a.m. and waking naturally before the alarm. She reduced caffeine to one coffee before noon. She replaced the afternoon nap with a 15-minute walk. Her mood lifted measurably within three weeks.

Non-clinical takeaway: Sleep interventions are often the highest-leverage change a student can make. The phone-out-of-bedroom intervention is the single most reliable sleep fix, despite being initially uncomfortable.

VIGNETTE 10 · FOOD INSECURITY AND ACADEMIC DECLINE

"I have been skipping meals to afford textbooks, and my grades are crashing."

Carlos, 21, is a third-year student whose family financial situation deteriorated after his father lost his job. Carlos is working 18 hours a week, but it is not enough. He has been skipping breakfast and lunch most days to save money, eating one meal in the evening. His concentration in afternoon classes has collapsed, his mood is low, and he has missed several deadlines.

What is happening: Food insecurity (Chapter 8) is producing direct cognitive and emotional effects — the brain, undernourished, cannot sustain attention or regulate mood. The shame around the situation is preventing him from accessing the support that exists.

What helped: Carlos's academic advisor, noticing the missed deadlines, asked gently how things were going. Carlos disclosed the financial situation. The advisor connected him with the basic-needs office, which enrolled him in the campus food pantry, helped him apply for emergency aid, and identified a government food-assistance programme he was eligible for. Carlos started eating three meals a day within two weeks. His concentration and mood improved substantially within a month.

Non-clinical takeaway: Food insecurity is common, addressable, and not a personal failing. The cognitive and emotional effects of undernourishment can mimic or worsen depression and anxiety; addressing the food often substantially addresses the symptoms.

VIGNETTE 11 · SOCIAL ANXIETY

"I avoid classes where I might have to speak, and I have started avoiding the lectures where I might be seen."

Priya, 20, has always been shy, but in the last year her social anxiety has escalated. She avoids seminars where participation is expected, has started avoiding large lectures because she fears being called on, and spends hours after social interactions ruminating about what she said. Her avoidance is now significantly restricting her academic and social life.

What is happening: Priya's social anxiety has crossed from ordinary shyness into a condition that is significantly impairing her functioning. Self-care practices alone are unlikely to be sufficient; professional support is warranted.

What helped: Priya contacted the counselling service after reading a screening tool result that suggested elevated anxiety. She was assessed and offered cognitive-behavioural therapy, the evidence-based treatment for social anxiety. Over twelve sessions, she learned to identify and challenge the catastrophic thoughts that drove her avoidance, gradually exposed herself to feared situations in a structured way, and developed social confidence that had not previously been available to her. She did not become an extrovert; she became able to do the things that mattered to her without being paralysed by fear.

Non-clinical takeaway: When self-care is not enough — when avoidance is significantly restricting life — professional support is the right next step, and effective treatments exist.

VIGNETTE 12 · POST-GRADUATION VOID

"I graduated with a first-class degree, got the job I wanted, and I have never felt more lost."

Elena, 23, graduated six months ago. She has a good job in her field, a flat she shares with a friend, and a life that looks, from the outside, like a success. She feels persistently flat, disconnected from her work, and uncertain about whether any of it is what she actually wants. She has not spoken to anyone about this because she feels she "should" be happy.

What is happening: Elena is experiencing the post-graduation transition (Chapter 11), compounded by the contrast between her external success and her internal experience. The expectation of happiness is itself preventing her from seeking support.

What helped: Elena eventually spoke to a friend from university and discovered that several of her peers were having similar experiences. She started a weekly therapy appointment, where she explored the gap between the life she had planned and the life she actually wanted. She joined a recreational sports club, which gave her a structure and a social connection outside work. The flatness did not lift immediately, but over six months she began to feel more like herself, with a clearer sense of what she wanted from the next phase.

Non-clinical takeaway: The post-graduation transition is real and under-acknowledged. External success does not insulate against it. Honest conversation with peers, professional support, and patient identity work are the typical path through.

The vignettes above are not exhaustive; they are representative. Most student mental-health experiences do not look exactly like any of them, and most contain elements of several. The point is not to label yourself but to recognise that the experience you are having is shared, that the patterns are known, and that the practices and supports described in this guide are designed for exactly these situations.

APPENDIX B

Resource Directory

A curated directory of helplines, apps, books, websites, and search strategies. Inclusion does not imply endorsement of every claim made by a resource; it implies that the resource is widely regarded as reputable and useful.

SAFETY NOTE This directory is for signposting only. Inclusion of an app, book, or website does not constitute medical advice or a guarantee of effectiveness. For mental-health concerns, please consult a qualified professional. For emergencies, contact your local emergency services.

B.1 Crisis lines (international)

The crisis lines listed in Chapter 15 are the primary emergency resource. For convenience, the international directory **findahelpline.com** maintains a current list of crisis lines by country and is the most reliable starting point if you are outside the regions explicitly listed in Chapter 15.

B.2 Mental-health apps

Apps can be useful supplements to — but not replacements for — professional support. The following are widely used, evidence-informed, and free or low-cost. None is a substitute for therapy.

APP	FOCUS	COST	NOTES
Smiling Mind	Mindfulness	Free	Developed by psychologists; broad age range; well-evidenced.
Headspace	Mindfulness, sleep	Free tier + paid	Well-designed; student discounts often available.
Calm	Mindfulness, sleep, anxiety	Free tier + paid	Strong sleep stories and breathing exercises.
Insight Timer	Mindfulness	Free	Largest free library of guided meditations.
Woebot	CBT-based chatbot	Free	Evidence-based; not a replacement for therapy.
Finch	Self-care gamified	Free tier + paid	Gentle, popular with students.
Sanvello	Mood, CBT, meditation	Free tier + paid	Often free through university or insurance.
Streaks / Habitica	Habit tracking	Paid / free	Useful for building protective-factor routines.

B.3 Websites and online resources

RESOURCE	FOCUS	URL
World Health Organization — Mental health	Global mental-health information	who.int/health-topics/mental-health
National Institute of Mental Health (US)	Authoritative information on conditions	nimh.nih.gov
NHS Every Mind Matters (UK)	Practical well-being advice	nhs.uk/every-mind-matters
Headspace Health — Student Resources	Student-specific content	headspace.com/student
Active Minds	US student mental-health advocacy	activeminds.org
Student Minds (UK)	UK student mental-health charity	studentminds.org.uk
Jed Foundation	US teen/young-adult mental health	jedfoundation.org
Nightline Association	UK student-run listening service	nightline.ac.uk
ULifeline	US campus-specific resource finder	ulifeline.org
Mind (UK charity)	Comprehensive mental-health information	mind.org.uk
Mental Health America	US screening tools and resources	mhanational.org
The Trevor Project	LGBTQ+ youth crisis and resources	thetrevorproject.org
Therapy for Black Girls	Directory and resources for Black women	therapyforblackgirls.com
Identity LGBTQ+ support	International LGBTQ+ resources	(search locally; many country-specific services exist)
BEAT (UK)	Eating disorders	beateatingdisorders.org.uk
NEDA (US)	Eating disorders	nationaleatingdisorders.org
RAINN	Sexual assault (US)	rainn.org

Love is Respect	Dating abuse (US)	loveisrespect.org

B.4 Books (non-clinical, evidence-informed)

The following books are written for general readers, are evidence-informed, and complement the topics in this guide. They are not substitutes for professional care but can be useful companions.

- **Viktor Frankl, *Man's Search for Meaning*** — On finding meaning in difficult circumstances. Classic, brief, profound.
- **Kristin Neff, *Self-Compassion: The Proven Power of Being Kind to Yourself*** — The leading researcher's accessible introduction to self-compassion.
- **Jon Kabat-Zinn, *Wherever You Go, There You Are*** — An accessible introduction to mindfulness practice.
- **James Pennebaker, *Opening Up by Writing It Down*** — On expressive writing for emotional processing.
- **Kelly McGonigal, *The Upside of Stress*** — On reframing the stress response.
- **Brene Brown, *The Gifts of Imperfection*** — On shame, vulnerability, and authenticity.
- **Matt Haig, *Reasons to Stay Alive*** — A memoir of depression and recovery; widely loved by student readers.
- **Oliver Burkeman, *Four Thousand Weeks*** — On time, mortality, and the futility of productivity-as-salvation; useful antidote to perfectionism.
- **Johann Hari, *Lost Connections*** — On the social and environmental drivers of depression; popular, controversial, but thought-provoking.
- **Wendy Suzuki, *Healthy Brain, Happy Life*** — On exercise and the brain; accessible neuroscience.
- **Matthew Walker, *Why We Sleep*** — The leading popular science book on sleep; very readable.
- **Lori Gottlieb, *Maybe You Should Talk to Someone*** — A therapist's account of therapy, from both sides; useful for demystifying the process.

B.5 Finding a counsellor or therapist

If you are seeking ongoing therapy beyond what your university counselling service provides, the following directories can help you find a qualified provider. Insurance, location, and specialism will all be factors; many providers offer sliding-scale fees.

DIRECTORY	REGION	SPECIALISM
Psychology Today	US	General therapist finder
BACP Find a Therapist	UK	BACP-registered counsellors
BPS Directory	UK	Chartered psychologists
CBC Directory	Canada	Canadian certified counsellors
PACFA	Australia	Registered psychotherapists and counsellors
Open Path Collective	US	Sliding-scale, low-cost therapy
Therapy for Black Girls	US	Black female therapists
Latinx Therapy	US	Latinx therapists
Inclusive Therapists	US/International	Therapists from marginalised communities
NHS Talking Therapies	UK	Free NHS therapy (self-referral)

B.6 Glossary of key terms

TERM	MEANING
Anhedonia	Loss of interest or pleasure in activities usually enjoyed; a core feature of depression.
Biopsychosocial model	The framework that mental health emerges from the interaction of biological, psychological, and social factors.
Circadian rhythm	The body's internal ~24-hour clock, which regulates sleep, hormones, and many other processes.
Cognitive behavioural therapy (CBT)	A structured, evidence-based form of psychotherapy that targets unhelpful thought and behaviour patterns.
Cortisol	The body's main stress hormone; produced by the adrenal glands.
Elevated screening risk	A screening score in a range suggesting higher probability of a clinically significant condition; warrants follow-up but is not a diagnosis.
Flourishing	A state of high mental health combining emotional, psychological, and social well-being.
GAD-7	Generalised Anxiety Disorder 7-item scale; a widely used anxiety screening tool.

Languishing	A state of stagnation and emptiness, below flourishing but not meeting criteria for a mental disorder.
Minority stress	The chronic excess stress experienced by members of stigmatised groups.
Parasympathetic nervous system	The branch of the autonomic nervous system responsible for rest, digest, and recovery.
PHQ-9	Patient Health Questionnaire 9-item scale; a widely used depression screening tool.
Protective factor	A condition or practice associated with lower probability of a mental-health condition.
Risk factor	A condition or experience associated with higher probability of a mental-health condition.
Rumination	Repetitive, narrowly focused negative thinking; a central feature of depression.
Sleep hygiene	The set of behavioural and environmental practices that support good sleep.
Sympathetic nervous system	The branch of the autonomic nervous system responsible for fight-or-flight activation.

B.7 How to use this directory

This directory is a starting point, not a complete map. For most students, the most useful first step is the university's own counselling service or student mental-health service — they know the local landscape, can refer to appropriate specialists, and are usually free. The apps, websites, and books listed here can supplement that support, can be useful while waiting for an appointment, and can be valuable for students who are not yet ready to seek formal help. They are not, in themselves, treatment.

If a resource listed here is not a fit — for any reason — please do not conclude that no resource will be. The landscape of mental-health support is wide, and the right fit for one student is the wrong fit for another. Keep looking until you find what works for you.

You are not alone in this. Please let support in.

If you are in immediate danger, contact your local emergency services now. In the EU and many regions: 112. In the US and Canada: 911 / 988. In the UK: 999 / Samaritans 116 123. In Australia: 000 / Lifeline 13 11 14.

IMPORTANT

This guide is for prevention and psychoeducation only. It is not a substitute for professional care, diagnosis, or treatment. It does not provide therapy instructions, medication advice, or crisis intervention. If you are struggling, please reach out to a qualified counsellor, mental-health professional, or student support service.